

Jeena Sikho Life (JSLL)

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Call: May 2024

21st May, 2024

The Manager -Listing Department,
National Stock Exchange of India Limited,
"Exchange Plaza", 5th Floor,
Plot No. C/1, G Block, Bandra -Kurla Complex,
Bandra (East), Mumbai – 400 051.

Scrip Code: JSLL

Dear Sir/Madam,

Sub.: Transcript of Analysts/ Investors Earnings Conference Call held on 16th May 2024, pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulation, 2015.

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we hereby submit the Transcript of analysts / investors Earning Conference Call conducted on 16th May 2024 at 05:00 PM to discuss the Company's Financial Year 2023 -2024 results.

You are requested to kindly take the same on your record.

Thank you.

Yours faithfully,

For Jeena Sikho Lifecare Limited

Manish Grover
Managing Director
DIN: 07557886

Encl.: a/a
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"Jeena Sikho Lifecare Limited
H2-FY24 & FY24 Earnings Conference Call"
May 16, 2024

MANAGEMENT : MR. MANISH GROVER -- MANAGING DIRECTOR -
JEENA SIKHO LIFECARE LIMITED
MR. NANAK CHAND - CHIEF FINANCIAL OFFICER -
JEENA SIKHO LIFECARE LIMITED

MODERATOR : MR. RANVIR SINGH -- NUVAMA WEALTH
Jeena Sikho Lifecare Limited
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Moderator: Ladies and gentlemen, good day and welcome to the Jeena Sikho Lifecare Limited H2-FY24 and FY24 Earnings Conference Call. As a reminder, all participant lines will be in the listen-

only mode. There will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference, please signal an operator by pressing star and then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Ranvir Singh from Nuvama Wealth. Thank you and over to you, sir.

Ranvir Singh: Thank you, Dorwin. So on behalf of Jeena Sikho Lifecare Limited, I extend a very warm welcome to all participants on the H2-FY24 and FY24 financial results discussion call. Today on our call, we have Mr. Manish Grover, Managing Director, and Mr. Nanak Chandji, who is CFO of Jeena Sikho Lifecare Limited.

Before beginning with this call, I would like to give a short disclaimer that this call may contain some of the forward-looking statements which are completely based upon the management's beliefs, opinions, and expectations as of today. These statements are not a guarantee of the company's future performance and involve unforeseen risks and uncertainties.

With this, I would have to hand over the call to Mr. Manish, sir, for his opening remark. Over to you, Manish, sir.

Manish Grover: Namaskar to everyone. Thank you, Ranvir. I am very happy to be able to share Jeena Sikho Lifecare's strong financial and operational performance with all of you on behalf of H2 and FY24. We have put in a lot of hard work to become the best, and we are reaping the benefits. We can see that our revenue is increasing from both our services and products. So, in H2 and FY24, our revenue is INR167 crores and the total turnover is INR324 crores. This means that we have grown by 42% and 59%.

We have also seen good results from our hospitals, clinics, and daycares. This year, the revenue from their services has increased by 122%. This is a proof that we are committed to providing better quality and alternate healthcare services. Let me tell you about our returns for the past 4 years. In 2021, our revenue was INR134 crores and the profit was INR10 crores. We made a profit of 8%.

In 2022, the turnover was INR145 crores and the profit was INR11 crores. We made a profit of 8%. In 2023 after IPO, we made a profit of INR204 crores and made a profit of INR33 crores, which is 16%. In 2024 as of yesterday's balance sheet, we made a profit of INR324 crores and made a profit of INR69 crores, which is 21%.

In the last half-yearly 2 of 2023, we had a turnover of INR117 crores. In this half-yearly 2, we have a turnover of INR167 crores. This is a growth of 42%. In H1 FY24, we had a revenue of INR157 crores and in H2 FY24, we had a revenue of INR167 crores. In total, we have made a profit of INR324 crores. This year, we increased the number of beds in our hospital. Last year, there were 460 beds in our hospital, but we added 817 beds. In total, we have 1270 beds in our hospital in 2024.

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Now, we have 25 states, 97 cities and villages in our country. The reaction of people to our newly opened hospital has been very enthusiastic. Our products vertical has grown by 32% year-over-year, which shows our ability to increase our free cash-generating revenue system. With this growth, we have seen a growth of 61% year-over-year EBITDA and 66% year-over-year profit, which also includes a profit margin. This shows that we are able to operate our business well and also increase it.

Mr. Nanak will highlight the financial performance. Before that, I would like to share some data with you. Mr. Nanak, please share your financial data and then I will share some data. Good afternoon, everyone.

Nanak Chand: As mentioned by Manish sir, we

have achieved a healthy financial performance during FY24

and FY25. Here are some key highlights. FY24 results, revenue from the operation grew by 59% year-over-year basis to INR234 crores. Mainly driven by the hospital services, which grew by 1.22% year-over-year basis to INR139 crores, while production grew by 32% up to INR186 crores. The EBITDA during FY24 grew by 102% year-over-year basis to INR93, implying EBITDA margin of 29%.

The FY24 PAT grew by 105% year-over-year basis to INR69 crores. We attained the healthy balance sheet with net cash in equivalent of INR62 crores by the year-end of FY24 and the ROE on the ex-cash ROC is 36% and ex-ROC is 80% respectively.

We spent the INR80 crores capacity in FY24 and with this, we are open to follow the question and answer session. Thank you. One minute.

Manish Grover: Before question and answer, I would like to tell you that there are 20 lakh beds in India, out of which 60%-70% are occupied, which means 12-14 lakh patients per day and Jeena Seekho has worked on 1300 beds till now and out of all the NABH accredited centres in India, 10% share is

with us. In India, NABH is 240. We have more than 24 within 2 years and this is a very big opportunity that we sell medicines worth INR2.5 lakh crores in India. Whereas, we have sold medicines worth INR136 crores in the last few years, this is a very big opportunity that we have. Apart from this, thousands of allopathy hospitals fight among themselves. We have complete overall, Jeena Seekho is working in Ayurveda. And we are only one and one. There is no one around us. And our business model is very easy. And we solve the biggest health problems of human beings. Because of which we get benefit of mouth to mouth marketing. Today, there are 7.5 lakh registered Ayush Prakashna doctors in India and 13 lakh allopathy doctors that is why we do not lack any talent and we are very different from allopathy because allopathy works on the disease and we work on the root of the disease. And our capex per bed is approximately INR3 lakhs, whereas, against that we spend INR50 lakhs to INR1 crore on allopathy hospitals. Our cost of INR3 lakhs per bed is INR50 lakhs to INR1 crores and our average revenue per occupied bed is INR7,900 in this balance sheet and the ROCE of the company is 71% remain and the ROCE CAGR of the last 3 years is 35%. And from SME, we will come to the main board in the financial year 2025 and for management assurance, we have started Big 4. And we have appointed E&Y for the management performance review for Jeena Sikho Lifecare Limited
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financial review 2024-25. And very soon, our goal is to become the number one corporate governor and the number one ayurvedic company.
So sir, now you can ask your questions and answers.
Moderator: The first question is from the line of Dhruv Agarwal from Nivisha Investments. Please go ahead.
Dhruv Agarwal: Namaste, Acharya ji. Sir, I got the first question from you. Sir, you are saying that there are 24 NABH centers. Sir, I had visited the NABH website. So, no doubt, there are more centers than NABH. But what I was able to see is that there are only 14 centers accredited by NABH?
Manish Grover: Yes, I got the message. The sheet is not updated. The daycare center is not included in it. The daycare centers in Delhi and the centers that we have got, every year or every year and a half, there is a review of NABH again. So, centers which completed the year, we did it again, but they did not update it. So, their site is not updated. We have all the certificates.
Dhruv Agarwal: Okay. So, sir, you are saying that there are 24 NABH certificates. Sir, what is your plan for the future? How many centers are you going to get NABH accredited?
Manish Grover: Sir, we are going to increase the number of 500 beds approximately in the next year. We are going to add this new number in our 270 beds. By the end of this year, we will have 1800 beds.

Dhruv Agarwal: Sir, you are saying 500 beds by 2025, right? Sir, 500 beds by 2025.
Manish Grover: Yes, by 31st March 2025, we will add 500 beds. The 1300 beds, there are 1270 beds now. We will add 500 new beds in that. And out of the first 1300 beds, in the last year's balance sheet, there is 37% occupancy. So, I will take it up to 55% within 3 months. So that I can increase my profit by up to 50% in the next year. And apart from this, we are working on a clinical trial and we are also doing an open market OTC survey. We are also planning to launch it in the market soon.
And from 1st April, there is a cashless health insurance. So, we are going to get the benefit of that which will be seen in this financial year. Earlier, the health insurance that we got, we got it through reimbursement. It is cashless from 1st April. And on 15th April, I have got 3 new hospitals, NABH, of Lucknow, Mumbai and Meerut. These are 3 big hospitals. With this, I have increased the admission of about 200 patients daily, average. And the ticket size of 1 patient increases by INR8000. So, you can calculate how much my sale is going to increase. That is why I told you that with 38%, I have made a profit of INR69 crores last year. If my bed occupancy, I am telling you about my old bed. If it becomes 55%, then I will take a growth of 50%. If it becomes 80%, then I will be able to make my profit by 3 times. So, the whole game is about the occupancy of the hospital bed.
So, now we have started some new TV channels, some newspapers, some social media campaigns. We have started working on some new diseases. For example, we have started working on infertility. We have started working on heart problems. Earlier, we were working on Jeena Sikho Lifecare Limited
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kidney failure, liver failure and cancer. Now, we have started a new heart segment. We have started an infertility segment.
Even though I have a separate hospital of 100 beds, I am only starting it for infertility. So, within 3 to 4 months, in 6 months, I will have 400 beds and in the whole year, I will have another 500

beds. So, before next year, we will be at 1800 beds. And the open market planning is also going on.

Dhruv Agarwal: Sir, you said that there will be an addition of 500 beds by 2025. In addition to that, you are thinking of adding another 1800 beds for infertility?

Manish Grover: No, no. This is included in 500 beds. Out of 500 beds, I will add 100 beds for infertility. Out of 300 beds, I am working on heart and liver problems.

Dhruv Agarwal: Okay, sir. Sir, you said that on an average, a patient comes to you and gives you a revenue of almost INR8000. Sir, that patient how many days...

Manish Grover: Per day, per bed.

Dhruv Agarwal: Per day, per bed. Right, sir. So, how many days does that patient stay with you, on an average?

Manish Grover: Minimum, from 6-7 days to 21-25 days. We advise the patient to stay for a minimum of 10 days.

The average patient stays for 7 days. Because, in health insurance now in India the villages do not have health insurance, the people of the cities bring health insurance policy. They get their treatment from that. Right now, awareness is increasing in the villages. And we hope that some other states are also bringing new policies.

For example, in Rajasthan, our government, from where our panel is running, three new panels have opened on that site. One, Uttarakhand has given a new panel. Karnataka has also given a new panel. And apart from this, DGHS is starting again in Delhi. Apart from this, we have also started in Karnataka, in NDMC, in CAPF. Now, ECHS is also coming. Ayurveda has also been approved in Army. So, we will also get that in 1-1.5 months.

Dhruv Agarwal: So, Acharya ji, as you said that the occupancy of 37% you are going to take it up to 55%. So, what steps are you taking so that the occupancy goes up to 55%, sir?

Manish Grover: You open the channel. On AB

P News, at least 15 times a day, I have started a new slot on all

ABP News channels. I have started a new slot on Zee News. I am continuously working on the India team. Now, I have taken up the other channels of Zee. We are talking about the packages of newspapers. So, we work from social media, newspapers and TV from all three places. And now, in Ayurveda, cashless has also come, health insurance, which was not there before. And in Ayush, it is effective from this 1st April, we will reap that benefit.

Dhruv Agarwal: Right. So, Acharya ji, you feel that this advertisement, you can get your occupancy level to 65%. Do you feel it is possible through advertisement.

Manish Grover: I have been working like this for the past few years. I know how much and when I have to add TV because we are the masters of our business. When do we have to add it? Which promotional

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ad do we have to run? Which social media do we have to catch? We have been doing that for the past few years. You just notice one thing.

In April 2022, my turnover was INR16 crores. On 31st March, my turnover was INR39 crores.

So, I did not increase the expense of TV, the expense of TV increased by 5%. But how did the sales increase from INR16 crores to INR39 crores? So, we have been continuously trying to cure the patient. We have only one aim, the patient should be cured. When the patient is cured, then he sells to another patient.

Now, we have started a new system. To maintain the patient for a year, we sell him a one-year package. So, the patient stays with us. So what happens is that, if the patient has any relative who is sick, then he calls us directly. And Ayurveda is a totally blue machine market. We do not have any competition. We are unique. The more mouth publicity we get, the more benefit we get.

Moderator: Thank you. The next question is from the line of Ronit Kapoor from Elara Capital. Please go ahead.

Ronit Kapoor: Can you just give me a segment-wise break-up of revenue and the guidance segment-wise guidance in terms of revenue and EBITDA margins for the next two years?

Manish Grover: Nanak ji Can you tell us once, Nanak ji?

Nanak Chand: Segment-wise revenue in EBITDA is right now not handy. So, we will provide this data later while we are going for the offline discussion, sir.

Ronit Kapoor: Okay. And one more question. Overall guidance you can give, right? Overall guidance in EBITDA?

Manish Grover: Can we tell us about last year? How much did it grow from the previous year from 23 to 24? We will tell you about 2023 and 2024. So, I will start telling you from 2021 and 2022. Financial year 2021-22, we closed the total turnover to INR146 crores, out of which only 10% of the EBITDA was generated and in 2022-23, we closed at INR204 crores total revenue balance sheet was closed. Out of which only 31% only was services, panchakarma services and after that 23-24, we closed the total revenue balance INR324 crores out of which 43% and the rest was from medicine.

Ronit Kapoor: Yes, so can you about the margins segment-wise?

Nanak Chand: Right now, the EBITDA margin is not handy with me, so I will provide you the one on the offline mode.

Ronit Kapoor: Overall guidance for next 2 years, what would be revenue and EBITDA would be?

Manish Grover: I only understand PAT. . My plan for the next year is that if we take our occupancy at 55%, which we have just closed at 37%, then our revenue will be around INR450 crores. And my target for the PAT will be around 25% because all our basic expenses are getting completed.

Like the hospital. Right now, my Mumbai hospital is running at 25% occupancy. So the expenses

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are there. If I take it from 25% to 55%, then my expenses will not increase, but the profit will increase.

Similarly, my Lucknow is running at 50%. If I take it from 50% to 80%, then my expenses w

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remain the same. But my profit will increase. And I am planning to increase 500 beds this time.

Out of that, 100 beds will start in 3 months and 300 beds will start within 6 months. I don't want to wait till the end of the year. They will start in the middle of the year because we have planned accordingly. And now we are opening 15 new hospitals in this year. I have told you about 500 beds. Basically, these 15 hospitals are opening there wherever the government's plan is.

Wherever we have CGHS dispensaries and wherever we are already getting a lot of inquiries from this area.

So the area from which we are getting more inquiries from the patient, we are starting there first because we get data from the call center. That from which area we are getting more calls. In which area our patient is more or in which area the impact of our newspaper and TV is very good. So we are starting a new hospital in that area first.

Ronit Kapoor: Okay. And how many SKUs do you have for the Ayurvedic medicines that you sell?

Manish Grover: We have about 300 formulas now which is a patent, which is only sold in our clinic. But we have worked on 10 formulas now which we are planning to launch in OTC within 4 to 6 months.

Ronit Kapoor: And what is the average selling price of one SKU?

Manish Grover: The medicines that are being sold in our clinic so far. In that, the minimum is starting from INR600 per product. After that, the average is around INR1200 rupees and all this is a patent. It is not available anywhere else. It is only available in our clinic. What I am planning in OTC now. I am planning products from INR150 to INR600. There is diabetes, blood pressure, liver, kidney, stress. Research is going on these. And I am doing market survey. I have hired a team now which we are planning to launch in OTC.

Ronit Kapoor: Okay. And the last question is in the industry. What is the minimum intensity like? Who is your nearest peer and all that? Understood will also work.

Manish Grover: No, sir. We do not see anyone working like us. People are working on wellness. We are working on treatment. We are going to start the wellness sector now. We are also going to start wellness together. So there is no one now, sir.

Moderator: Thank you. The next question is from the line of Priyam Khimawat from Ask Investment Managers. Please go ahead.

Priyam Khimawat: Sir, as you said that we would be planning to open 500 new beds in this year and if I look at our company from the perspective of 3 to 5 years, can we easily double our bed count from 1400 to say 2800?

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Manish Grover: Yes, yes. I have taken the target of increasing 500 to 600 beds every year for the next 5 years. I will keep increasing 500 to 600 beds every year. I have done a lot of planning. Which cities do I have to go first? Then which cities do I have to go? Then which cities do I have to go?

Priyam Khimawat: Sir, so according to you, what is going to be the biggest challenge for you in this journey? Is it finding good doctors in our hospitals? Or is it just that we have to focus on marketing? And getting more patients accustomed to Ayurveda?

Manish Grover: Sir, we are working on it on the basis of document. We understand the nerve of the world. We have developed a research team. We have about 20 research papers in our pipeline and 3 researches, 100 beds, a clinical trial and a clinical trial of medicines. All this is going on already. So no one can even say that Ayurveda is behind allopathy because we are working on a paper together. So we are working with full preparation, sir. My goal is to make it India's number one Ayurvedic company.

Priyam Khimawat: Got it, got it. Sir, now I see my medicine revenue we have closed it at INR186 crores FY24, but when I go and look back at volumes that you have given in the presentation in that on page no.

30 medicine order volumes are declining. It used

to be 2.67 lakhs. In FY22 it became 2.27. And then it declined to 2.09. Any reason for this?

Manish Grover: No, we have reduced our COD business. We have reduced it ourselves. We had a COD business, cash on delivery. We have reduced it and shifted it to clinical. Earlier, what we used to do until 2 years ago our COD business was more and patients used to come to the clinic less.

So in 2 years with a strategy because when a patient comes to the clinic and meets the doctor.

So his trust is of a different kind. Some patients get out of it who get admitted, some patients last for 3 months, 4 months, 6 months. What used to happen in COD business. His life used to be 1 month to 2 months because once a patient used to come to the doctor's office and take medicine. The percentage of repeated visits was 30%, but when a patient comes to the clinic.

His repeat ratio is 60%, 65%. So we are converting that business ourselves towards clinic sale and hospital sale. A patient brings medicine from the doctor that is our part of the strategy, sir.

Moderator: Thank you. The next question is from the line of Rohan P Parikh from OHM Stockbrokers.

Please go ahead.

Rohan P Parikh: Sir, I have a question. When we open clinics and hospitals what is the first primary difference between them? When a patient comes to the hospital what type of illness does he come for and what type of illness does a patient come to the clinic for?

Manish Grover: Okay, sir. For example, there is knee pain and the doctor said that the knee needs to be operated.

It needs to be changed. So the patient has to come to our day care center or clinic or hospital. In the clinic, only medicine is available and diet sheds are available, but in the day care center, the patient comes, takes therapy and goes home. In the hospital, the patient is admitted with us for 7 days, 10 days.

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And all the health insurance covers or the government panel only when the patient is admitted there, he gets his bill. So, for example, there is knee pain. The doctor said that the knee needs to be changed. So, we will treat the knee by doing Janu Basti.

So, for that, the day care center or the hospital is preferred, but in the clinic, the patient comes, takes therapy and goes home. Similarly, the doctor said that the knee needs to be operated. We will admit him to the day care center or hospital without operation. There is a kidney failure patient. There is a liver failure patient. There is a cancer patient. All these are not cured by medicines. For them, they have to be admitted or have to come to the day care center. There is sugar. There is BP. There is simple joint pain. There is gas. There is acid. These all patients come to the clinic.

Rohan P. Parikh: So, when a patient comes to our hospital. Let's assume a patient comes for cancer or any disease. Typically, you said that the cost is [inaudible 29:34]. You said that the setup cost is INR3 lakhs for one bed. What is our breakeven for beds, sir?

Manish Grover: What is our break-even? All of us come to any hospital on the first day for break-even we are there as soon as we open and we are not investing in any capex whatever building we are taking, we take it on rent and we open the hospital there itself where my clinic is already set up and where I have more than 400 patients visiting for a month. I open the hospital there itself and wherever we open the hospitals the very first day it comes in profit.

Rohan P. Parikh: No, you just said that the setup cost of one bed is INR3 lakhs. So, when is your payback period? In how much time does your bed breakeven?

Manish Grover: So, I told you we get INR8000 per day per bed as revenue. So, you think if my bed runs for 30 days. So, I got INR2.5 lakhs. So, in 3 months, let's assume after deducting all our expenses and get the full payback.

Rohan P. Parikh: And sir one last question from me when

we are selling this product, we are manufacturing it ourselves?

Manish Grover: No, we are not manufacturing it ourselves. Right now, we don't have our own factory. We have third party tie-ups and agreements. They make it only for us. They will make our formula only for us.

Rohan P. Parikh: Understood.

Manish Grover: And no one else can make the same formula with the same name because we have all the patents.

Rohan P Parikh: And for this, what licenses do we need?

Manish Grover: We need Ayush's license from Ayush Ministry from Ayush Ministry. Ayush Ministry has branches in every state like in UP there is Lucknow, in Punjab, there is Mohali. There is Chandigarh. In Haryana there is Gurgaon. So, there is a body in every state that approves the formula of that body and no medicine is sold without a seal. We don't sell a single pill without a seal.

Moderator: Thank you. The next question is from the line of Shashank Rastogi an individual investor. Please go ahead.

Shashank Rastogi: Guruji, I just want to reconfirm. Next year, you are giving guidance of INR450 crores as revenue and for PAT you are giving guidance of INR100 crores above?

Manish Grover: Yes, sir. I had given an estimate. It will be around INR50 crores and I have calculated this very easily if you calculate your 38% bed even if we do only 55% occupancy it becomes a growth of 50%.

Shashank Rastogi: Guruji, in the next 5 years your target is to take your beds around INR3000 internally, am I correct sir?

Manish Grover: Yes, absolutely. In the next 3 years we will do 3,000 beds. In 5 years, we will be above 4,000 beds.

Moderator: Thank you. The next question is from the line of Jinesh Sipani from Niveshaay Investment Advisors. Please go ahead.

Jinesh Sipani: I wanted to know what incentives do you give to doctors for their salaries, what incentive system do you set for them?

Manish Grover: Sir, there is a target base. I have made some pointers for them patient satisfaction rate, ambience of the clinic, doctor's presence. We randomly take feedback from patients. For example, 100 patients visited a clinic. So, we will call 5 patients. If 4 out of those 5 patients gave the doctor a rating then we give them 1 point. This means that the doctor's salary is earned by 20%, 50% incentives. The salary and incentives we have are the best in India. Because we have doctors who are 4 years, 8 years old who have not left us.

Jinesh Sipani: I have seen this in Patanjali. Patanjali has its own doctors. They also sell their medicines. What will be the difference between our work and their work?

Manish Grover: Sir, actually, the results of our medicines are very good. We do not compromise on the quality of our medicines. If the manufacturing cost increases, we do not cut it down and we do not believe in just selling medicines. We believe in making patients believe that if they recover, they will sell more medicines.

Our aim is not to see one patient as a patient. We see 10 patients in 1 patient. We put all our efforts in that patient and our biggest healthy part which is our slogan is that medical education is more important than medical treatment. We believe in educating patients. There is a daily class in the hospital for 1 hour in the afternoon and 1 hour in the evening. They are educated on their illness so that they do not fall ill again and if someone else falls ill, we help them. Doctors are very happy with us.

And for the well-being of doctors on 16th March 2024 we have done ESOP. We have divided 1,30,000 shares in ESOP for doctors and old staff. We have done ESOP for about 738 employees on 16th March whoever stays with us for 5 years, they will get ESOP. Naturally, they will get

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an extra fund in addition to the salary incentive. We have also given 3 cars to 3 doctors on 16th March.

And in November, we have also announced to give 21 cars on Diwali whoever gives a good performance, a very outstanding performance, we will also distribute cars this time. This is a very good thing.

Jinesh Sipani: Tell me one thing, what is your margin in the medicine business?

Manish Grover: Approximately 85% gross margin.

Jinesh Sipani: And what is the net margin if you add up all the expenses in the medicine business, how much will it be?

Manish Grover: We got 22% in the balance sheet. Actually, everything is sold. From that ad the patient comes to the hospital, from that ad the medicine is sold, from that ad we have our COD and from that ad government business comes. So, our business is a compliment of each other. That's why you notice that in 2022, our profit was 8%. Our revenue was just 2.5 times, but my profit was 21%. I had a profit of INR10 crores in 2022. The profit of INR33 crores in 2023 is now 69 because if you look at the turnover it has become 324. My turnover has increased by 2.2, but my profit has become 3.75 times. So, I think we will bring a profit of more than 25% next year because our expenses are being covered and what we are planning to bring in OTC will be an extra benefit for us.

Moderator: Thank you. The next question is from the line of Harish Kumar Gupta an individual investor.

Please go ahead.

Harish Kumar Gupta: My query was that you are targeting around 500 beds this year. So, after 4 years because the market is empty in India. So, I believe that after 3 years 1500 beds because cash is available and the market is available. So, why are we not targeting 1500 beds in a single year after 3 years? I think after 3 years, it will triple.

Manish Grover: Sir, I will say 500 beds and add 700. You will all be happy, but I said 700 and if I can do 500, I will have a problem. My plan is very big. My plan is to make India's number one Ayurvedic healthcare company and naturally, I will need at least 10,000 beds for that, but I did not overcommit and planned to increase 500-500 beds. I want to make the patient happy. I still have 40% occupancy in my 1400 beds.

I still have 60% empty. If I fill that 60 my profit will be more than double though I am adding 500 new beds. So, I will invite all of you to come to our hospital and support that you come in our hospital and see our patients, see the kidneys, see the liver fails, see the cancer patient, eat our diet, the dosa made of coconut and coconut chutney with our Ayurvedic diet, taste those too.

Harish Kumar Gupta: I did want to meet you that is possible.

Manish Grover: We have a camp in Meerut. Learn how to reverse diseases in 72 hours. I stay there for 3 days in Meerut so, any time. Many people from Nuvama people have also come. Ranveer has also stayed

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there. Sharad Tripathi has also come. His father is also undergoing treatment in Lucknow. So, many people from your investor community are taking treatment from us and many have also benefited. If you ask each other, many people will be found. For those whose kidneys fail, for someone's sugar, for someone's BP.

Harish Kumar Gupta: Do you have any overseas plan?

Manish Grover: Yes, sir. Within two months, we have a plan to open a center in at least three countries. On the 15th, I have been inaugurated in Nepal.

Harish Kumar Gupta: Very good.

Manish Grover: And now we are talking about Vietnam. We are talking about Dubai in the Middle East. And we are planning clinics and hospitals. I have been to Canada. I have visited there. I have been to London. I have been to all the foreign countries. And now I have an MSME organization in the West. In which there are 112 countries. I have had two meetings with them. I have also had a meeting with their 40 ambassadors. And the Indian government has helped a lot. In 112 countries, they have got Ayurveda approved. So Ayurveda doctors can practice there.

I have

been to Mauritius a month ago. We are planning a center there. So a lot of plans are going on, sir. We are doing everything together. So in 2-3 years, if all of you are together, we will take the company to the next level.

Moderator: Thank you. The next question is from the line of Deepak Poddar from Sapphire Capital. Please go ahead.

Deepak Poddar: Namaste, Acharyaji. This is a very good result. And many congratulations for that.

Manish Grover: Thank you, sir.

Deepak Poddar: Just a couple of things. You have said 55% occupancy target in this year FY '25. And a revenue of INR450 crores. So in that, is the new facility of 300 beds added? Or is it over and above that?

Manish Grover: Ask again. The hospital of 300 beds.

Deepak Poddar: Is it added or is it separate? In this target that we have given?

Manish Grover: No, we still have a total of 1277 beds.

Deepak Poddar: Correct.

Manish Grover: Out of which, 315-324 are added. 300-plus are added. In January 2023, we bought a new hospital in Chandigarh. 300 beds near Rajpura. So within 4-5 months, it will be added. As I said, we are adding 500 beds. So 300 beds are in Chandigarh. 100 beds are in Panchkula. And 100 beds are in rest in India.

Deepak Poddar: Correct.

Manish Grover: My idea is that we will add 650-650 beds. But I am making a commitment of 500.

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Deepak Poddar: Okay. But it is included in this. The outlook that we have given.

Manish Grover: The merit one is included in this.

Deepak Poddar: The merit one is included. Okay. And this is similar. What sort of occupancy target are we keeping for FY '26? What kind of revenue will we target in that?

Manish Grover: Sir, the way the Indian government is promoting Ayurveda and is helping Ayurveda, we have

also received a new letter from the Uttar Pradesh government. If any of their patients are admitted in our hospital, the government will pay for their treatment. This letter came just 15 days ago from the Uttar Pradesh government. And in Uttar Pradesh, we have planned 5-6 hospitals. In Meerut, 350 beds. In Lucknow, almost 100 beds. In Kanpur, Agra, Banaras, and in Saharanpur, we have planned many such hospitals. We are also looking for a place for a hospital in Noida.

Deepak Poddar: Correct. So, what occupancy are you targeting in FY '26?

Manish Grover: I am targeting 85%.

Deepak Poddar: So, if there is 85% occupancy, then the revenue will be around INR700, INR800 crores.

Manish Grover: No, no. I have planned only 600. If I add 500 beds, then it will be around 650.

Deepak Poddar: 600, 650.

Manish Grover: So, I will take 35 occupancy for 500 beds. I will take 85 occupancy for old beds.

Deepak Poddar: Fair enough. And the PAT margin should be more than 25%.

Manish Grover: Yes, it will be more than 25%. But the expenses will remain the same. The expenses will not increase. The expenses will not increase.

Deepak Poddar: Absolutely. This is what I wanted to understand. All the very best to you. Thank you so much.

Moderator: Thank you. The next question is from the line of Viraj Mahadevia from Moneygrow India. Please go ahead.

Viraj Mahadevia: Namaskar, Manishji.

Manish Grover: Namaskar.

Viraj Mahadevia: Congratulations. Fantastic numbers.

Manish Grover: Thank you, sir.

Viraj Mahadevia: I wanted to ask you, the 500 beds that you will add for the next 2-3 years, yes. What will be the total capex of each year?

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Manish Grover: Sir, as I told you, the capex that we add for each bed, it will be around INR3 lakh per bed on average. Sometimes it will be INR4 lakh, sometimes it will be INR2 lakh. So, on average, we add one bed for INR3, INR3.25 lakh. For example, if you are adding a hospital of 300 beds, then according to that, it will be INR9 crores.

Nanak Chand: Right.

Manish Grover: I am adding a hospital in Meerut, Chandigarh, R

ajpura, of 300 beds.

Nanak Chand: Yes.

Manish Grover: So, my budget for that is around INR7.75 crores. So, if I add INR3 lakh, it will be INR9 crores.

Viraj Mahadevia: Okay. So, the maximum capex of 500 beds will be INR20 crores.

Manish Grover: Maximum INR15 crores.

Viraj Mahadevia: INR15, INR20 crores. Okay.

Manish Grover: Yes.

Viraj Mahadevia: Okay. Alright.

Manish Grover: And we don't spend money ourselves. Whatever hospital we take, we take it on rent. And we get the money from the owner. We get it from the owner. We only do the interior. The bathroom, toilet, walls, infrastructure, we get it from the owner. For example, I have opened 40 beds in Bangalore, I have opened 24 beds in Ahmedabad, I have opened 16 beds in Vadodara. I have invested money in all of them.

Viraj Mahadevia: Right. And are you running a franchisee outlet or have you winded down?

Manish Grover: We don't give a new franchisee. We have reduced 16 of our old franchisees in the last 6 months. Now we are running the company only.

Viraj Mahadevia: But how many were there earlier?

Manish Grover: Earlier it was around 70, now it is around 50. And this will go to 0 in 50 in the next 2-3 years?

Now there is no hospital franchisee, there is no clinic franchisee.

Nanak Chand: Okay. And this will remain 50.

Manish Grover: There is a challenge in working with a franchisee, sir. We think of the well-being of the patient, he only thinks of money.

Viraj Mahadevia: Right.

Manish Grover: So that's why we have started reducing the franchisee.

Viraj Mahadevia: Will you make it 0 in 50 in the next 3-4 years or not?

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Manish Grover: Absolutely, sir. Sugar in your mouth.

Viraj Mahadevia: Okay. All the best. Thank you.

Manish Grover: Thank you, sir.

Moderator: Thank you. The next question is from the line of Ajay Surya from Niveshaay. Please go ahead.

Ajay Surya: Sir, congratulations for a good set of numbers. Sir, my question was that as you said that your occupancy will increase, so I wanted to ask you that the new patients who are coming, they are coming because the major changes that have been made in the industry regarding insurance, that you have included Ayush, so the patients are coming because of this? And all the new patients who are inflowing, can you give a bifurcation that how many are those patients who are coming to us for insurance coverage and how many patients are coming without insurance?

This was my question. Yes. And what regulatory changes are you seeing towards Ayush that some budgetary commitments of the government are going to increase because of which we can get end benefit? So I wanted to know your opinion on that as well.

Manish Grover: Look, sir, first of all, I won't be able to bifurcate this, but I can tell you that last month, in Dehravasti, Chandigarh, I had a sale of INR3 crores in which I got INR65 lakhs from health insurance. And in Meerut, I had a sale of INR4 crores in which I got INR74 lakhs from health insurance. So last month, I got a total of INR2.5 crores from health insurance. And now, the patient has ignorance of health insurance. We are spreading awareness. So now, the patient comes to us for a disease, to get well.

We give him an extra advantage that do you have health insurance? And naturally, every person is greedy so that he doesn't have to spend money. So, whoever has health insurance, he says that I want to get treated with health insurance. So now, a lot of companies, about 15 companies, have started giving cashless treatment. And other companies are doing reimbursement only now. But the Indian government has a clear mandate to give cashless treatment to all companies. But they have never done it in Ayurveda. For the first time, the body of Ayurveda has been made and started in Ayurveda. So, we are the only players in India who are working properly in this. For this, we need NABH first.

And the second thing, you said, what is the next regulation? So, the Indian government has a clear mandate.

Now, the High Court of Delhi has passed a quarter. The Indian government has ordered that the Ayushman Yojana, which is a INR5 Lakhs Yojana, a Yojana for poor people, should include Ayush in it, should include Ayurveda in it. So, they have given 3 months time. The High Court of Delhi has given 3 months time to the Indian government. So, if that comes, then my occupancy will be 90% this year. If Ayurveda comes in Ayushman. But I am not expecting that. That's why I have claimed 55%.

Ajay Surya: Understood, sir. Understood. Sir, there was one more question. Sir, you said that your bed capacity is around 1300. Sir, I wanted to know that can you give a bifurcation on this? Which are the Panchakarma, or Day Panchakarma, or Shuddhi Clinics, and which are the rest?

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Manish Grover: No, no. These are 1270 proper hospitals. This is not a bed, this is not a Panchakarma bed. This is the bed on which the patient is admitted. Panchakarma beds are different. They are 300-350 different. In every clinic, 20-25, like in the hospital, there are around 10-12 Panchakarma cabins. In the day care center, there are 4-6. They are different. These are 1270 proper beds on which the patient sleeps at night. IPD bed.

Moderator: Thank you. The next question is from the line of Amit Jeswani from Stallion Asset. Please go ahead.

Amit Jeswani: Congratulations, Manishji. I am very happy.

Moderator: Thank you, sir. Namaskar.

Amit Jeswani: Namaskar. All good. All good. Manishji, you are an execution machine. Now, you have executed a new execution within 2 to 2.5 years and you have built 1,300 beds within 2 to 2.5 years.

According to you, how big is the OTC market? Because our margins are extremely high in medicines. And according to you, how big can we make OTC? That is an optionality, of course.

Manish Grover: Sir, in India, 10 crores people take blood pressure pills daily and they don't have any other option other than allopathy medicines. In India, 10 crores people take sugar pills daily. In India, 1.5 crores are kidney patients. Approximately 1.5 crores are cholesterol high patients. Sorry, 1.5 crores are heart patients. 4 crores are cholesterol high patients. So, I have told you about 4 diseases. So, 25 crores patients have 4 diseases. And I am working of these 4 diseases.

In India, approximately 10 crores people have liver fatty. Approximately 1.5 crores people have liver fail, liver fibrosis, liver cirrhosis patients. In India, 5 crores people have depression and anxiety patients. So, the first 5 formulas that I am getting approved in CTRI are BP pill, sugar pill, kidney fail, sorry, kidney medicine, liver medicine and stress medicine. Apart from this, I am bringing a product Rakht Shuddhi which is useful in purifying blood. In all the young blood of India, some people always have bruises, keels, pimples, pimples on legs, pimples on face,

pimples on feet and ladies have pain in this or women have pain during periods. So, Rakht Shuddhi is its product. Apart from this, approximately 12 crores people do not have a clean stomach. For this, they take Kaam Churan, Petsafa or Nityam Jandu tablets. I am bringing such a product for them which will not only clean the stomach but will work on the root of for this, I have hired a team. Right now, only market survey is going on. Right now, on packing. In 3 to 6 months, we will start launching the product one by one. First, we will do it from one state. We will do all the learning and mistakes in one state. When one state is set, we will launch in the other state.

Amit Jeswani: Wow! Whenever I talk to you, Manishji, I get a motivation to do business like you, to do execution like you. You have been...So, OTC is not for your opportunity. How will you work on numbers in margins?

Manish Grover: W

Whatever the cost of my medicines will be the retail price will be 7 times minimum. For example, if I keep it 7 times, then I will give it 3.5 times, then I will give it straight to dealer, distributor and CNF. If I 1.5 times the profit, I will 1 times in the newspaper, social media, TV

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ad. I will keep 1 times the profit from the That is, I will think of 15% profit in the from the and the margin OTC market. And the biggest thing is that its brand value will increase.

Amit Jeswani: Very good, Manishji. Keep rocking. And keep being super well.

Manish Grover: You were quietly listening to our podcast. Your parents had joined. Everyone is listening. What was your parents' experience when they came to our hospital?

Amit Jeswani: Super good, Manishji. Super good. Very good. And, very happy. It is something that everyone should try. Very happy.

Manish Grover: Everyone should come to our hospital once. Someone should be sent to their family.

Amit Jeswani: Your camp is coming. It's a super thing. My parents had also gone for the camp.

Manish Grover: That is in the last week of this month.

Amit Jeswani: And there also, it costs INR50,000 for 3 days. And it's worth it. For anyone and everyone.

Manish Grover: The camp will start on 31st. It will end on 2nd.

Amit Jeswani: Manish ji, your advertisement is still going on at Corn Call.

Manish Grover: I told you that our base is all about mouth publicity.

Amit Jeswani: Super. Keep rocking, Manishji. Thank you. Thank you so much.

Manish Grover: Thank you, sir. Namaskar.

Moderator: Thank you. The next question is from the line of Yash Kukreja from Equitree Capital. Please go ahead.

Yash Kukreja: Namaskar, Acharyaji.

Manish Grover: Namaskar, sir.

Yash Kukreja: Sir, my question is that like our cashless has been approved. Like our average revenue per occupied bed is INR8,000. So, what all is covered in cashless? And what is not covered? Like in allopathy, 90%, 95% claim is passed. So, in cashless, what all is settled in the claim?

Manish Grover: Look, sir. What we have experienced till now is that if the patient's bill is below INR1 lakh, then 95% of the bills are passed. If the bill is above INR1 lakh, if the bill is INR1.5 lakh, if the bill is INR2 lakh, then 80% of the bills are easily passed. Rest, we have to fight with them. Or they ask for more documents.

And the calculation that we have done is that if the patient stays with us for 12 days and pays INR8,000 per day on average, then our bill is less than INR1 lakh for 12 days. So, that's why we suggest patients to stay with us for 7 days, 8 days, 10 days. So, we are getting health insurance claims very easily.

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Yash Kukreja: Okay. Got it. Thank you so much, sir.

Moderator: The next question is from the line of Dhruv Agarwal from Nivisha Investments. Please go ahead.

Dhruv Agarwal: Hello.

Manish Grover: Yes. Namaskar.

Dhruv Agarwal: Namaskar, sir. Sir, as I can see, our presence is more on the north side. So, sir, do you have any plan to expand further on the south side?

Manish Grover: No, sir, we have already planned. We have opened in Bangalore. We have opened in Chennai. We have opened in Hyderabad. We have opened 6, 7 in Maharashtra. We are entering Kerala. We are planning to open in Odisha and Bhubaneswar. Sir, now we are opening in the north, east, west.

We are opening everywhere. Earlier, the north side was old. Now, we have found a place in Nagpur. We have found a place in Goa. Now, we have started two hospitals in Gujarat. We have started in Patna. Now, sir, we are covering the whole idea. There is no problem, sir.

Dhruv Agarwal: Response, sir, on the Bangalore side, on the Hyderabad side, on the Goa side, as you have planned.

Manish Grover: I used to have a clinic in Bangalore. I had never sold more than INR5 lakhs. Now, I have opened a hospital.

It has been 5 months. Last month, my sale was INR22 lakhs. Now, NABH has not come yet. Okay. And INR22 lakhs per month. I had never sold more than INR5 lakhs in Hyderabad when I had a clinic. Now, my last one, I lost INR18 lakhs.

Dhruv Agarwal: Okay.

Manish Grover: I have made two hospitals in Andheri and Thane in Mumbai. In Andheri, my sale was never more than INR20 lakhs. And now, last month, it has gone up to INR45 lakhs. In Thane, my sale was never more than INR16 lakhs, 17 lakhs. Now, last month, it has gone up to INR50 lakhs.

Means, wherever we are converting the clinic into a hospital, our sale is increasing there.

Dhruv Agarwal: Right.

Manish Grover: Because the hospital's FPS increases [58:57], patients.

Dhruv Agarwal: Sir, I have a question.

Manish Grover: Yes.

Dhruv Agarwal: Sir, as you are planning to bring medicine in OTC, so, sir, like last time when you called, you said that your plan is to bring it by March or May. But I think, sir, what is the status there? And sir, whenever we will come, how much revenue can be made and what margins can be made there in the coming time?

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Manish Grover: I have told you about the revenue. I have a plan of 15%. I will take the same profit margin as my manufacturing cost. Rest, I have kept everything in expenses, on TV, newspaper, social media. Earlier, we were going to enter our own market. So, by chance, we found a company in the middle.

We took their 7 products and made them. Already, their market is going on. From next month, their sale will reflect in our company. So, we are transferring funds from them. We took Arshar, a company from Gujarat. We took their 7 products. And we are launching 5 products of our own. And we will launch the clinical trial products in about 6 months.

Dhruv Agarwal: So, sir, the 15% you are talking about, is it the 15% of INR450 crores? Or what is that 15%?

Manish Grover: No. I have told you that whatever will be the costing of the product, it will have 7 times retail price. The formula I have made.

Dhruv Agarwal: Okay.

Manish Grover: From INR350 crores, 50%, 55% will go to distributor, retailer and C&F. And it will go to tax. I will spend the cost of the product on social media, TV and newspaper. So, I will save the profit equal to the cost of the product. So, if I invest 7 times, it will be 15%.

Dhruv Agarwal: Thank you. So, if it is worth INR100, I will save INR15.

Moderator: The next question is from the line of Prateek Chaudhary from Saamarthya Capital. Please go ahead.

Prateek Chaudhary: Hello.

Manish Grover: Namaskar, sir.

Prateek Chaudhary: Namaskar. Sir, you had told me that you sell some packages in such a way that you take the whole years' service money at once.

Manish Grover: I have started it 2, 3 months ago.

Prateek Chaudhary: Okay. And how do you feel about your monthly revenue? How much contribution do you get from such packages?

Manish Grover: I have sold INR1.5 crores like this.

Prateek Chaudhary: Like this?

Manish Grover: I have taken the whole year's money from the patient. I have named it as Shuddhi Sankalp. When the patient was discharged from the hospital, I told him that for the whole year, his medicines, consultations, and everything will be free. He should give me the whole year's money. And in return, I gave 25%, 30% discount to the patient. And for the whole year, I allotted him a doctor and an agent so that if there is any problem, he will talk to me.

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Prateek Chaudhary: Okay. The last sale was 1.5 crores. In this package?

Manish Grover: Yes, it is extra from this package. Earlier, the patient used to leave us for 3, 4 months. So, I named it as Shuddhi Sankalp so that you have to stay connected with us for a year so that you don't need any other assistance. If you need any assistance, we will give it to you.

Prateek Chaudhary: B

ut there is no medicine sales included in this?

Manish Grover: Medicine is included in this. There is no medicine in this, if we collect INR1 lakh or INR50,000 that we have collected, it will be medicine-free for a year.

Prateek Chaudhary: So, how much do you think the percentage of our entire sale will come from this?

Manish Grover: Sir, the patient who is being admitted to the hospital, we are pitching him. So, as the occupancy of my patient's hospital bed increases, the percentage of this will also increase, right?

Prateek Chaudhary: Yes.

Manish Grover: For example, if the patient is admitted to the hospital and left the house, then he follows up for 3, 4 months, comes to take the medicine, but does not return. Then he goes back to allopathy or somewhere else. Now, if we collect the money for a year, then he will stay connected with me for at least a year.

Prateek Chaudhary: Will you book the revenue of this together or will you divide it in 12 months?

Manish Grover: Hold on for a minute. Let me ask him how he booked it. as soon as the treatment goes on, we will book it.

Prateek Chaudhary: Okay. So, you will not book it together, right?

Manish Grover: No. If there is a direct impact, then it will reduce in the next few months.

Prateek Chaudhary: So, one more question. Since our organization has become so big, so at the senior level or mid-level, what type of hiring are we doing? And the ENY [64:17] that you are appointing, will it help us to build our middle and senior level organization?

Manish Grover: Yes. We have talked to ENY completely. We have just come for the management performance review, but we have also consulted them. We have also talked to a consultation firm in Delhi.

As soon as we need recruitment, we are bringing in proper professional people to the company.

People whose business is similar to ours or whose thoughts are similar to ours. For example, the head I have hired for the OTC market, I have hired the head the biggest brand maker in India, I have hired the head of that company. He is Mr. Tiwari. So, the name you heard Dr. Ortho. I have hired him from that company.

Rohan P. Parikh: Which Dr. Ortho?

Manish Grover: Dr. Ortho, Pet Safa, Roop Mantra, Kesh King, Divisa Herbal. He used to work in that company as a marketing head I have hired him in my company. Dr. Ishu Arora, he was in Jeeva, Dr.

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Abhishek he has come from Nirav Street. I have hired a lot of people Properly professional people.

Prateek Chaudhary: Yes. Okay sir. All the best for your future.

Manish Grover: Thank you, sir.

Prateek Chaudhary: I am very happy that you have reached this level. Thank you so much.

Moderator: Thank you. The next question is from the line of Akshit Agarwal from Akshit Agarwal HUF.

Please go ahead.

Akshit Agarwal: Namaste [Acharyaji 65:56], Is there anything about Going to the main board? Is there anything about?

Manish Grover: Yes, this year in 2025 It will come.

Akshit Agarwal: Okay Is it after March?

Manish Grover: I think, it will come in April because we will complete 3 years in April.

Akshit Agarwal: Okay

Manish Grover: Our IPO came on 19th April.

Akshit Agarwal: So we will complete 3 years in April.

Manish Grover: Yes sir, as you have taken 7 products from Gujarat. Do you have any takeover plan to buy a company?

Manish Grover: Right now. We are having meetings in many places, we are also looking at factories, we are also looking at products. In India and outside India, we are talking about many tie-ups. But that is on the first and second level. We will take over or we will pick up a product or we will launch it ourselves. We have to do it. We are working day and night, we have only one mission to make this company No.1.

Akshit Agarwal: Yes sir. Is there any future bonus or anything else?

Manish Grover: I think we did dividend yesterday. How much percent did you take 15% dividend...

15%

dividend has been announced.

Akshit Agarwal: Thank you.

Manish Grover: I don't have technical knowledge of that.
Akshit Agarwal: Okay. Thank you.
Manish Grover: Thank you, sir.
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Moderator: Thank you. The next question is from the line of Ajay Surya from Niveshaay. Please go ahead.
Ajay Surya: Thank you, sir. The question was as we see many players like, Kerala, Ayurveda or especially Ravi Shankar or Osho. They run many such activities. So sir is there any competition? Are we different from them? How do you Differentiate because, nowadays any patient's trust is First on Ayurveda and then on Jeena Seekho. So what activities are we doing, so that they come to us first. And why won't they go them first?
Manish Grover: We are promoting the patient first. Whatever promotion activity we do, we interview the patient Only after listening to the patient the patient comes. And we have changed our marketing strategy since the last 7 months Because of which our sale has increased. Earlier I used to call the patient, Now I make my videos informative I told you, I am focusing on medical education. Just now 20 days ago, we announced a medical academy where we will prepare doctors and nurses. And it has been tied up in Jeena Seekho and it has been tied up in our 3 colleges Where nurses and doctors, we will get the first right to recruit.
So our plan is in 3 colleges and 3 nursing colleges. And the second thing, as you said about other people, so none of them are NABH and no one is working on the hospital model, and no one in India is working on wellness and not on treatment. We are exclusively working on treatment.
Ajay Surya: Understood. Sir, but as I see In NABH hospital, our share is 40%. So sir, don't you think That the government won't let a player grow up or is there a regulatory issue till now?
Manish Grover: There is no such issue and it is not in our notice. And no matter how hard we try, we won't be able to reach even one hospital. So where is their Billing of crores of rupees, we are nothing. So I don't think there will be any issue.
Ajay Surya: Understood sir.
Manish Grover: Because last month, Our total government billing is of INR6 crores of India, Whereas Medanta has a billing of INR10 crores-INR20 crores of one hospital.
Ajay Surya: Understood sir. Sir, One last question, In our books trade receivable amount has increased, Last year from INR22 crores to INR42 crores. So sir, Will you give a clarification on that?
Manish Grover: It will take a minute to Nanakji. Nanakji will tell you.
Nanak Chand: Sir, our trade receivables have increased, the reason is our revenue has increased from 60%. So our revenue has increased in proportion to 42-46 days. The reason behind this is the government Billing Receivable cycle is 3-6 months. So because of that it has increased.
Ajay Surya: How long will it take for us to recover from? 3 months?
Nanak Chand: Yes. Definitely.
Ajay Surya: Thank you sir. All the best for the future.
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Moderator: Thank you. The next question is from the line of Dhruv Agarwal from Nivisha Investments. Please go ahead.
Dhruv Agarwal: I have two questions. Like you were saying that we have 1370 beds, right? So, Acharya ji, what is there in Shuddhi Panchakarma Day Care? And in Shuddhi Clinic, how many beds do we have? And how many beds are there in the franchisee model? Can you give us some bifurcation on this, sir?
Manish Grover: Sir, there is no hospital in the franchisee model, right? All the hospitals are company-owned.
Dhruv Agarwal: No, there is no such thing, sir. Because in the previous presentation, you had mentioned that we have about 50-60 beds No, no.
Manish Grover: There is no hospital in the franchisee model.
Dhruv Agarwal: There is no hospital, sir. Clinic. Shuddhi Clinic.
Manish Grover: In Shuddhi Clinic,
how many franchisees are there in 70 clinics? Out of 70, 55 are franchisees.
Dhruv Agarwal: Okay.
Manish Grover: The rest are owned. The rest 20 day care centers are also owned. And the 35 hospitals are also owned.
Dhruv Agarwal: And like you were telling your vision, sir, that you have to phase out the franchisee model a little. So, sir, if this is phased out, then where will you get your sales from? Because they take commission and they are interested in commission so they also want to promote medicines if you give them medicines, they will get commission so if this fails, don't you think medicine sales

will decrease?

Manish Grover: In our business model, franchisee doesn't sell anything franchisee is just a manager all the marketing is done by the company. Franchisee is just a manager, they don't put any effort to grow the business. That's why we have reduced the number of franchisees in the last 6 months. Because franchisee thinks I have invested 25 lakhs, I want 75,000.

Now I have a payment in my account, why should I invest 25 lakhs why should I invest 25 lakhs, I have a 25000-30000 manager, he handles it better. His money increases by 3% whereas my money is 6% in the bank. That's why we decided to reduce the number of franchisees. If they give me 15 lakhs I have a total of INR7.5 crores, INR7.5 crores 3% of means I am giving them 25 lakhs-30 lakhs whereas I am getting 5 lakhs from the bank. So I have a loss of 17 lakhs. So I returned their money if they work, we are happy. But most of the franchisees used to be distributors when we used to have a COD model. Now we are reducing the COD model we are converting ourselves into a daycare and hospital. We don't have to deliver medicines at home we have to meet the patients, we have to talk to them one-to-one whereas 2 years ago.

We used to send medicines to the patients at home in this model, our ticket size has increased revenue and earnings have increased in that model, there was a turnover of INR134 crores in Jeena Sikho Lifecare Limited

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2021 my profit was only 8% now when I changed the model, my profit is 21%. So continuing the wrong model is also wrong.

Dhruv Agarwal: How many panchakarma day centers are there?

Manish Grover: We have 20 day centers and we have 35 hospitals out of which 4 are big and 28 are small.

Dhruv Agarwal: So what will be your focus in the future? are you going to increase it to 25 or 20 centers?

Manish Grover: No, I will convert at least 15 clinics into hospitals this year. And I will take the franchisees to me and make it a hospital. My future plan is not to keep clinics I will make all hospitals. I will take care of the patients and they will be admitted I will make it an NIVH. I will take care of the government business I will take care of private business and I will run my clinic there I will open 3 or 4 hospitals. In Mumbai I will open 3 or 4 hospitals in Mumbai. We have 14 centers in Delhi and 11 are on the CGHS panel CAPF and CGHS and 11 are on the CGHS panel. So I will open 4 or 5 hospitals in Mumbai.

Dhruv Agarwal: One last question if we look at the other expense in FY '23 and FY '24, there is a drastic increase so what are the major components that are not allowing the margins to increase has there been any major margin increase in FY '23?

Manish Grover: There was a profit of INR204 crores in FY '23 margin was 16% let me check what is in FY '21 let me check what is in FY '21...

Dhruv Agarwal: If we reduce other expense it will be good for the market.

Manish Grover: I have made a team of three people to centralize India's purchase. I will make a report in 3-4 days on the expenses for the next year. I have made a committee to reduce expenses and increase profits. We are working internally. If we reduce 3% per month, we will increase INR1 crores profit. We are centralizing the purchase. In our hospitals, fruits, vegetables, and cooking expenses are purchased e

verywhere.

We have outsourced food to patients. We will compare the benefits of outsourcing and managing the kitchen. We are evaluating the benefits of outsourcing and managing the kitchen. In our hospitals, other than the patients, all the attendants, all the relatives have free food. It is costly, but we consider it as a service. We are helping people.

Moderator: Thank you as there are no further questions. I would now like to hand the conference over to the management for closing comments, over to you sir. Mr. Manish Grover, Mr. Nanak Chand if you have any closing comments you may go ahead.

Manish Grover: Thank you for listening to me. We will take this company forward. I would like to request all of you to visit our hospitals, meet our patients, check our services, use our medicines, and use our products. Thank you. Thank you Ranveer ji. Thank you Nuvama Capital.

Moderator: Thank you. On behalf of Nuvama Wealth, that concludes this conference. Thank you all for joining us. You may now disconnect your lines.

Call: Nov 2024

Date: November 18, 2024

To,

The Manager

Listing Compliance Department

National Stock Exchange of India Limited

Exchange Plaza, Bandra Kurla Complex,

Bandra (East), Mumbai -400051

SYMBOL: JSLL

ISIN: INE0J5801011

Earnings Call Transcripts

Pursuant to Regulation 30 and 46(2)(a) of the Securities and Exchange Board of India (Listing Obligations and Disclosure Requirements) Regulations, 2015, as amended, the transcript of the audio call recording of the Company's Analyst /Investor conference call held on November 13, 2024 on the unaudited Standalone Financial Results of the Company for the half year ended September 30, 2024 is attached herewith.

The transcript of recording can also be accessed on the Company's website using the following link :
<https://jeenasikho.com/wp-content/uploads/2024/11/JeenaSikho-H1FY25-Earning-Call-Transcript.pdf>

You are requested to kindly take the above on your records

Thank you,

Yours faithfully ,

For JEENA SIKHO LIFECARE LIMITED

Manish Grover

Managing Director

DIN: 0755788

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"Jeena Sikho Lifecare Limited

H1 FY25 Earnings Conference Call "

November 13 , 202 4

MANAGEMENT : MR. MANISH GROVER -- MANAGING DIRECTOR -
JEENA SIKHO LIFECARE LIMITED

MR. NANAK CHAND - CHIEF FINANCIAL OFFICER -
JEENA SIKHO LIFECARE LIMITED

MODERATOR : MR. RANVIR SINGH -- NUVAMA WEALTH

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Moderator: Ladies and gentlemen, good day and welcome to the Jeena Sikho Lifecare Limited H1 FY25 earnings conference call. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone.

Please note that this conference is being recorded. I now hand the conference over to Mr. Ranvir Singh from Nu vama Wealth. Thank you and over to you, sir.

Ranvir Singh: Thank you, moderator. On behalf of Jeena Sikho Lifecare Limited, I extend a very warm

welcome to all participants on H1 FY25 financial results discussion call. Today on our call, we have Mr. Manish Groverji, Managing Director, and Mr. Nanak Chandji, the CFO.

Before beginning with this call, I would like to give a short disclaimer. This call may contain some of the forward-looking statements which are completely based upon the management's needs, opinions, and expectations as of today. These statements are not a guarantee of the company's future performance and involve unforeseen risk and uncertainty.

With this, I would have to hand over the call to Mr. Manish for his opening remarks. Over to you, sir. Thank you, Mr. Groverji.

Manish Grover: Namaste to all. My name is Acha rya Manish. I am happy to share from Jeena Sikho with all of you and I am happy to share my experience of the financial year 2025 with you all. I am very happy that we have been able to benefit from the Products and our services. Our revenue has been increased in both segments.

So, for '24 and '25, I will tell you that when H1 '24 had a turnover of INR157 crores and in H125, we have a turnover of INR214 crores in which we have registered a 36% annual growth. And we have registered a 28% annual growth in the growth we have taken. We have benefited from the economic attention we have given to the expansion of our hospitals, clinics and day care. This year, our revenue from the service has grown by 72% annually. And this is a very good growth. Our people who are connected, who are providing the service, we have a good result. Now, the service that has come from our customer service, this is 53% of the entire payment. Last year it was 42% and this year it is 53%. Last year we were managing 1,277 beds by 31st March. Now we are managing 1530 beds.

Our development has reached 100 cities and cities in 21 states of the country. And our customers have received very hopeful results from the new rise of 32% yearly EBITDA and 47% of the annual PAT's which makes our profit margin strong. Our confidence and our strength to expand will be explained by me after Nanakji statement about our books, %age, sales and profit hence I handover the call over to him.

Nanak Chand: Good afternoon everyone. Thank you Manish sir. As mentioned by Mr. Manish sir, we have achieved the healthy financial performance during the H1 FY25. Here are some key highlights.

We generated revenue from operations and grew by 36 % on year-on-year basis to INR214 crores, mainly driven by the hospital services, which has grown by 72 %, Y-o-Y to INR114 crores.

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While product sales grew by 10 % to INR100 crores, the EBITDA during the H1 FY25 grew by 32%, Y-o-Y basis to INR59.28 crores by implying the EBITDA margin 28 %. The H1 FY25 PAT grew by 47 % Y-o-Y basis grew to INR46.88 crores. We retain a healthy balance sheet with net cash and equivalent of INR69 crores by end of the H1 FY25. Our ex-cash ROCE stood 82% respectively. And we spent the INR16.72 crores in capex in H1 FY25. Thank you so much.

Moderator: Sir, should we open the floor for questions?

Nanak Chand: Sir, you can open floor for questions.

Moderator: Thank you very much. We will now begin the question and answer session. Our first question comes from Sagar Jain, Individual Investor. Please go ahead.

Sagar Jain: Hi, thanks for giving me the opportunity. Congratulations, Manish and Nanak for posting the very good results. However, I just have one question regarding the expansion that you are expanding to the 21 states of India and you have already crossed more than 100 towns of India. So I just want to know that how this revenue is being generated majorly from the few hospitals or is evenly distributed, how it is coming from few locations of India or it is coming from every part of the 100 hospitals or maybe hospitals in 100 towns is coming from? So, how it is distributed?

Manish Grover: I will tell you from which area it is coming. Our total sale of '20-'21 was INR135 crores. And '21-22 total sales were INR146 crores in which medicine sales were around INR131 crores and panchkarma sales were INR8 crores and government panel sales were INR6 crores. In 22-'23 total sales were INR204 crores, medicine sales were INR141 crores, panchkarma of INR8 crores were increased to INR38 crores, government panel sales of INR6 crores were increased to INR24 crores.

Now talking about 23-24 full year, medicine sales of INR140 were increased to INR185 crores, panchkarma services in hospital from INR38 crores went to INR86 crores and government panel sale of INR24 crores were increased to INR52 crores. So, the total sale of 6 months before 2023-2024 was INR157 crores, which is INR214 crores this time. The medicine sale was INR90 crores, which is INR99 crores. The hospital sale was INR45 crores, which is INR62 crores. And the government sale was INR22 crores, which is INR52 crores.

Now, our major hospitals are in Lucknow, Mumbai, Chandigarh, Gujarat and Meerut. And after this, the list of the expansion of the new hospitals, I will tell you the list. Now, the new hospitals

that we have opened, we have started 3 in Gujarat, Vadodara, Ahmedabad and Surat. Apart from this, Chennai, Bangalore, Assam, we have covered every area of India. That is, we do not care about any winter or summer. We are getting hospitals from every area.

Now, the hospitals that are starting in the next 6 months, I will read the list once. 25 beds in Nagpur, 40 beds in Gorakhpur, 40 beds in Guwahati, Assam, 25 beds in Goa, 60 beds in Bhopal, Madhya Pradesh, 35 beds in Siliguri, West Bengal, 27 beds in Ranchi, Jharkhand, 25 beds in Bhubaneswar, Odisha, 25 beds in Jammu Kashmir, 25 beds in Jabalpur, 40 beds in Raipur, 25 beds in Cuttack, 27 beds in Hyderabad, 25 beds in Pune, 40 beds in Prayagraj, 30 beds in Alwar, Jeena Sikho Lifecare Limited

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30 beds in Sangha, Rajasthan, and two of 20: 20 beds in Delhi. We are adding 600 beds right now. These are small hospitals.

Apart from this, we are looking for a big one like we are looking for a new one in Chennai and Bangalore. So, as soon as we get a big one, it will be different. So, in total, we have 1570 operational beds, 1530, in which the occupancy is 51% in the 6-month result, which was 38% last year.

And the average revenue per bed was 7900 earlier, now it is 8100. So, this revenue is mixed up everywhere. I mean, there is no special problem because we are in the hospital line, in the patient area, in the sugar area, in the kidney failure area, in the BP failure area, in the cancer area. So, our sale is distributed everywhere.

Sagar Jain: Yes, sir. Thank you, sir. Even I am also a diabetic patient, so I definitely wish to...

Manish Grover: You can contact us whenever you want, we will reverse your diabetes and help you completely.

Sagar Jain: Absolutely, sir. Thanks, sir. I can see you and team are very aggressively helping your business.

Manish Grover: Thank you. Thank you very much.

Moderator: Thank you. The next question comes from Sumit G

upta from Centrum. Please go ahead.

Sumit Gupta: Hi, thank you for the opportunity, sir. I have two questions. First is on the hospital side. So, the kind of extension that you are going to do, just want to understand on the doctor cost point. So, how many doctors do you plan to increase and how much will the cost increase overall?

Manish Grover: Sir, our cost increases very less when we open any hospital. So, our 100-bed hospital, we open a hospital for INR2.5 to INR3.5 lakhs per bed because we take all the buildings on rent. We don't open our own hospital. I mean, we don't buy it. And when the year ended in 2022-23, we had a total of 230 doctors including Ayurveda, homeopathy, naturopathy, and allopathy. When the financial year ended in 2023-24, we had 307 doctors.

Now, we have given the results of 6 months. So, our number of doctors is 411. Apart from this, I have 1010 doctors in waiting who want to work in our company. But as and when our requirement arises, we will hire them. Because in our company, in the whole of India, the best salaries, the best incentive plan, plus ESOP plan is in our company. So, that's why all the good doctors in India want to join our company.

And our attrition rate is the lowest in the whole of India. I mean, our doctors don't leave us. If we remove someone, then remove. Otherwise, we don't get any problem quickly. So, right now, our number of doctors in 6 months is 411. And the availability of doctors is very easy because we have Ayurvedic doctors. And the degree in Ayurveda is BMS. So, we get BMS doctors very easily because right now, they work in allopathy due to compulsion because there is no option in Ayurveda. But since our company has come, I have told you that I have received the resume of 1010 people who want to work with us.

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Sumit Gupta: Understood, sir. And sir, going forward, like I am seeing from FY23, your government revenue as percentage of overall revenue it has increased from 12% to 24%, which is double. So, going forward also, what kind of government empanelment?

Manish Grover: No, no. It didn't double, sir. In the revenue of 2022-23, the sale of the government was INR24 crores at INR203 crores. So, the government revenue was 12%. And the sale of INR324 crores at INR23-24 was only INR51 crores. So, it was almost 16%. Now, if I talk about the result of 6 months, then in 6 months, our government revenue was only 24%. The rest is private, from health insurance and cash.

Sumit Gupta: So, how can we think further?

Manish Grover: We will increase the private also. We are dependent on the government. We take the government as a side help. Because it has become very good now. From April 1, 2024, all health insurances have become cashless in Ayurveda, so we are getting more health insurance patients. We

consider the government as our extra income.

Sumit Gupta: Sir, how big is your medicine business? And how much is your share?

Manish Grover: Sir, I don't know about the share. I don't even know how to calculate it. I know that our medicine sale is only available in our clinics and hospitals. Our 6-month medicine sale is of INR100 crores. Last year, it was INR90 crores for 6 months. But now we are exploring a new market, OTC. 7 clinical trials have been completed by us; blood pressure pill, diabetes pill, kidney medicine, liver medicine, apart from this, migraine, constipation, and hair. These clinical trials have been completed. And we have started medicines in our clinic. And in the last 6 months, we have successfully launched 2 products and tested them. We launched a product for piles and a product for hair. Our team is ready. Within 2-3 months, we will enter the OTC market. We will launch medicines for blood pressure, diabetes, kidney medicine, and liver medicine in 2-3 months. It is a huge market. We have zero presence

in it. We are not in the OTC

market. Our medicines are so I'd on our medical prescription. Our doctor prescribes them.

Sumit Gupta: So, after 4-5 years, what kind of mix do you see, hospital/clinic versus medicine in overall revenue?

Manish Grover: Sir, this is a new venture. A new line is starting. We have 1500 beds. By 31st March, I plan to have 2100 beds and by next year, I will have 3000 beds. So, in the next 3-5 years, I plan to have 5000 beds. First, I have a target of 5000 beds.

After 5000 beds, I will buy a big hospital for 10,000 beds. Because then what will I do? You might know that 4 days ago, the Supreme Court ordered the Indian government to include Ayurveda in the Ayushman Yojana. So, if you have seen the news 4 days ago, the Supreme Court has ordered the government and state government to include Ayurveda in the Ayushman Yojana and they have given 3 weeks to the government. If that happens, then I will expand the number of beds very quickly. Because in India, we do not have the highest number of NABH beds. We have a total of 33 NABH beds. And I will have 4 NABH beds by next month. So, no one in India has this much NABH.

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Last year, we had 24 NABH beds. Now, in 6 months, we have increased 9 NABH beds. So, without NABH, we will not get the government panel. And now, 2 state governments only give cashless treatment. One is the Rajasthan government and the other is the Delhi State government and the central government gives and the army gives and 7 forces give. Now, the Supreme Court has ordered all the state governments to do the same. So, this will be a very big story. This is the story of the Sky is the Limit because people get the solution in Ayurveda without any hassle.

Sumit Gupta: So, sir, if I compare it with other hospitals, like I know you. The whole allopathy hospital market is different and your market is different. So, in that, how many percentage of patients come to you who first went to those hospitals and then came to you for Ayurvedic treatment?

Manish Grover: Sir, 80% patients come to us after being disappointed. I mean, they have come from there. The 80% patients we have, they did not get the solution there. Either they were told chemotherapy or radiotherapy or surgery, if it is a cancer patient. If it is a kidney failure, then they were told dialysis or transplant. After that, they come to us. I mean, we are such people who treat the body in healing -- because we are the only integrated in India. There is no one yet. We use the diagnosis of allopathy. We use Ayurvedic treatment.

Sumit Gupta: Okay sir, understood.

Manish Grover: And now we have published 19 research papers on kidney failure, liver, cancer. In India, no research paper was working on clinical trials. So now I have published 19 papers. And almost 60 papers are in my pipeline. I have set up a whole team for research which only understands the language of the world and is preparing the data in that language.

Sumit Gupta: Okay sir, understood

Manish Grover: Because these people ask for clinical trials and research.

Sumit Gupta: Yes sir. Thank you.

Moderator: The next question comes from Chirag from White Pine. Please go ahead. Chirag, your line is unmuted. Please proceed with your question.

Chirag: Namaskar Acharya ji, how are you?

Moderator: Namaskar Chirag ji, how are you?

Chirag: Very good. Acharya ji, there were 2-3 basic questions. How many types of insurance companies are there at the Pan-India level and how many are left? How many insurance companies are still pending for cashless?

Manish Grover: Sir, the Indian government has notified on 1st April 2024 that all companies have to be cashless. But practically the companies are not accepting it yet. So just 4 days ago, we had an agreement with GIPSA, all our hospitals, NABH. GIPSA's National Insura

nce, Oriental Insurance ; all 4 are government companies. So we had a tie-up with GIPSA 4 days ago. GIPSA is a mediator, a provider which looks at government companies.
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Chirag: Okay.

Manish Grover: Government companies don't directly tie -up. They do it through GIPSA.

Chirag: Okay. So that is done.

Manish Grover: So we had a tie -up with GIPSA for 4 days.

Chirag: That is done. So now it is effective.

Manish Grover: And in private companies, Niva Bupa , Care , ICICI, IFFCO -Tokio . SBI, Cholamandalam . We have done this with about 11 companies. Now 16 companies are pending. From which cashless did not happen. Reimbursement has started. Means all the companies have started giving reimbursement.

Chirag: Sir, I wanted to know in cashless. Because this is a big trigger. 16 companies are still pending.

Sir, how is this process?

Manish Grover: On the 10th day, one or two companies are joining. Like we have joined 11 companies in 6 -7 months. And now we have joined 4 government companies. The major big companies have joined. Like Niva Bupa , Care , ICICI, HDFC. These have become cashless.

Chirag: Okay, big companies have happened.

Manish Grover: Big companies have mostly happened.

Chirag: Okay. Sir, this is a pan -India type ...

Manish Grover: Whatever is happening is of all the hospitals. My NABH is of all the hospitals. And one more thing. Even if my NABH is not a hospital. The file for reimbursement is also being approved there.

Chirag: Sir, reimbursement is happening. That is there.

Manish Grover: Sir, the patient is also getting reimbursement. The patient is happy with it. The patient is getting money back. This is a big thing. Because in Ayurveda, it was not available before. The patient is happy with this. That at least in 30 -40 days, it will come back.

Chirag: Sir, in today's date, the hospital revenue. How much cashless will it be? This is an estimate. This is not an exact number. This is just broadly. If the hospital revenue is INR100. Is INR80 being cashless?

Manish Grover: Like I give you an example. In the last month, my Dera Bassi sale was INR2.5 crores. In the sale of INR2.5 crores, INR80 lakh was of health insurance. So you calculate the average percentage. It is almost 30%.

Chirag: It was cashless. No, no.

Manish Grover: It was total. Including cashless reimbursement.

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Chirag: Okay, okay.

Manish Grover: Meaning around INR80 lakh from total insurance.

Chirag: And how much cashless will it be? It will not be much.

Manish Grover: Sir, I do not have that data. You do one thing. Either give me the mail. I will reply to you. I do not have such exact data now. I will make it.

Chirag: Because this is a big opportunity. As soon as this happened.

Manish Grover: We started the company 20 years ago. We were listed 2.5 years ago. In front of you, you have been involved in growth from the beginning. Exactly. Look at the things in front of you.

Approved in front of you. The number of hospitals has also increased. The number of NABH has also increased. The government sale has also increased.

The footfall of the patient is increasing. Now I will tell you the number of patients. Which in 2022 -23. 5700 patients were admitted in the whole year. 13,000 patients were admitted in 2023 - 2024. Now 12,000 have been admitted in 6 months. How much we did in the whole year. This time in 6 months, we have admitted that many patients.

If you talk about OPD. Patients who have come to meet. In 2022 -23, there were 1,57,000. In 2023 -24, there were 2,50,000. Now in 2024 -25, I have admitted 1,57,000 in 6 months. What was done in the whole year in 2022 -23. I did it in 6 months of 2024 -25.

Chirag: Right. Sir, this is true. Sir, the number we told in the utilization. Where can it go p

ractically?

Manish Grover: Sir, the day the Ayushman Yojana comes. It will go around 80%-90%.

Chirag: What is the issue in the Ayushman Yojana? Because we have been talking for almost a year. Ayushman is the intention of the government.

Manish Grover: Even if Ayushman does not come, it will come further. It came in the army. ECHS has come till then. Apart from this, BSF, CIPF, CRPF. NSD Commando, Assam Rifle, ITBP. And it has come in all this. And now the Punjab government. The UP government. And the Madhya Pradesh government. And the Punjab and Haryana government. Reimbursement has also started in these four.

Chirag: Okay, it has happened.

Manish Grover: Yes, all this work is going on. Every month, every two months, some good news is coming.

Chirag: And sir, when this reimbursement comes from the government, then it will keep coming from the government.

Manish Grover: No, we will not get the reimbursement. The patient gets it. The patient pays us in cash. The patient gets money back. Like the tie-up of the Punjab police with us. All the government officials of Punjab got it, Haryana got it. So we get cash for all this money.

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Chirag: Yes, I mean, how is your reach level here? You have to see that immediately.

Manish Grover: No, no, we don't have any loan. We don't have any loan. And the companies that have a cashless insurance policy, their money also comes in 10 days.

Chirag: Cashless money comes in 10 days.

Manish Grover: Absolutely, cashless comes in 10 days.

Chirag: Okay, and sir, we are going to launch this OTC. What are you thinking initially? How are you thinking? Because in this, you will also have to spend on marketing, you will also have to invest initially. So would you like to go state-by-state or all India type launch together?

Manish Grover: No, no, sir, we will go state by state. If we do all India together, it will be foolish. We will do all the mistakes in one state first by taking training in one state. Our team has done all the homework. We have been doing it in two states Punjab and UP. And a small part in Gujarat and Maharashtra. We have done all the trials with two products.

Chirag: Okay.

Manish Grover: We have done all the homework and prepared the whole team, kept the boys, kept the marketing heads, kept the marketing manager. We have been doing the homework for the last 6 months.

We had reached the sale of INR27 lakhs per month.

Chirag: Okay, you have reached. Great. So when will you do the actual launch?

Manish Grover: I will do the actual launch within 3 months. Some approvals are left because we have taken approval from ICMR. We are going to the clinical trial of Indian governments. So we are launching clinical trial products.

Chirag: Right.

Manish Grover: It takes time to get the name. The names that I have asked are objectionable. Like I have named it Dr. BP, so I have got it. I have got the names of three. Actually, if I launch one product, how much will be the expense? If I launch five products, the expense of my marketing time will be the same. That's why I am waiting for the approval of 5-6 products.

Chirag: So for the approval of ICMR, you have to get the satisfaction of the research paper.

Manish Grover: Everything has been completed. Research has been done, clinical trials have been done. We just have to get the file.

Chirag: Till when will you get the names?

Manish Grover: We have got the names. We have kept the names. But we have got 3 names from the government. We are waiting for the approval of 3 names.

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Chirag: Sir, the initial guidance that you gave, are you maintaining it or given the way H1 has played out, are you trying to improve it or the initial number is your focus?

Manish Grover: No. The number that I had committed

last year, is that the number? Do you want to increase it?

Chirag: Maybe you don't want to...

Manish Grover: No, sir. We will increase it. We are doing our best.

Moderator: The next question comes from Yash from Stallion Asset.

Yash: Hi, Manish Ji. Thank you again for the opportunity. Manish Ji, your acquisition strategy, in October, I think you bought Oregon Life Company in INR70 crores cash. Can you tell us about your acquisition strategy? What are your plans for the next 2-3 years and how can you scale in organic?

Manish Grover: Sir, my acquisition is being discussed in 3-4 places. We are doing acquisition planning from there so that our business gets benefit. Along with that, we are working on our growth. We are

doing our personal growth. My acquisition is being discussed in 3 -4 places. But nowadays, people ask for multiple so we are in negotiations. We are going for our products so that our business gets benefit. Clinics, call center setup, or hospitals. We have visited 5 -6 hospitals in the last 6 months. But we are not doing acquisition. We are doing revenue sharing model with them. We are not giving them money. We will run their running hospital. We will share the revenue of the collection. We are working on this model. We are not giving money. We will give some money and share the revenue. We will get management rights and payment receiving rights. We are working on this model. For acquisition, we are talking to 2 factories and 3 -4 other places which are connected to our business. For example, Oregano Life. There are other businesses with turnover of INR100 crores or PAT of INR 12-INR13 crores. There are 2 -3 such businesses. It is going on. We are trying to get multiple of 5 -6-7. We are also expanding in Dubai. The setup in Nepal has started. Dubai will also be in agreement within this month. We will open 6 new setups in Dubai within 1 year. We are also talking about acquisition in Dubai.

Moderator: The next question comes Prerana from Shome Partnership Limited . Please go ahead.

Prerana: I have 2 questions. First, you said that occupancy will be 50% and 500 beds will be added. We have made 50% occupancy in H1. Can this occupancy increase further?

Manish Grover: Thank you ma'am. It will increase for sure .

Prerana: What is the target?

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Manish Grover: The number of patients who are recovering from kidney failure, liver failure, or cancer. They are the ones sending more patients .

Prerana: Okay. My question is can we target 50 %-70% occupancy in this year?

Manish Grover: Yes. Our target is 80%. We hope that it will increase. But beds are also increasing. Otherwise, if I don't increase beds, then my occupancy is 65%. Last year, it was 38%. Now, it is 65%. Every month, beds are also increasing. If I reduce the number of beds, then it will reach 70%. Actually, there are 1530 beds in total. 775 are occupied. So, 40 beds were added 3 days ago.

On 28th September. But, if you want to give percentage wise results, then I have a problem. 120 beds were added in September. If I reduce it, then occupancy will increase by 60%. So, beds are increasing every month. So, by the end of this year, we will have 2100 -2200 beds.

Prerana: Sir, I have one more question. Our target was 500 beds. We have already increased it to 250 beds.

Manish Grover: No, we have increased it to 530 beds. This time, we have increased it to 250 beds. We will have 518-521 beds.

Prerana: Okay. So, there will be a total of 500 beds in this year.

Manish Grover: No, there will be a total of 700 beds. There will be a total of 725 beds. I had said 500 -600 in the last con-call. If we get a bigger hospital, then we will acquire that too.

Prerana: Okay. Sir, what is the volume growth in our medicine business? H1 vs H1.

Manish Grover: It was INR90 crores. Now, it is INR

100 crores.

Prerana: No, sir. I mean volume growth. We used to give volume growth in PPT.

Manish Grover: I don't know. Nanak Ji will tell you. I don't know what volume growth is.

Nanak Chand: Volume growth in medicine has increased by 10%. And in the service sectors, it has increased by 42 %-53%. In which way ? Volume?

Prerana: Okay. Sir, this is the last question. Our guidance is to make a revenue of INR450 crores and a 25% PAT margin. Do you want to increase it? Because it seems that you will do more than this.

Manish Grover: This year, this is my target.

Prerana: Okay. This year, this is my target.

Manish Grover: After 31st March, I will come with a new figure. Because I am working on the planning.

Prerana: Okay. Sir, I have only one request. When doing con -calls, you can talk in Hindi, but the numbers that you say, please speak in English. I don't understand INR1.5-INR2.5 crores. That's it. All the best, sir.

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Moderator: Okay. Thank you.

Prerana: Thank you. The next question comes from K Rajesh, an Individual Investor.

K Rajesh: Now, sir, I think, as per the numbers, we are going as per the guidance of INR450 crores. Can

we expect INR1000 crores revenue in FY26, sir? As per the bed occupancy, it is also increasing.

Manish Grover: In FY26, my target is INR650-INR700 crores. I want to take INR1000 crores till FY28. Because in Ayurveda, we have to increase things step by step. I have planned a path from here 5 times within 3 -5 years.

K Rajesh: Sir, what about the main board plan? Are they going or not?

Manish Grover: The main board will come this year. We will come to the main board in FY25. We will complete 3 years in March. So, we will apply in March and by June, we will come to the main board.

K Rajesh: Sir, what about our global expansion?

Manish Grover: We have also assigned E&Y for management review. And our signing as a state auditor will also come to the main board. Now, they are associated with us for financial and management review.

K Rajesh: Sir, what about our global expansion? It is going to open in Dubai.

Manish Grover: Dubai, Vietnam, Mauritius, London, America, Australia, New Zealand. I am talking about 7 -8 countries. We have got the norms studied there. So, we are going to open a day care center everywhere. Now, we are starting in Saudi Arabia and Dubai. I have talked about Turkey from there. But all these things will take 6 months to 1 year. Because the rules, regulations, approvals are all time-taking.

K Rajesh: Sir, what about people's attraction to Ayurveda?

Manish Grover: Sir, people believe in therapies. And there is a good news. I have been in Dubai for the last 1.5 years. Now, I have planned that 6 months ago, the Dubai government has implemented all health insurance alternatives. So, all the people who have health insurance have started getting reimbursed. Now, the government is also starting cashless. Now, the government is also starting cashless.

K Rajesh: Sir, are you also approaching the Navy and Air Force?

Manish Grover: ECHS has come.

K Rajesh: It has come.

Manish Grover: Navy, Air Force and Army have got 3 centers. It has come in 3 centers. It has come in Jaipur and Jodhpur. We have got 10 more files. We have got files from Meerut, Lucknow and Mumbai.

We have got 2 places. ECHS has come in Jaipur and Jodhpur.

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K Rajesh: Sir, what about South? Tamil Nadu, Kerala, Karnataka?

Manish Grover: Sir, we are opening a hospital in Bhubaneswar. We are starting in Bhubaneswar. We have opened in Chennai and Bangalore. My hospital has started in Hyderabad. My hospital in Hyderabad has started 3 days ago. Earlier, there was a clinic for day care. Now, we have converted it into a hospital.

We are converting old clinics into hospitals. We are

converting old clinics into hospitals. We

leave them there and take a bigger place. Dehradun also started a week ago. And just yesterday, I am going to Ahmedabad and Vadodara. So Ahmedabad and Vadodara has started and I am starting in Surat.

K Rajesh: Sir before main board is there any bonus? Any planning?

Manish Grover: We had put a policy for the bonus, Na? We have made a policy for the bonus and we have been giving it every year for the last two years. Dividend, sorry, dividend. No, bonus is not possible before the main board, sir. Because the limit of the government is not eligible according to the norms. It has become a dividend policy.

K Rajesh: You are generating good revenue.

Moderator: Thank you. The next question comes from Nishant Gupta from Minerva Global Capital .

Nishant Gupta: I have 2 -3 questions. Your business model is of 1100 doctors. Is it a plug-and-play model? Will the treatment be the same from one hospital to another? Or will it be different according to the doctor?

Manish Grover: No, sir. Our treatment is the same in any hospital. Our protocol is fixed. If there is a kidney failure, we have to do this. If there is diabetes, we have to do this. If there is blood pressure we have to do this . Our business is not doctor -driven. Our business is process -driven. We have a body healing base. We have a detox base.

Our company's base is . Sarveshanam Roganam Nidanam Kupita mala. It means that if there is any disease, we have to detox the body, clean the internal dirt, strengthen the body's immunity, and the body gets better. We have any kind of patient who feels that nothing can happen in the world.

They come to us and comes with hope, and we fulfil it as well . Because whatever we say, they follow it and the patient gets the result.

Nishant Gupta: Actually, I myself prefer Ayurveda. My second question is that where I show Ayurveda, there is a normal local center where the doctors sit. They use medicines of Baidyanath. Can you tell

us a little about your medicines? The market is so big that you can launch your medicines. There are other players in the market whose medicines are already available. Maybe other people also prefer them. Is the market so big that can you tell us a little about the scope of your medicines? Manish Grover: For example, there is Baidyanath, Dabur, Jandu, or Patanjali. In the market of all these, there are classical medicines. Classical medicine means that which is written in our books. They have 5% Jeena Sikho Lifecare Limited
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GST. So, in our company, we have not kept the range of classical medicines. In our company, the whole range is patent. In classical medicines, for example, Baidyanath sells a medicine for INR200. But when I make it properly, my cost is only INR150. So, I have to sell it for INR600, INR500, or INR700. So, my results are also good. In the market, we do not enter the competition of Dabur, Baidyanath, Jandu, Patanjali. Because these people go neck to neck. For example, in the market, there is Churan, Pet Saffa, Kayam Churna. This is sold for INR110 in the market. Now, the powder I make, its cost is only INR90. Because they put Sany in it. Now, after a few days, Sany harms. We do not do any such work that gives a short-term benefit and harms the patient in the future. We neither use preservatives in our herbal tea, nor do we use any chemicals. Our expiry is also less. Our treatment is patient-based. For example, a patient's nature is Vata. According to Vata, its treatment is Agni Mand. We have a doctor-driven treatment. But there is a protocol. If this is the case, then this is what you have to give. Select from this. Along with this, our biggest plus point is beauty. We focus on the diet very well. We focus on not eating after sunset. Whenever you eat, do not drink water with it. Eat fruits before

breakfast. Eat salad before lunch.

We give more guidance and education to the patient. So that the patient is sent to us all his life. We make the patient our own forever. Whenever he has any relationship or illness, he sends us the patient.

Nishant Gupta: Got it. One last question. Are you going to put your own facility in this or are you going to outsource and make it on a contract basis?

Manish Grover: Sir, we are making it on a contract basis. We do not want to invest in capex. So from our factories, we have taken two loan licenses. We took a loan license 6-3 months ago with a factory in Amritsar. It will be made in our name. In a way, it will be called our factory. We have given them a small deposit. So that formula will be patented in our name. They will make all the products in our name. In the OTC that we are launching, it will be manufactured in our name. Jeena Sikho name has been come. Because we took the loan license in our name.

Nishant Gupta: Got it. So they will make it exclusively for you and you will oversee the raw material ...

Manish Grover: They have made a unit exclusively for us. They have made an exclusive unit for us. A research laboratory. They have made it exclusive for us. So our raw material will be shifted there. It has started to be made. We have started getting 2-3 products. It is being manufactured in our name. But we did not invest. We made an agreement with them and gave a little security.

We assured them that we will take the raw material from you. We have kept a person who will ensure quality. Whenever we buy or manufacture our team stays there. We mix the raw material in front of us. It is packed in front of us. We have got access to the cameras. We took the factory on a loan license. But without any investment.

Nishant Gupta: Thank you. Sir.

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Moderator: Thank you. The next question is from Anurag Agrawal from Multifly Wealth. Please go ahead.

Anurag Agrawal: Manish ji, I have a question for you. What is the occupancy rate in our oldest hospital? As you said, our word of mouth is very strong. Right now, we are running around 60% occupancy. Our first major hospital should have a higher occupancy right?

Manish Grover: Yes, you are right. Meerut hospital, Lucknow hospital and Dehra Basti hospital are the oldest hospitals. The occupancy rate in all three is 80%-90%. Mumbai hospital is running at 40%-45%.

If you add the average, the occupancy rate is increasing every month. For example, I closed my year on 31st March. I had 1270 beds. My occupancy rate was 38%, because 300 beds were added in the last month. Otherwise, the occupancy rate would have been 45%. Now, I have added 120 beds in the month of September. If I see behind 1140 beds I will see 775 beds then my occupancy rate is 60% plus.

Anurag Agrawal: One more question. The likes of Patanjali have big day centres. They NABH credited or not?

Manish Grover: Till now, we have the highest number of NABH in India. Today, it is 33. Four are coming in

this month. After us, we are far behind. There are less than 10 NABH. There are 5, 6, 4, 3, 2.

Anurag Agrawal: You have mentioned the number of NABH. Is it possible that there are more beds per location?

Manish Grover: No, sir. Without NABH, we don't get any government policy. For example, we have Rajasthan, Delhi and Central Government policies. First of all, we don't get it without NABH. We don't get Aishwarya Mani Yojana without NABH. This is the norm. Government applies NABH to hospitals.

Anurag Agrawal: Your competitors may have less hospital numbers but more beds per hospital. This is also a possibility, right?

Manish Grover: No, sir. We have the highest number of beds in India. Patanjali has it. We have it at number 2. Patanjali's location is mainly Haridwar based. Patanjali has 11 centres in India. Out of those

11

centres, 5 people have been contacting me for the last 3-4 months because they are not getting beds. We inaugurated Panchkula Hospital in September. It was associated with Patanjali for 1.5 years. Patanjali's agreement was closed and we got it.

Anurag Agrawal: Okay. When did Panchkula get inaugurated?

Manish Grover: In September. In the last month, its sale was INR50 lakhs. In the first month, its sale INR23 lakhs. In the next month, its sale was INR50 lakhs. October's sale was INR50 lakhs and it was INR51 lakhs. Patanjali's sale was INR32 lakhs. We took it on rent. I am going to make it India's first IVF hospital. Ayurveda's IVF. A combination of Ayurveda and naturopathy.

Anurag Agrawal: Right. Thank you for the information.

Moderator: Thank you. The next question comes from Akshay from CD Integrated Services Ltd. Please go ahead.

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Akshay: Namaskar, sir. I have a question. We are in the Ayurvedic sector. What is our competitive age with other hospitals? Allopathic medicines from other hospitals like cancer medicines. We also treat cancer and liver. What is the difference between allopathic and Ayurvedic?

Manish Grover: Sir, we don't have any competition. We have never thought about it. We are doing our job. The patient chooses where to go. Allopathic or Ayurvedic? The most important thing is that in allopathy, the cancer treatment is recurrent. For example, most of the patients come to us in stage 4. They don't have any treatment. It is unfortunate that Ayurvedic patients come last.

They should come first. But when they try everything, we don't have any competition. Our target is to increase the patient's lifespan, detox his body, give him knowledge, training, diet guidance, educate him and make him our own doctor.

He should treat himself. We give such guidance about diet and lifestyle. In Ayurveda, there is a sutra that when a disease is cured, then fasting is the best medicine. We have prepared some methods for fasting. For example, don't eat for 12 hours, don't eat for 16 hours, don't eat this, don't eat that.

It happened in such weather. We give all such training. We don't have any competition with anybody.

Akshay: Okay, sir. The second thing is that we are expanding internationally in different countries. What will be the difference between our domestic price and the international price?

Manish Grover: Yes, sir. The homework I have done is that we get INR1500 for Shirodhara. In Dubai, we get the same rate of INR3000-INR3200. If we compare ourselves with the normal centers there, our income will increase from Dubai to the Middle East. Because there is a difference in the value of currency.

People come to us from India, Dubai, Middle East, New Zealand, Australia. They say that they have to bear the expenses of coming here. So, it is better if you keep a rate of INR50,000, we don't have any problem.

Akshay: Okay, right, sir. Sir, I wanted to know about the hospital. You said that you have been connected with the patients from the beginning to the end. How do you manage the quality across all the hospitals in our country?

Manish Grover: Sir, we have made a department for diet. Diet is the main thing in our country. The management staff in our hospitals have been brought from hotels. The hotel management staff who change the bedsheets and bathrooms are taken care of by the hotel staff. So that they maintain cleanliness. We have also trained the cooks of Ayurveda so that they know how to cook the millets.

So, we take care of hygiene and cleanliness. We have made a standard protocol which is applicable everywhere. The same checklist is followed everywhere. The doctor has to go around twice in the morning and evening. The doctor also has the power to rule out any slight deficiency.

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Akshay: Okay, sir. The last question is that we have a revenue target of INR700 crores. What will be the margins of the profit after tax target in FY26?

Manish Grover: Sir, my target will be 27% -30% to increase the PAT. In 2021 -22, my profit was 6%. In 2022 -23, it increased to 11%. Sorry, in 2021 -22. In 2022 -23, it increased to 16%. In 2021 -23, it increased to 21 %. So, our target is to increase the PAT by 25% in this year and by 27% -30% in the next year.

Moderator: Thank you. The next question comes from Isha from VT Capital. Please go ahead.

Isha: I had two questions. One was related to capex. Since the number of beds target was 2100 in FY26, what will be the guidance of capex over the next 2 -3 years?

Manish Grover: Ma'am, whenever we open a hospital, like a hospital with 40 beds, we invest a maximum of INR1 crores. Till date, we have invested an average of INR3.5 crores per bed. We have invested a maximum of INR3.5 crores on 100 beds. Till date, we have invested Rs. 3.5 crores. We have invested INR1 crores on 100 beds in Panchkula. We have invested an average of INR 1 lakh per bed.

We have invested INR70 beds in We have not made any investment. We have invested it for free. Someone made it, we are running it. We have not made any investment. We are very particular about capex. We do not invest. We will not invest where my money got stuck or my return came late.

Isha: Okay Mr. Acharya. And the second question was that the target of your average revenue per bed is INR8700 in FY26. So any guidance that you want to give that you are still at Rs. 8700 per bed.

Manish Grover: No ma'am, we don't want to increase the patient's charges. We want to provide good services for rich and poor people at affordable rates. So I won't increase the price. My average revenue per day per bed in FY22 -23 was INR6100 in FY22 -23. In FY23 -24, it became INR7900. Now my average is INR8100 in 6 months.

So I expect it to go from INR8500 to INR8700. It won't go above INR9000. Because my target is not to charge more from the customers. I want to provide affordable services and good facilities to people in less money. And despite this, I am getting good margins. Even now, I have made a profit of 21% -23% in 6 months.

Last year, it was a profit of 21%. The year before that, it was 16%. The year before that, it was 11%.

Moderator: The next question comes from Priyam Khimawat from Ask Investment Managers. Please go ahead.

Priyam Khimawat: Acharya ji, I want to know one thing. Currently, there are around 50 beds in our hospitals. So at any given point, considering 50 beds occupancy, 70 -- 50 beds are occupied. So which is the most selling therapy? Is it kidney? Is it liver? What is it?

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Manish Grover: See, till now, the patients who come to us are those who have kidney failure. Those who have increased creatinine, those who have increased urea, or those who have kidney issues the most. Number 1 is for kidney issues. Number 2 is for cancer. For healing, for cancer reversing. Number 3 is for liver.

Then there are heart patients. After that, there are general patients for diabetes, blood pressure, amputation, or those who have autoimmune diseases. There are a lot of people with autoimmune diseases. In today's date who don't even know why they have this disease. But what happens is, as soon as we detoxify the body and fix the diet, the autoimmune disease automatically reverses. Because the body works only through our intake. And as soon as we fix the diet, reduce their cooked food, increase their natural food, their symptoms go away.

Priyam Khimawat: And in the last one year, how many therapies have we added to increase the number of patients?

Manish Grover: No, no, no. There is no meaning of therapies. A patient comes for treatment. There is no specific therapy for treatment. A patient comes after

hearing our name, after seeing our goodwill, after

seeing our results. We have a total of more than 85 therapies. But in the routine, 14, 15, 16 are used. There are some therapies that vary from patient to patient. What we do is, we take a total charge.

For example, there is a general ward. We took INR8000 per bed from the patient. Now we have to give 3 therapies, 4, 5. We don't take any extra charge from that. We haven't made it a business to earn money. We help others. Once we decide the rate in our hospital, we don't take a single rupee from it.

While leaving they have to buy some medicines that's it. That's why they think that there is no

robbery over here, that it's a good thing, that it's just helping people. That's why our goodwill is increasing right? If we start taking money from everything, if we put money in everything, then our goodwill will decrease. That's why we don't give preference to money.

We give preference to customer satisfaction. That the customer should be happy. The customer should get well and go.

Priyam Khimawat: Sir, how many of your 450 plus doctors are on payroll and how many are on consulting basis?

Manish Grover: 411 are on payroll. Our doctors are not on consultation. They are all on payroll.

Priyam Khimawat: On an average, if a doctor is having 10 -year or 5 -year experience, what kind of an annual income do they make?

Manish Grover: In Ayurveda, if someone has 5 -6 years of experience, then he gets INR60,000 -INR70,000 salary.

If someone is a little good, then he gets up to INR1 lakh. In our country, there are at least 25 doctors in that range. In all the big hospitals, there are at least 3 -4 doctors. In every big hospital, there are doctors above INR1 lakh.

Priyam Khimawat: Got it. When you were talking about ESOP, have you given ESOP to your top 25 doctors?

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Manish Grover: Yes. In the first round of ESOP, we had set the criteria for all the employees who are 3 years old. All the doctors, call center agents, helpers, support medical staff, plus guard, plus gardener.

We have given ESOP to 675 people in the first phase. Till December 31st, based on their performance, the ESOP that we have left, we had given half ESOP and kept the other half. We will announce the other half by December 31.

We have kept the criteria of performance and we are giving it to all the doctors. To doctors and healthcare workers. For example, healthcare workers like nursing staff, GNM, those who don't get it, and those who get old, those who are valuable to me, I have given ESOP to all of them. I have given ESOP to people who are 3 years old. Now on December 31st, we will give ESOP to everyone.

Moderator: The next question comes from Amit Jeswani from Stallion Asset Pvt. Ltd.

Amit Jeswani: Ram Ram Ji.

Manish Grover: How are you sir? Namaskar Amit Ji. How are you?

Amit Jeswani: All good. Thank you.

Manish Grover: We get better just by hearing your voice. You guide us like an elder brother. You teach us.

Amit Jeswani: Thank you so much Manish Ji. Manish Ji, you are going for international expansion. We have done a new acquisition in between. Your OTC is also coming. How are you focusing and how are you increasing the management bandwidth? I have no doubt on your capabilities. We are doing so much business together. The scale will come. You will win.

But how are you managing your time? Because people are everywhere. But still how are you managing your time?

Manish Grover: Sir, I have a flight to Ahmedabad and Vadodara tomorrow night. I go to every hospital and do all the training and motivation. After that, I have developed my training team. The company we acquired in Oregon, we have retained the entire team of that company. Right. We have retained the team of that company and the top people of that company, we have given them more work in our company.

We have told them that they will get ESOP and they have joined us. We have given different work to everyone. We have hired new doctors of good level. I mean of high level. You know that we are planning on IVF and cancer very soon. We are going on IVF plus Ayurveda combination which is named as Garb Dharan.

This will be the first business in India. It will be the first center where Ayurveda and allopathy will sit together and we will solve all the conceiving related problems and we will solve all the conceiving related problems all the conceiving related problems and we will solve all the conceiving related problems.

Amit Jeswani: You are going in new blue ocean one after another.

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Manish Grover: I have taken a vow that I will not go in red ocean. Why do I go in jungle and fight with people.

I am not getting into competition. The products which I am getting from OTC have no product in their market. We are single person and single company. I am not getting into that range where many people are sitting.

Amit Jeswani: So you first opened a medicine business in Ayurveda, from there you went to the hospital. Now you are going international from the hospital and expanding the hospital and OTC as well. What kind of feedback are you getting in OTC? Like we have just started a new sale, Mr. Manish Ji.

Manish Grover: We have done a 6-month homework trial. So as soon as they heard that we are launching, they are very excited. And we have also made dealers in the market. We will start in the first phase in Punjab and UP. And we are also bringing our own Shilajit, our own kidney detox product, liver detox product. I am coming to the market with about 12 products.

And in which there are 6 clinical trial products and 6 general products. But they will also be approved by Ayush. And I hope that in the next 1-2 years, within 2 years, I will reach number 2, number 3 in medicine business in India.

Amit Jeswani: In the Ayurveda medicine business?

Manish Grover: In the Ayurveda medicine business, whether it is Dabur, Vaidyanath, Patanjali or Zandu, I will go above those categories. Because my products have not been thought of or made by anyone else in the market. I have developed such products from my experience that the common man has no solution in the market. Like now you have kidney stone, what will you do? You will go to allopathy.

But you know that there is a product called Kidney Shuddhi. So you will buy it from the market yourself.

Amit Jeswani: Right. I always called you a rocket. You are a proper rocket. Manish Ji keep going. We will be with you.

Moderator: The next question comes from Akshay from CD Integrated Services Limited. Please go ahead.

Akshay: Sir, we have included international sales in our FY26 guidance of around INR650 -INR700 crores.

Manish Grover: Yes, we have included international sales. But we have not included OTC sales in it. So we will get extra OTC sales.

Moderator: The next question comes from Nitin Gupta, an Individual Investor.

Nitin Gupta: I have been watching and listening to your interviews for the last couple of times. I like your plans and expansions. I have a small question. The rest of the questions have been answered. To fund those expansions, do you have any plan to raise funds in the future? Or everything can be funded by internal approvals?

Manish Grover: Sir, everything will be done by internal approvals.

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Nitin Gupta: So there is no need for extra funds in the foreseeable future?

Manish Grover: Sir, if there is a need for funds. If my tie-up is final. If our company needs factory plus product. Then we will require funds. Otherwise, we will do it internally.

Moderator: As there are no further questions from the participants. I now hand the conference over to the management for closing comments. Manish sir, you can give the closing remarks if there are any.

Manish Grover: Thank you to everyone. Thank you very much. To keep our enthusiasm high. And we assure you. That in India, with Ayurveda and naturopathy. Our mission is to become the biggest and most helpful company in India. We will continue on that mission. Thank you to everyone. Thank you.

Moderator: Thank you. On behalf of Jeena Sikho Lifecare Limited. That concludes this conference. Thank you for joining us. And you may now disconnect your lines.

Call: May 2025

To,
The Manager,
Listing Compliance Department
National Stock Exchange of India Ltd.
Exchange Plaza, Bandra Kurla Complex,
Bandra (East), Mumbai -400051

SYMBOL: JSLL
ISIN: INE0J5801011

Sub: Intimation to Stock Exchange – Transcript to the Conference Call for the half year and Financial Year ended March 31, 2025 held on 20th May, 2025

Dear Sir/Madam,

Pursuant to Regulations 30 read with Schedule III of the Securities and Exchange Board of India (Listing Obligations and Disclosure Requirements) Regulations, 2015, as amended, please find the enclosed herewith the transcript of the conference call held on Tuesday, May 20, 2025 at 02:00 P.M. for discussion of Audited Financial statements for the half year ended and Financial Year ended March 31, 2025.

The aforementioned transcript to the conference call is available on our website at:
<https://jeenasikho.com/wp-content/uploads/2025/05/Jeena-Sikho-H2FY25-FY25-Earning-call-Transcript.pdf>

Kindly take the above intimation on your record.

For Jeena Sikho Lifecare Limited

Manish Grover
Managing Director
DIN: 07557886

Date: May 26, 2025
Place: Mohali

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“Jeena Sikho Lifecare Limited
H2 FY25 & FY25 Earnings Conference Call ”
May 20, 2025

MANAGEMENT : MR. MANISH GROVER -- MANAGING DIRECTOR -
JEENA SIKHO LIFECARE LIMITED
MR. NANAK CHAND - CHIEF FINANCIAL OFFICER -
JEENA SIKHO LIFECARE LIMITED

MODERATOR : MR. RANVIR SINGH -- NUVAMA WEALTH AND INVESTMENT LIMITED

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Moderator: Ladies and gentlemen, welcome to the Jeena Sikho Lifecare Limited H2 -FY25 and FY25 Earning Conference Call. As a reminder, all participant lines will be in listen -only mode and there will be an opportunity for you to ask questions at the end of today's presentation. Should you need assistance during conference call, please signal an operator by pressing star then zero on your touch -tone phone. Please note that this conference is being recorded.

I would now like to hand the conference over to Mr. Ranvir Singh from Nuvama Wealth and Investment Limited. Thank you and over to you, sir.

Ranvir Singh: Thank you, moderator. On behalf of Jeena Sikho Lifecare Limited, I extend a very warm welcome to all participants on the H2 FY25 and Full Year FY25 Financial Results Discussion Call. Today on our call, we have Acharya Manish Groverji, the Managing Director and Mr. Nanak Chand, CFO.

Before beginning with this call, I would like to give a short disclaimer. This call may contain some of the forward -looking statements which are completely based upon management beliefs, opinions, and expectations as of today. These statements are not a guarantee of the company's future performance and involve unforeseen risks and uncertainties.

With this, I would like to hand over this call to Acharya ji for his opening remarks. Over to you, sir.

Manish Grover: Namaskar to everyone. Right now, I am presenting the results of H2 -FY25. I will also be presenting the comparison of H1 and H2. Thank you, Mr. Ranvir . We are very happy to inform you that Jeena Sikho Lifecare has given excellent financial and operational performance in both H2 and FY25. In H1 FY25, we had a revenue of INR214 crore , In H2 FY25 which increased to INR255 crores .

The total sale was INR469 crores , which is around 53% in the sale and 45% in the annual growth. Service vertical has witnessed highest growth because of which we have seen random growth. In H1FY24 we had a revenue of INR157 crores , which increased to INR214 crores in H1FY25 . In H2FY24 , we had a revenue of INR167 crores , which increased to INR255 crores H2FY25 . Our hospital, clinic, and day care center have extended the revenue and have contributed a lot in this year's growth. In our service vertical, the revenue which was INR 138 crore in FY23 -24 it has increased by 84% in 24 -25 which increased to INR254 crores . And this shows we are Pan - India, Ayurvedic and naturopathy brand.

This shows this year we have added 573 new beds, and by 31 march is 2173, approximately 2200 beds. We are now present in 23 states and more than 100 cities have been present. We are connecting with patients in new hospitals and by 31st December 2025 there are 650 beds which will be added.

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In our product vertical you can see 16% annual increase. In 2024, INR186 crores has been now INR215 by 2025 which strengthen our free cash generative revenue system. We are moving our new product range forward.

In December 2024, we launched the INJK Water device and people liked it a lot. Now we have 10-15 new products in the pipeline. The sale of the INJK Water device has also reached INR1.5 crores per month. In the last 3 months, approximately 500 beds have been added. Along with revenue growth, our EBITDA is 34% and the PAT is 31% higher than the previous year.

Our EBITDA was INR93 crores on March 31, 2024 and INR125 crores on March 31, 2025. Due to the increase in the business and 500 beds in the last few days, there is a slight difference in the margin of only 1.5% and that is also a temporary reduction. But we hope that as the revenue of the new centre increases, the margin will improve again. In April, our sale has doubled from last April to this April. In last April, our sale was INR30 crores and in this April, it

has increased to INR60 crores .

Now I would like to tell you that in 2024, we had 1277 beds and in H1, we had 1530 beds. In H2, we have added 2173 beds. That means we have added approximately 650 beds. Earlier in H1, we had added 250 beds. In H2, we have added 573 beds and 650 beds are in the pipeline.

Now I would like to invite Mr. Nanak to share the financial details. December 2024 .

Nanak Chand: Thank you, Manish sir. Good afternoon, everyone. As mentioned by Manish sir, we have achieved a healthy financial performance during the H2 FY '25 and FY '25. Here, there are these key highlights of H2 FY '25 results.

Our revenue grew by 53% on Y -o-Y basis, that is INR255 crores in H2 FY '25. The EBITDA during the H2 FY '25 grew by 36% Y -o-Y rate INR65.60 crores, implying the EBITDA margin of 26%. The H2 FY '25 PAT grew by 17% Y -o-Y INR43.85 crores.

Now FY '25 results, the revenue from the operations grew by 45% Y -o-Y to INR469 crores.

Given why the hospital service business, which is grew by 83% Y -o-Y, i.e. INR253.80 crores, while the product sales grew by 16% to INR215.27 crores. The EBITDA during the FY '25 grew by 34% Y -o-Y to INR124.88 crores, implying to EBITDA margin, 27%.

The FY '25 PAT grew by 31% Y -o-Y to reach INR90.73 crores. We maintained a healthy balance sheet with the net cash equivalent to INR26 crores by the end of the FY '25 or FY '25. Our ROE and ROCE stood 39% and 63%, respectively. We spent INR42 crores capex in FY '25.

With this, we can open the floor for the question and answers. Thank you.

Moderator: First question is from the line of Agastya Dev from CAO Capital. Please go ahead.

Agastya Dev: Namaste, Manish ji. Sir, I wanted to understand one thing from you. Your hospitals are fully functional, where you have not added a single bed. What is the occupancy there, sir? Sir, when

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the hospital is in the growth phase or maturity phase, how much occupancy is there in a year according to the operational bed?

Manish Grover: Sir, the occupancy of this year is 53%. Shall I tell you the number of patients and the IPD volume?

Agastya Dev: Sir, I saw the presentation.

Manish Grover: No, no, just a minute. In 2022-23, a total of 5700 patients were admitted. In 2023 -2024, 13000 patients were admitted. In the first 6 months of 2024 -2025, 12000 patients were admitted. In the next 6 months, 12500 patients were admitted. So, the total number in 2024 -2025 was 24,500 patients. Last year, 13000 patients were admitted. So, if I take out the whole year, it was 53%.

Agastya Dev: Sir, my question was a little different. The hospitals that you have fully constructed, where everyone in the local area has come to know that you have a hospital, how much occupancy is there there?

Manish Grover: Like, our oldest hospital is Dera Bassi.

Agastya Dev: Yes.

Manish Grover: It always runs at occupancy of 80 -90%. The second number is Lucknow. It always runs at 80 -90%. The third is Mumbai. It always runs at 70 -80%. Now, on 1st April 2025, in this financial year, we have increased the rate of all hospitals by 15%.

Agastya Dev: Yes.

Manish Grover: I mean, we have increased the rate of all the big hospitals. Like, the oldest one is Dera Bassi, Lucknow, and our Mumbai. There are three big hospitals. One is in Meerut. But in Meerut, when we started, there were 250 beds. Then, 6 months ago, 80 beds were increased. Now, in April, 150 more beds were increased. So, the number of beds is increasing every time. Now, it has reached 460 beds. It has become 500+.

Agastya Dev: Sir, this is why I asked you this question . Because, the number you gave in the presentation of 1600, that is your year -end number. And you are adding beds so fast that the occupancy is not understood properly when the hospital is ready ?

Manish Grover: I will tell you what it is. By 31st March, we have incre

ased the total number of beds by 2200.

But, some of them did not get the clinical certificate. Some did not get the NABH . So, I could not take the patient there. Like, the one in Gurgaon was opened on 15th May ...

Agastya Dev: Sir, I am asking this because these are all temporary things. These will be sorted out in a month or two.

Manish Grover: I have told you the number of beds by 31st March.

Agastya Dev: Yes. Sir, do you think that by the end of this year, 3000 beds will be crossed?

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Manish Grover: By the end of this year, it will be around 2800.

Agastya Dev: Sir, next year?

Manish Grover: By 31st March, it will be 3000+.

Agastya Dev: Yes, I am asking about the end of March. And till March 27, what is your plan, sir?

Manish Grover: I have papers for March 27. By the end of this year we will be 3000 beds. Next year, instead of increasing our beds, we have come up with a new idea. In India, there are a total of 600 Ayurvedic colleges. In India, there are 600 Ayurvedic colleges. Every college has a hospital of 100 beds. We have started talking to them. I have been finalized in three places. Within 3 to 6 months, we will have about 300 beds added. But our expenses will not be incurred in that. We will take over the entire college hospital. We will run it. It will cost them money.

Agastya Dev: I understood, sir.

Manish Grover: So, we are coming up with a new model. So, we have talked about the first hospital. It has an agreement on this Friday. We have taken Saraswati College. Saraswati College is in Mohali and Chandigarh. And after this, the second agreement will be with Sanskriti University. Those who have 150 beds are in Vrindavan, Mathura. This is not added in these 650 beds. This is extra. I have told you that these 650 beds are separate.

Agastya Dev: Understood, sir.

Manish Grover: We are not doing investment in these.

Agastya Dev: Yes, I understood, sir. Sir, my second question was that you said that you have given a lot of expenses in the front end. I mean, you have taken a lot of doctors. And you have also faced a little cost pressure in the procurement of Ayurvedic products. So, in that, sir, whatever cost we have incurred in this quarter, it is completely because of that. Right, sir?

Manish Grover: No, no, sir. When did I say that there is a cost pressure? I said that the last six months from that the last three months, about 500 beds have been added because of which our salary has increased.

Our rent has increased. And depreciation has come. But its benefit has come in April. In last April, my sale was INR30 crores. And in this April, my sale is INR60 crores which is closed. And in May, the same run rate is running.

Agastya Dev: Yes. Sir, in your presentation, in your comments, in the second column, the paragraph is that Slide compression gross margin attributable to increased cost of procurement for some consumables and medicines. Sir, I have understood your operating expenses. But what you have said about consumables and cost pressure, what is the reason for this?

And sir, generally, what is this? The price you have increased by 15% has been done across the board, sir. If it has been done in every hospital, will it pass through? Or will it take a little longer?

Manish Grover: Sir, let me explain it to you like this. One minute, let me tell you. The H1 in 2023, H1 in that, my average sale was INR26 crores. In H2, my average sale was INR27.80 crores. In H1 in 2024 - Jeena Sikho Lifecare Limited
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25, my average sale was INR35 crores and in H2, my average sale was INR42 crores. This time, my average sale is not even INR60 crores. As the sales are increasing, So my consumables will also increase. Because my patients are increasing, so my food will also increase and their oil expenses will also increase.

Agastya Dev:

I have understood your point, sir. Thank you very much, sir. All the best, sir.

Moderator: Thank you. Next question is from the line of Abhishek from AB Capital. Please go ahead.

Abhishek: Sir, it feels good to talk to you. I have been following you for a long time. I am talking to you for the first time today. Sir, how much revenue will you do next year in FY26?

Manish Grover: Sir, the way I have sold INR60 crores in April, If I hold this running, then INR720 crores will be the minimum, right? But if there is no happiness or sorrow in between -- in the time of the war between India and Pakistan, I had a little shock for 4 days. The number of patients had reduced for 4 days because the war between India and Pakistan was going on. So some patients took a leave from the hospital. But as soon as the war ended, we managed it again. So we have reached the same average as last month.

Abhishek: So next year, you are saying that the margin will also increase a little. As this time it reduced a little. Yes, because I have increased the rate.

Manish Grover: I have increased the rate. And all my investments came this year. The investment came this year and the revenue will come this year.

Abhishek: Sir, how will you do it? You are saying INR720 crores.

Manish Grover: I have sold INR60 crores in April. We don't need this capital, sir.

Abhishek: Okay, okay. And those who are saying that you will run Ayurvedic hospitals, for that, you will not use capex next year?

Manish Grover: No, no, I have taken out a model like this that we will not use capex. What are we doing? There are 600 hospitals in India. There is an Ayurvedic college in every city. Every college does not work. So we started tying them up. My first agreement. Saraswati College in Mohali. It will be signed on this Friday. It is a 100-bed hospital.

So first I will give 50 beds in the first phase. And in the next phase, I will give 50 beds after 3 months. I have tied up with Sanskriti University. I will get 150 beds from them within 3 months.

So they are giving AC and fans. They are also buying an ambulance. I am getting all the expenses from them.

Abhishek: So, sir, would it be right to think that that ROI will be negative, sir. Because of that, our ROI will increase a little more, sir.

Manish Grover: Sir, I will make it a cheap hospital. I will keep the rate a little less in them. So that poor people can also come. For example, the rate in my hospital is now INR10,000 or INR9,000. I will keep the rate there at INR6,000 -INR7,000. And I will get half the staff there for free because it is a Jeena Sikho Lifecare Limited

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college. So there are already staff in the college. There are children and doctors to study there. So I will get all the doctors and nurses included. So we have developed this new model.

Abhishek: Okay, sir. Sir, how will our OTC business run?

Manish Grover: Sir, we deliberately delayed OTC. I avoided OTC once. Because I saw the opportunity here first. So I said that we will do OTC later. It is a new business. Let's increase this first. So that's why I increased my sale this year. I mean, I planned to increase this business first. OTC is fully prepared. What is there in OTC?

A new guideline of the government has come. 1954 Magic Remedy Act. So we are completing its laws. That's why we delayed OTC and put all the effort here. That's why my sale which I told you is now 40% more than the average in April and May.

Abhishek: Okay, sir. Sir, if you had given us quarterly results, it would have been better.

Manish Grover: Yes, we will get quarterly results this time because we are coming to the main board. So we will have to give quarterly results. We have spoken for the main board. We have spoken for the main board. We have put an AGM. As soon as it is completed on the 30th, we will apply for the main board. We are fully

prepared. Our paperwork is ready.

Abhishek: Okay, sir. Thank you.

Moderator: Thank you. Next question is from the line of Akshay from AK Investment. Please go ahead.

Akshay: Hello, sir. Yes, sir. My first question was that recently I have launched one more medicine and we are going to launch 15 more medicines. What was the traction in the initial 2 -3 months? In the last quarter, when we opened, you said that in the market, we are getting food and other things. Our cost is very high. What is our uniqueness in that? What are we giving in that price? Please explain that.

Manish Grover: We have run a trial and we have appointed our market team. But we are still studying the market. We have not launched it yet. We have the product. We are waiting for the trademark of the product. We have applied for the trademark and we have also taken the website. We have finalized 10 -15 products. The first product will be like Pet Saffa Pet Shuddhi . It will be in the form of a kit. Single products are sold in the market. Now Pet Saffa is sold for INR115. Kayam Churna is sold for INR115. But I have launched a product for INR960. I have launched a kit for that.

I have tied up with Dainik Jagran, Amar Ujala, Hindustan, Punjab, Kesari. So within 2 -3 months, it will be in the market. We have chosen UP as the first state. We will launch the first product in the same state. The team is in the market. They are making distributors in the state. My team has been in the market for 15 days. Some are in Lucknow, Kanpur, Bareilly. We are creating a distributor network.

As soon as the distributor network is created, we will launch it. Our uniqueness is that no product will come single. It will come in the form of a kit. The selling price is INR960. Whereas Pet

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Saffa is for INR115. Kayam Churna is also for INR115. Zandu's Nityam is around INR200. Our selling price is INR960.

Akshay: Okay. I understood. The second thing is that our per bed revenue is INR8,200. We have made a good expansion in the financial year so that our margin has reduced. What can we take as average revenue next year? Occupancy rate 53% in the next year?

Manish Grover: Sir, where has the margin reduced? In 2022, my margin was net PAT 8%. My turnover was INR145 profit 11% . In 2023, my turnover was INR204 33% profit was 16% . In 2024, my turnover was INR324 INR69 profit 21%. In 2025, my revenue is INR469. And the profit of INR91 is 19.5%. That has also made a difference because in the last 3 months, I have invested a lot. My salary increased by INR9.9 crores in 6 months. And my rent increased by INR7.6 crores when I acquired a new hospital. Okay. I have also changed the rate from 1 April.

Akshay: We have tied up hospitals with colleges. The expenses will be paid by the colleges. Do we have

to do any revenue sharing? What is our business model?

Manish Grover: Rent and revenue sharing . In Vrindavan -Mathura, I have given them a rent of INR2 lakh and 6%, which is the highest. I have made such an agreement. And in Chandigarh, I have given them a rent of INR5 lakh and 6%, which is the highest. We keep the percentage so that the other person also supports us. If we take an empty rent, they don't support us.

Akshay: Right. Sir, my last question is that we hear a lot about you regarding cancer and allopathy. I wanted to understand the cost difference in cancer treatment. For example, let's assume that allopathy, chemotherapy and any other surgery costs INR20 lakh. What are the benefits of Ayurveda and what is the gestation period according to allopathy? How many days does the patient get cured? What is the cost difference between the two?

Manish Grover: Sir, we don't talk about the cure. Let me tell you what we do. Listen to what we do. We do the overhauling of the patient. There is a shloka in Ayurveda that all diseases are

due to the fire of

the mind for all diseases. The lack of proper digestion, weak metabolism in the body, and lack of body cleaning are the reasons for the disease. We work on the root cause of the disease. We do Panchakarma. The body fights to fight the disease.

There is another shloka in Ayurveda. Sarvesha Naam Roga Naam Ni danam Kupita Mala It means that no matter what the disease is, get rid of it. We get rid of the mind of the patient. We light up the fire of the patient. We increase the metabolism. We don't cure the cancer. The body fights the cancer. We increase the immune system. We don't claim to cure the cancer. We don't cure the cancer. The research papers have been published. The cancer of the patient has been cured. The patient says it himself.

Akshay: Alright Acharya ji. Best wishes for the FY26. You do well. Cure the patients well. Thank you sir. Thank you.

Moderator: Thank you. Next question is from the line of Naveen Baid from Nuvama Asset Management. Please go ahead.

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Naveen Baid: Thank you for the opportunity. Sir, I have a question. In terms of our pension plans, what are we looking at in the next 2 -3 years in terms of our hospital business?

Manish Grover: I don't understand. Ask again.

Naveen Baid: In the next 2 -3 years, you said that there are 600 hospitals which are there. You want to go in the asset life model. How much bed capacity can we get in the next 2 -3 years?

Manish Grover: In the next 5 years, my plan is to reach 10,000 beds. That's my plan. In the next 5 years, I will reach 8,000-10,000 beds. I won't open a new one. I will take colleges' beds. I got a new model which doesn't even require 2 -3 lakhs. Why will I invest 2 -3 lakhs?

Naveen Baid: So, all these percentages of revenue plus minimum guarantee will be on this model?

Manish Grover: Yes, sir. There is no pain in giving 6%. I will try to get it done in 5 minutes.

Naveen Baid: So, in this, when you tie up with hospitals or medical colleges, the doctors will be on their payroll or they will come on your payroll?

Manish Grover: Approximately 50 %-50%. Approximately 50% will be on our payroll. Approximately 50% will be on their payroll. Our expenses will also be less. But our revenue will also be less. Because most of them are general wards. All the government colleges, government colleges, private colleges, all of them have general wards, not private rooms.

So, their general rate will be 6,000-7,000 per day. But our expenses will also be less. Won't we have an investment? There will be an investment. So, to set up a hospital, a hospital with 100 beds will get 1 crore . Which will be 1 lakh per bed.

Naveen Baid: Normally, if we set it up ourselves, it will be 3 crores?

Manish Grover: Now, if we set it up ourselves, it will be 2.5 -3.5 lakhs per bed. But if we do it next time, it will be 1 lakh per bed. But as of now, there have been 2 tie -ups. So, I have prepared both of them. It's not that you do it, both of them are doing it themselves.

Naveen Baid: So, we are not incurring any expenses?

Manish Grover: No, we are not incurring any expenses. Okay. And the rent will also start when we get the hospital handover.

Naveen Baid: Got it. And this year, in FY25, our product mix, our product revenue is around INR215 crores. So, what kind of growth do we see in that? Can you tell us about the break -up?

Manish Grover: Sir, we are not focusing much on product revenue. We are focusing more on service revenue. There is more margin in that. And the patient recovers fast in that. Like I told you, the service revenue in 2023 -2024 was INR138 crores. And in 2024 -2025, it was INR254 crores. Meaning, I increased it by 83%. But if you see the product revenue, it increased from INR186 to INR215 crores. I only increased it by 16%. So, my focus is on the service vertical. There is direct

profit
in that. And the patient's ticket size has increased.
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Now, that patient gives me 50 -60 thousand per patient. In product, the patient gives me 5 -6 thousand per ticket size.

Naveen Baid: And how much is the margin in the product business, sir?

Manish Grover: The gross margin is 85%.

Naveen Baid: And the EBITDA margin?

Manish Grover: I already told you that the EBITDA of the entire company is 34%. The EBITDA of the entire company is 34%. If I acquire a government hospital, then my EBITDA will increase. Because my expenses will be reduced.

Naveen Baid: Right. No, I was only asking about the product. Okay, sir. That's helpful. Thank you, sir. Okay.

Moderator: Thank you. Next question is from the line of Prerna Amanna from Equity Research Program. Please go ahead.

Prerna Amanna: Hello, sir. I have a question. We had a target of 25% this year. I understand that you had a bed addition and there was a miss. But next year, can we reach that 25% PAT margin?

Manish Grover: No, I can't claim 25%. But the planning that I have made is for a 25% margin. Because there are ups and downs. For example, India -Pakistan. We were stuck for 4 days. We left the patient for 4 days. Then we called him again. I can't say that this will be an act of God. But the rate that I have changed, I hope that I will get a margin of 23 %-25%.

Prerna Amanna: Okay. And the 15% rate that you said, did you increase it above 8,200 means on that pace?

Manish Grover: No, ma'am this 8,200 it is an average of the whole year. The rate that I increased was on 1st April.

Prerna Amanna: Okay.

Manish Grover: 1st April will have a crystal effect in 2025 -2026.

Prerna Amanna: Sir, I have a final question. We had 1273 beds last year. As of now, how many beds do we have?

Manish Grover: Meaning?

Prerna Amanna: At the end of FY24, we had 1273 beds. Now we have 2100 beds as we are talking right now?

Manish Grover: 2173. In the first 6 months, we added 250 beds. In the last balance sheet, we added 650 beds which was the previous balance sheet. In this balance sheet we have added 650 beds we have added.

Prerna Amanna : Means in H2, right?

Manish Grover: Yes. In H2, we added 650. In H1, we added 250.

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Prerna Amanna: Okay.

Manish Grover: In comparison to H1, in H2, we added 400 extra beds. It was supposed to be 250 -250. So, we added 400 extra.

Prerna Amanna: Okay. How much are we going to add this year?

Manish Grover: Right now, 650 is in our pipeline. We are talking about 650. We are preparing for it. We have made an agreement. 650 is ready to move. Out of which, 100 has already started. On 16th May, I did a Gurgaon on a rat in that 100 added to it.

Prerna Amanna: Okay, sir.

Manish Grover: By 31st March, 2026, we will have 3,000 beds.

Prerna Amanna: Okay.

Manish Grover: By 31st March, 2026.

Prerna Amanna: Okay, sir. Thank you so much, sir. Congratulations once again.

Manish Grover: Thank you, ma'am.

Moderator: Thank you. Next question is from the line of Pravesh Kothari from Kothari Investment & Infrastructure. Please go ahead.

Pravesh Kothari: I wanted to know that when you see the results of March 2024 and September 2024, your other income is around INR5 crores, INR6 crores. But this time, when we see in March 2025, the other income is not coming. Can you tell us what was the other income? What was the other income?

Manish Grover: The other income was that we had an FD from which the other income came. This time, we used that money for acquisition. I started getting the benefit in April -May.

Pravesh Kothari: Okay sir.

Manish Grover: We did an acquisition. We purchased a company. The business we purchased they have 85 beds and 178 beds pipeline. That 250 beds is added because of that in

our revenue.

Pravesh Kothari: Okay, sir. What was the other question? You said that you want to open something in UAE and

Dubai hospital or something?

Manish Grover: One has opened in Dubai and the other is about to open. So within two months the company will be on. We have planned to make a total of six.

Pravesh Kothari: And have you seen any response in that? Are you getting any response from growth or Dubai?

Manish Grover: Sir, we have only made one so far. And it has started taking out its expenses. It is not in profit.

It has started taking out its expenses. It has started taking out its expenses. It is neither in loss

nor in profit. In a couple of months, the company will be formed Jeena Sikho International it is

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the subsidiary of this company. It has a 100% subsidiary in which we will do international business. We are not able to do it in this company because it is a legal system.

Management : Sir, we are acquiring the existing business of a company. It is a running business. We are putting stakes in the same company. The business opportunity is already generated. We just have to stretch it a little more and enhance it.

Pravesh Kothari: Will you get any revenue from the company from which you have invested this year?

Manish Grover: No, sir. We have not made any investment yet. We have not made a company yet. We have already tied up there. Our patients have started going there, but if the company becomes international, then the business will be able to be transferred. It has taken so much time in the paperwork. The government policy. We have filled the form there.

We have done everything. The consultant has also been hired. Everything has been done. In one to two months, the entire paperwork will be completed. The clinic that is running in the company will be transferred here. And it has already started taking out expenses. We hope that it will be profitable from this month. Now only one center has been prepared. A total of six centers have to be prepared there.

We have not given any money there yet. We have not given a rupee there yet. Only the INR5 lakh, INR 6 lakhs that have been spent in forming the company have been spent.

Pravesh Kothari: Got it. And sir, this migration that you are going to do. So which month can you get this migration done?

Manish Grover: On the main board. On the main board, we have told our consultant yesterday. That is, it will come in the middle of August -September. The consultant has given a timing of three months.

Pravesh Kothari: When it will get split your shares?

Manish Grover: It will happen within this month. It will happen within this month and the first week of the next month.

Pravesh Kothari: Okay, sir. All the best for your future. Thank you for the future.

Moderator: Thank you. Next question is from the line of Akshit Agarwal from Akshit Agarwal HUF. Please go ahead.

Akshit Agarwal: Greetings, sir. In September, the PAT guidance was INR100 crores last year, it was. So this time it will be INR91 crores. So the guidance has come down to INR9 crores?

Manish Grover: Sir, actually, I have to get a profit of around INR130 crores to INR 140 crores next year. For that, I will have to prepare some land from where I will fly the aircraft. So the hospitals have opened in the last 3 months. 500 beds have been added in the last 3 months. So the revenue of the hospitals added in the last 3 months will come from this month. Its expenses have been added till 31st March.

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For example, the employee benefit has increased by INR9,92,00,000. Rent expense has increased by INR7,60,00,000, the depreciation expense has increased by INR3 crores and the housekeeping expense has increased by INR4 crores. So the expense has come this time and the profit will come

at this time. I just told you that last year in April, my sale was INR30 crores. This year in April, I have closed INR60 crores.

Wait a second, I will tell you the average. In the first 6 months, in 2023-2024, my average sale for 6 months was INR26 crores. The average for the next 6 months was 27, 80,00,000. This year in 2024 -2025, the average sale for the first 6 months was INR35 crores. And in H2, the average sale was INR42 crores. This time, in the second month, I am running on an average of INR60 crores.

And if I check the sale for 20 days till 19th May, I am maintaining an average of INR60 crores.

Even if India and Pakistan suffer losses for 4 days, I am maintaining an average of INR60 crores.

Akshit Agarwal: The annual sales will be INR720 crores will be estimation?

Manish Grover: If I run on this average and God has kept everything happy.

Akshit Agarwal: And what are you expecting from the PA tT?

Manish Grover: Sir, I have been maintaining 20% since the last 2 years. This time also, it has come to 19.5%. Last year, it was 21%. So, you can accept this much.

Akshit Agarwal: Okay, sir.

Moderator: Thank you. Next question is from the line of Amit Agarwal, an Individual Investor. Please go ahead.

Amit Agarwal: Good afternoon, sir. Sir, first of all, like you said last time, that the Supreme Court had ordered that Ayurveda will be included in the Ayushman Yojana for every state. So, sir, what is the latest update regarding that?

Manish Grover: Sir, the government had a meeting the day before yesterday. The day before yesterday, there was an update from the Ayush Secretary and the Health Secretary that the government has finalized the package and its rates. The good news in that is that earlier, Ayurveda did not have a cure for cancer. The new update that came the day before yesterday, if you say, I will share the booklet if anyone wants it.

I will share it with Ranveer ji. Cancer has also been included in that. So, the booklet has been finalized. The rates have been finalized. Now, I do not know when they will implement it, sir. I am writing letters in my own way. I am writing letters. I also complain to the government officials whom I meet. I also request them. Everyone is saying that it will come soon, but even if it is not coming, I am still on my growth, sir.

Amit Agarwal: Okay. Thank you, sir. Sir, there was one more question. Sir, we have not yet done a tie-up with private insurers in cashless insurance?

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Manish Grover: No, sir. There is a cashless tie-up with 12 companies. The rest is being reimbursed. And the biggest thing is that in our 5 big hospitals, GIPSA has also come. That is, National Insurance, Oriental Insurance. The four government companies run in GIPSA. So, that is also our tie-up. If I talk about last month, then last month my revenue in Meerut was INR12 crores. Out of which INR2.7 crores came to me from health insurance.

Amit Agarwal: Okay. Yes, sir. Sir, are we converting the Day Care clinic to a hospital?

Manish Grover: We are converting Day Care. We are not opening any new Day Care. Neither are we opening a new clinic, nor are we opening Day care. We are opening a hospital only because in Day Care, you do not get the benefit of health insurance policy. Sir, we are closing it and converting it to a hospital. And whatever hospital we are converting now, we are doing it in such a way that there is a lawn facility.

Earlier, we were taking only closed buildings, now we are not taking because in a closed building, the patient does not stay for more than 7 days. So, due to being a lawn building, the patient stays for 20 days, 25 days. We have made a minimum stay protocol. We have made a protocol for a cancer patient to stay for 21 days. Many patients are now staying for 21 days.

Akshit Agarwal: Yes, sir. Thank you.

Manish Grover

: Thank you, sir.

Moderator: Thank you. Next question is from the line of Chirag Shah from White Pine Investment Manager. Please go ahead.

Chirag Shah: Namaskar Acharyaji.

Manish Grover: Namaskar. How are you, sir?

Chirag Shah: I am good.

Manish Grover: You are always in our concalls. Thank you, sir.

Chirag Shah: Sir two questions which is a little different. One our profit margin is so good. Why did we need to increase the rates question one. And question two is also a little linked. Till now, we have increased occupancy on bed occupancy by 1600 beds. I am taking the last year's number. We increased occupancy on 1650 beds. Now, our rate is increasing. So, what are you doing as a new incremental effort apart from word of mouth of the patients so that the patient inflow should come?

Manish Grover: Sir, we increased the rate last year because all patients come in big hospitals and do not go to small ones. So, we increased the rates of the big ones. I did not increase the rate of all the companies. Only six big hospitals increased the rates. Not the smaller ones. Because of that, people started going to the smaller ones too. And my occupancy in the big hospitals also maintained. So, this is one answer. Sir, I did not increase the rate of the small ones.

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I did not increase the rate of all companies. The rate of hospitalization has not increased. Only the big hospitals have increased.

Chirag Shah: Only the five big hospitals have increased?

Manish Grover: Six big hospitals have increased. Now it has increased to seven. Gurgaon has also opened. In today's date, it has increased to seven.

Chirag Shah: Big means more than 250 beds.

Manish Grover: No. Big means more than 100 beds, rest are less than 100, some are 70, some are 50, some are 20, some are 40 and the hospitals are ready. Last year it was 2200 beds, but 573 beds around 450 beds my approval did not come from the government. So in 2 days, 4 days, 10 days approval will come so in that also the patients will start increasing. Like there is a government clinical establishment act is there in that we have to do the registration.

The doctors who are there, we have to do their local registration, for nursing registration we have to do. Government have made a new rule that whichever state the doctor works in in that state the he has to do the registration. Earlier, the whole of India used to be interested and now for state doctors registration we have to do and now we have to do the medicine registration. So we have started everything.

The way government norms are changing, we are also immediately changing, but 15 days, 20 days we are behind from them.

Chirag Shah: Sir I have understood that. Sir my second question is that what efforts we are putting that the patient inflow comes and especially the kind of patients whom we can treat 100%, so in that new efforts we are making?

Manish Grover: We have run new six verticals. We have a new slogan If you have problems with your knees You have to come once to get a knee repair. You have to come once to get a knee repair. If you are told about a knee job. You have to come once to get a knee repair. You have to come once to get a knee repair. We have started six new verticals. We have focused on knee pain, back pain, depression, anxiety. One we have focused on is infertility.

Till now, last month, in April and May, we have conceived 16 women whose IVF also failed. So this is a new line that is developing for me. Apart from IVF, Ayurvedic treatment plus knee pain, back pain, depression, anxiety. And now, next month, we are going to start sexual wellness. So the hospital beds that have been increased, we will start admitting sexual patients who have sex problems. With a 7-day package and a 14-day package which no one in India is doing yet. When a sex patient too

in medicine, I have introduced a package for the first time that you have to get admitted, get a Panchkaram done and you will get a benefit in your sexual problem.

Chirag Shah: And sir, the different types of therapies that we treat, some of the therapies will be like diabetes, or as I am giving the name where the success rate will be very good. Say, 19 out of 100 patients would be doing very well. It cannot be in certain therapies. Everyone's body is different. Every
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case is different. So where our success rate is good, there we are putting some effort, because our base is also increasing. So what new effort are we putting? More inflows and we are doing some bifurcation like this. Therapies where our success is almost 100%?

Manish Grover: See, sir, we have a very good result of the liver. Earlier, we used to do a free checkup of the liver in only two hospitals. Now, free checkup of the liver has started in seven hospitals. So my liver patient is increasing. Chandigarh Clinic, which never went above 30 lakhs, in the last month, it has crossed 70 lakhs, by starting a liver checkup there. Panchkula's new hospital, it has just started 4 months ago, 5 months ago.

It reached a sale of INR2 crores last month. The hospital in Kurukshetra has just opened, it has been 3 months now, and it has crossed a sale of INR1 crores. So we have caught new diseases, sir. We have caught knees, we have caught back, we have caught infertility, we have caught sexual, we have caught depression, anxiety, we have caught infertility.

So we are doing this, sir, we are engaged. But our aim is such that we can admit the patient, so that we increase the revenue from the service sector, in which the margin is more for us, and in which GST is also not applied. Now, the Panchkarma that we do, GST is also not applied in this. 12% GST is also used in medicines. That's why I have increased the revenue sale, I have increased the Panchkarma sale.

Chirag Shah: Right. And lastly, sir, if you have any view on team management, how have you expanded it? Because the number of hospitals is increasing, the number of employees is increasing?

Manish Grover: Within 3 months, our interviews have been finalized. In the company, CEO and COO, the interviews of both these posts have been completed. In the next 3 months, we will introduce you to both of them. We have taken a lot of interviews in that two, three of them have been shortlisted. Within a week, they will be finalized CEO and COO.

And we have appointed a doctor this week. It's been 10 days. He was in Mauritius, on the post

of Ayush Department, by the Indian government. We have involved him in our company, in training. In the training pattern. He has come from Mauritius. His name is Dr. Eish Sharma.

Chirag Shah: What is his name?

Manish Grover: Dr. Eish Sharma. He was in Mauritius, on the post of Ayush Department, by the Indian government for 3 years. After that, he didn't go in for an extension and joined us.

Chirag Shah: Okay.

Manish Grover: And in the independent board of the company, who has come now, Kaushal ji, he is also retired. He was the head of Chandigarh, on the post of Ayush Department, by the [Greek 50:24]

Department. We have included him in our company.

Chirag Shah: Okay. Okay. Thank you. The growth is coming, but to manage it, we have to make the base strong.

Manish Grover: Sir, I will call you for the next result. I will introduce you to the CEO, COO, and the director.

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Chirag Shah: Okay. All the best, sir. You are always doing a good job. All the best. Thank you, sir.

Moderator: Thank you. Next question is from the line of Nitin Gupta, an individual investor. Please go ahead.

Nitin Gupta: Acharya ji, 90% of my queries have already been answered. I just have a small ques-

tion. This

year, you said that the business of OTC, you are putting it on hold for this year.

Manish Grover: No, not for this year.

Nitin Gupta: I just wanted to know...

Manish Grover: I put it on hold for this year. It will start in 2-3 months. I have already prepared for it. But I have put it aside and focused on my ongoing business.

Nitin Gupta: Okay. I just wanted to know how big is that market. I mean, the OTC business if I say, I am saying this with respect to India as a country how big a business is this?

Manish Grover: Sir, in India last year, I had sold 14 lakh pieces of pet saffa, averagely. 8 lakh pieces of Kayam Churna were sold per month. 4.5 lakh pieces of Zandu Nityam were sold per month. Apart from this, 65 lakh pieces of altogether were sold per month. If within 2 years, I am bringing the Pet Shuddhi Kit, the Pet Liver Shuddhi Kit. Apart from this I had taken out one more data, in total, 4.5 crores pieces of Liv 52 were sold per year. Do you know Liv 52? 4.5 crores pieces of it were sold per year. That means 40 lakh pieces per month.

My product is a combined product of Pet and Liver. If I sell 5 lakh pieces per month on average, then I will get a profit of INR10 crores from that. I have calculated that if I sell 5 lakh pieces per month, then I will get a net profit of INR10 crores per month. Within 1-2 years. This is such a big market. Let me tell you one product. One product is Nutri Roz. I have launched one product, a water machine. It has also sold INR1.5 crores in the last month.

I have launched a machine to make alkaline and magnetic water. The machine in the market, Kang en Water, is a basic model of INR2,70,000 lakh. I have launched a machine of INR15,000. It makes alkaline water with a magnet. In the first month, we sold it for INR1 crores. In the last month, we sold it for INR1,70,000 crores. Now, I have only run ads on social media. Now, I am going to apply it in my clinics. Every doctor will give a demo to every patient. 40,000 patients are taking medicines from me every month.

If 10% of those patients convert, 4,000 pieces of mine are sold extra sold every month. Then just imagine get sold then my revenue of INR6 crores will increase. What is my margin in this? I have a margin of more than 50%. The machine is worth INR2200. I am selling it for INR15,000.

Nitin Gupta: Sir, you have already given a level of answer with respect to ARPOB, which is currently in the financial year 2024-2025. I have read all your calls and participated in them. One of your important points that I liked was that we don't want to stretch it too much. You said this in the last call, which I also find very positive. If I talk about 3-5 years, the current ARPOB is of INR8200, where do you see it will go? Will it remain the same? It will go up or down?

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Manish Grover: It will be the same. I don't take a lot of money because I get a lot of poor people. I get poor, weak, middle class people. I don't have high-end customers. These people become poor due to treatment. I am telling you that in every hospital, almost 10% of the patients get free treatment. We don't take money from them because they come to us so poor. The poor person is looted in allopathy or his wrong treatment is done or some part of his body is cut. We don't take money from them. You think we are getting this profit by treating them for free.

If I remove free treatment, then my average will be 85-87 instead of 8200. How do I forget my

responsibility? I am also an Indian and I have to help people. I am not in favor of increasing the rate. I increased the rate by 15% on April 1 because all the big hospitals were coming. Now, the average is set between big and small hospitals.

My Patna hospital went on a sale of INR1 crores last month. My Kolkata went on a sale of INR90 lakhs. Earlier, it was

going on a sale of INR60 lakhs. As soon as I increased the rate on big hospitals, I didn't increase the rate on small hospitals. So, the patient started diverting on small hospitals. This is why my sale on April 1 was INR60 crores.

Nitin Gupta: So, overall, our target is to maintain a margin of 25%...

Manish Grover: I will try to maintain a margin of 20%, I will try to bring it to 23%-25% next year, because I won't be able to invest. Depreciation of investment has also come. Now, I have changed the model. I won't be able to invest. I will invest in colleges' hospitals which are closed of 100 beds, not running. Why should I invest in them? I will take them only to run. I have an agreement with two hospitals in this week at Mohali. In a month and a half, I will have an agreement with Saraswati in Vrindavan, Mathura.

Nitin Gupta: Sir, will we be in control of management? I mean, which has to be administered, I mean, will there be anybody for our side...

Manish Grover: Everything will be in our control. We are giving them rent as well, rent and revenue sharing. When rent is imposed all together. There will be a big benefit which I forgot to tell you all. The colleges' hospitals which we will take, we will get a class A quality manpower. Because the 100 students who leave the college every year as doctors, we will already give them 20-30 out of 100 to join our company. Our team will identify that these are 20 out of 100 diamonds. These are 30 diamonds. Catch them. Now our manpower will be better. And we will get trained students.

Now we have to train doctors. It takes 3 months from our pocket. It takes 3 months for a doctor to get trained. Then we will get trained doctors from the first month, because in colleges, every year 100 students leave one college by doing BMS, MD. So we will get those doctors very easily.

Nitin Gupta: Yes. Thank you, sir.

Manish Grover: Thank you, sir.

Moderator: Thank you. Next question is from the line of [Vivith Agarwal 58:24] from FICOM Family Office. Please go ahead. Mr. Agarwal, we are not able to hear you clearly.

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Vivith Agarwal: Greetings, sir. Sir, I wanted to know what will be our capacity utilization in FY '26?

Manish Grover: What will it be?

Vivith Agarwal: Capacity utilization.

Manish Grover: Sir, I want it to be 70-80. That is why I will increase the new beds for the poor. I have increased the beds for the rich. Now I will increase the beds for colleges hospital. We hope that the rate will also reduce, because we are planning to keep average 6-7,000. So we hope that 52 is still going on. I will maintain it till 60-65, because there is never a 100. Because there are some emotions associated with us.

For example, the patient has a check-out at 12. The patient says that his train is at 11 at night. So we allow him to stay in the room. But he says that his room doesn't fit. So my main hospital is going up to 80-85. My target this year is that all the small and big hospitals should reach 60%-70% occupancy. Average.

Vivith Agarwal: 60%-70% Sir, you will also add something, like 800 beds. 60%-70% on what basis? What will be your levers?

Manish Grover: Sir, this is a very complicated question. I will tell you the reason. This year, my beds were 2173, when the 31st March ended. But my operational beds were 1600. 573 beds were added. But I was not able to use them, because of their approvals, their NABH, their registration. For example, now there are 250 operational. 300 will be added in the next month. So this is a challenge. It is a running company. So every time you have to look at the numbers. That's why this time I took out your beds at 1600, utilization. If I do 2173 then it will be 38%, 40%. But when it is not being used how do I count it? Like I said, I will add about 800 beds by 31st March 2026. Now the

800 that will be added may be 200-300 will not be operational. So I will do one thing. I will make a report like this, operational and non-operational. I will show the reports of both separately.

Vivith Agarwal: Sir, it will be very good.

Manish Grover: No, this time I have shown it like this. If you check, I have shown it like this. You notice. I just said like this, there are 2173 total beds. Out of which operational is 1600, and occupancy is 850. And this time my average revenue for 6 months per bed is 8200. I would like to tell one more

number. In 2023, in total 1,57,000 patients came to take medicines. In 2024, 2,52,000 came. In H1, in total 1,57,000 came in the first 6 months. And in the whole year, 3,37,000 came. In the next 6 months, 1,80,000 came to take medicines.

In IPT 2022, there were 57,000. In 2023-2024, there were 13,000 in total. In H1, there were 12,000. In H2, there were 12,500, admission. And I had doctors, in total, 230 in 2023. In 2024, there were 307. In H1, there were 411. In H2, there were 480. And the total number of employees ended in 2025, there were 3,700. In H1, there were 3,300. In H2, there were 2,500. And in '23, there were 2,400. So we are doing regular growth.

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And we have also finalized E&Y. As an auditor, as an auditor, we have talked to E&Y.

Negotiation is going on. And as soon as it comes to the main board, E&Y will be our statutory auditor. We have talked to them. As soon as the first 3-month balance sheet comes, the next balance sheet will be from E&Y. And E&Y has been with us for a year on management review.

Vivith Agarwal: Okay. That will be good, sir. Thank you, sir. Thank you so much.

Manish Grover: Thank you.

Moderator: Next question is from the line of Keshav Lahoti from HDFC Securities. Please go ahead.

Keshav Lahoti: Namaskar, sir. Just tell me one thing, your trade receivable was INR90 crores in FY '22. In FY '24, it was 411 crores. In FY '25, it was more than double, INR 98 crores. In FY '25, it was INR98 crores. Last year, it was INR42 crores. Why did your trade receivable jump so much?

Manish Grover: Sir, the government business that we used to do, I have reduced it since April. In March, I did a business of INR11 crores with them. Now, I have done a business of INR6 crores. I have put an email everywhere that if you give the previous money first, we will work later. The email reply came from 3 places that you don't stop the work, we are giving the payment.

This payment comes regularly. It is a cycle of 3 to 6 months. Some payments come up to 8 months. If you check, after 31st March, this year, INR6 crores payments have come from the previous month. Now, I have reduced their trade. My business, the INR11 crores business that I did in March, in April, I did a business of INR6.9 crores with the government. I said that if you give the previous payment first, we will work later. Their email reply came that we are giving you the payment.

We work with 3 people. One is CGH S, Central Government. One is CAPF, Central Armed Police Force s. CAPF got the payment 4 days ago from the government. 4 days ago, CAPF got the payment from the government. This happened after our email. I have sent the patient back that we are not taking your patient. First, give us the previous payment. Sir, the payment will come regularly. It is a regular cycle.

Keshav Lahoti: It will come. There is no problem in the payment.

Manish Grover: It is government's money, sir. It's government's money.

Keshav Lahoti: Understood. Sir, tell me, what will be your capex this year?

Manish Grover: Capex? We don't need capex now. We won't need it now. Because all the hospitals are full...

Keshav Lahoti: There will be a little bit in the bed. Your bed addition is still going on.

Manish Grover: It won't be much, sir. I

have changed the strategy. Because the bed that we are installing now, we have started taking beds of colleges.

Keshav Lahoti: Okay.

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Manish Grover: In India, there are a total of 600 colleges where 100 students graduate every year as doctors. I have been finalized with 2. And we have 4 or 5 more who have applied. So, according to our location, where do we need it? We will take their beds this time. And the expansion of the call center was completed last year. It won't be done this year. Earlier, my call center was of 450 seats. Now, it is of 800 seats.

So, the money that had to be spent on that has already been spent on 31st March. And it has become personal. So, on 31st March, we have induced a capital of INR42 crores. So, it won't take much this year.

Keshav Lahoti: Understood. Thank you, sir.

Moderator: Thank you. Next question is from the line of Gaurav Didwania from Qode Advisors LLP. Please go ahead.

Gaurav Didwania: My question is that you mentioned that there were 2200 beds operational. But in 1600, there was an approval. And in the rest, there was an approval pending. So, has the approval been done or is there something pending now?

Manish Grover: About 250 beds have been done.

Gaurav Didwania: Okay.

Manish Grover: So, if I calculate today's operational, then today's operation will be about 19, 19.5 in today's date.

Gaurav Didwania: Okay. And how long will the rest be? Do you have any such timeline?

Manish Grover: Sir, it is going on regularly. For example, someone's fire certificate is pending. Someone's air pollution is pending. For example, if a fire is already found, then it is pending again. It is a regular process, sir.

Gaurav Didwania: Yes, that's right...

Manish Grover: There is no work in it. We run OPDs and do day care. We don't admit the patient until the government approves it. If I admit him without the NOC of the fire, then there will be a problem in the future.

Gaurav Didwania: That's right.

Manish Grover: So, everything else is ready. For example, my fire NOC did not come in two places. So, I stopped the owner's rent. I said, I won't pay the rent until he pays it. Now, he is paying it soon. We are doing business practically on earth. So, we face many challenges. I have inaugurated a hospital of 100 beds in Gurgaon on 16th May. Acharya Lokesh Muni is a Sadhu of Jain community. He inaugurated it on 16th May.

Gaurav Didwania: My second question was, the government panel that you mentioned and the receivables are increasing due to it. Your maximum growth is due to the government panel. Earlier, it was 16% of revenue. Now, it is 25% of revenue. So, it has more than doubled ...

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Manish Grover: In the last month's sale, I increased it by 11%.

Gaurav Didwania: So, it must have brought a drop. 50% of the revenue is government revenue ...

Manish Grover: No, sir. Wait a minute. Let me correct it. In the sale of INR470 crores, the government revenue is INR117 crores.

Gaurav Didwania: So, both services and products are included in this INR117 crores?

Manish Grover: Everything is included. Everything is included in the government. So, in INR470 crores, how much percentage is in INR 117 crores. It was 24% last year. This time, I increased it by 11% in the last month, in April. I reduced it. Until I said that I won't work Sir, the big government officers of DIG level, DGP level, big levels, I stopped their admission. Everyone filed a complaint. The government approved that we are paying your payment.

Gaurav Didwania: Sir, how much is the receivable of the government, around INR92 crores?

Manish Grover: It is not INR90 crores, sir. It is around INR82 crores. It is visible on 31st March. Because in the last 6 months, I did a business of INR65 crores. There is a cycle of 6 months.

hs.

Gaurav Didwania: How much is the government's receivable out of INR97 crores?

Manish Grover: INR92 crores is the government's receivable.

Gaurav Didwania: Okay, then 95% ...

Manish Grover: Sir, out of INR92 crores, INR65 crores was booked in 6 months. From October to March, in 6 months. And when the financial year ends, their payment does not come. When the payment comes, the government department collects it.

Gaurav Didwania: Okay. But still, our revenue run rate has reached INR60 crores, even after deducting this?

Manish Grover: Yes. Not by deducting this, but by including this, INR60 crores has come in the last month. If I remove the government, then my private sale is INR54 crores.

Gaurav Didwania: Okay, sir.

Manish Grover: So, I have reduced the dependency on the government.

Gaurav Didwania: You are right, sir.

Manish Grover: I will do it when they give me money. I have said that I will not do business until you give me the last payment. Instead of this, I will reduce the rate on my vacant beds and pay them.

Gaurav Didwania: You are right, sir. Sir, the second thing is that I can see medicine order volumes. My understanding is that this is the revenue of the products. So, last year, the medicine order volume this year, it has become INR4.5 lakhs. But our product revenue is not that much. Can you tell us what this is?

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May 20, 2025

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Manish Grover: Sir, we have started a new business model. Earlier, we used to do COD business. We have almost finished the COD business. We have started prepaid. We have created a page in the name of Jeena Sikho and made an e-commerce website. We have put this product on e-commerce. So, we have shifted the business. We have started ordering single products.

So, the quantity has increased. Earlier, it was not INR2 lakhs. Now, it is INR2 lakhs in total.
There are 4 medicines in that order. Now, a patient can order a single medicine from the website.
So, the number has increased.

Gaurav Didwania: I understand, sir.

Manish Grover: Earlier, we were not available on e-commerce. Now, we are available on e-commerce. So, this is the issue. The number of patients has increased.

Gaurav Didwania: Very good, sir. Thank you so much for taking my question and all the best for the future. Thank you very much, sir.

Moderator: Thank you. Next question is from the line of Abhishek from AB Capital. Please go ahead.

Abhishek: Sir, I have a few questions. How much is the payment for cashless insurance?

Manish Grover: Sir, the cashless insurance comes in advance within 7 days. The reimbursement comes after the payment. For example, if we take the cash approval from the company then the payment comes in 7, 10 days. Apart from this, we also have a loan facility. If the patient can't pay, then we use Z-money from a company and they pay us. They take 6% commission and the patient pays them in 12 installments or 6 installments.

Abhishek: Okay. And how much is the payment from the government?

Manish Grover: Sir, earlier, it was a 3, 6 month cycle. Now, this is the first time it has been delayed. So, we have put a notice to the government that if you pay now, then we will do business in the future. So, we have got a mail from the government. We have also spoken to the Delhi Government. We have also spoken to the Central Government. Everyone is saying that it will be cleared within 3, 6 months. Within 3-6 months, it will be cleared within 18, 19 months.

There are 15-20 10% bills in which queries are put. We have put a bill. We are answering their questions. So, we hope that there will be a maximum recovery in 3, 6 months. You keep asking these questions again and again. Tell me, should I do business or not? I will do business now. Last month, I did a business of INR6.9 crores.

So, INR6.9 crores will increase again. There will be a booking of

INR6 crores, INR 7 crores this

month. Sir, this will run in a regular cycle. The payment will come and then I will do business.

Or I will stop it completely. So, according to me, according to the payment, I have made up my mind that I will do business according to the monthly payment. The average will be maintained.

Abhishek: Okay, sir. Thank you.

Moderator: Thank you. Next question is from the line of Shalin Parikh from MYCPE ONE. Please go ahead.

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Shalin Parikh: My name is Shalin Parikh. Sir, first of all, thank you. You told me that you are appointing a CEO. Yes, I am appointing a CEO and a CEO. Definitely come to the call. No one else will be able to say what you say. So, please come ...

Manish Grover: I can introduce everyone.

Shalin Parikh: Absolutely, sir. Sir, you talked about OTC. You told me that some regulatory changes have come. Because of which you have delayed the launch of OTC. In the last call, you also told me that there are some trials going on in ICMR? If you can throw some light on that?

Manish Grover: Yes, sir, the trial of 5 products was going on. In March, about INR3 crores of sales were booked from those products. In March, INR55 crores of sales were booked. So, out of INR55 crores, INR3 crores were from those products whose clinical trial has been completed. In October, there were INR35 crores of sales. In March, there were INR55 crores of sales. So, those products played their role. In March, they booked INR3 crores of sales. And INR1.7 crores of sales came from the water device I launched.

Shalin Parikh: Okay. Sir, are we planning to bifurcate our sales and see how much is prescription-driven and how the patient purchases...

Manish Grover: Let me tell you something. The INR469 crores revenue in which the medicine sale of the whole company is INR215 crores. The one which was admitted in the hospital and the patient got treated is a total of INR136 crores. And the government's panel is INR117 crores. So INR117 crores plus INR136 crores plus INR215 crores, so the total is INR469 crores. Now I have to bifurcate this INR215 crores for you.

How much did it come online and approximately? The one that came online is approximately INR4.5 crores 1 per month. The average per month is approximately INR4.5 crores. That means INR50 crores is online. The remaining INR165 crores is from the prescription. Means the one that is sold on the doctor's shelf from my clinic.

Shalin Parikh: Yes, absolutely. So our prescription led growth will be there. Our organic brand will be there. Our individual. We are also going on umbrella branding. That we will learn to live with all medicines ...

Manish Grover: Sir, I have launched some new things. I have also launched Honey. I have also launched Chyawanprash. I have also launched the first Ayurvedic Toothpaste of Ayurvedic. Ayurvedic

India's first Ayurvedic Toothpaste. In which there is no SLS. There is no SMS. So I would like. Whoever is our Ranvir ji. I will send him 100 packets in the next 15 , 20 days. So that you can also spread it to everyone. Our Honey. Our Chyawanprash. Our Toothpaste. So that you can also use it. Now we have started giving it to the patient in our hospitals. Shalin Parikh: Thank you sir. You are on a very good mission. My best wishes to you and your team. Manish Grover: Thank you sir. Jeena Sikho Lifecare Limited May 20 , 2025

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Moderator: Thank you. Ladies and gentlemen. That was the last question of the day. I now hand the conference over to Mr. Ranvir Singh from Nuvama Wealth and Investment Limited. Over to you sir.

Ranvir Singh: Thank you all. Thank you for participation. And thank you the management. Sir Acharya ji. Would you like to say something? Closing remarks. To all our participants.

Manish Grover: Thank you all. And as my mission. Our company's mi

ssion. That in India. First of all, no person

should fall ill. If he falls ill. Then first take the support of Ayurvedic . Later he goes to other positions in an integrated way. For the support of that Ayurvedic . To increase Ayurvedic . I started this Jeena Sikho Company.

And you all are contributing. Thank you very much for that. And you continue to contribute. So that in the whole world. We can support the Ayurvedic . Can stand as a very big organization in India. That the world recognizes India . Because of Ayurvedic . If India becomes a world guru. Then according to me. It will be because of Ayurvedic in India.

So if we are authentic. Proving Ayurvedic scientifically. Approximately 370 clinical. 370 research papers have been written. Of which 70 have been accepted. 17 have been published. In which kidney fail. Cancer and liver patients. If you say that too. I will provide you. So we are talented. To validate Ayurvedic . And are busy in moving the company forward. You all gave time. Thank you very much.

Ranvir Singh: Thank you. Moderator we can close the call now.

Moderator: Okay sir. On behalf of Nuvama Wealth and Investment Limited . That concludes this conference. Thank you for joining us , and you may now disconnect your lines.

Call: Aug 2025

Date: 21st August, 2025

To,
Manager - Listing Compliance
National Stock Exchange of India
Limited 'Exchange Plaza'. C-1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai - 400 051
SYMBOL: JSLL To,
Head of the Department,
Department of Listing Operation,
BSE Limited
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai 400001
SCRIP Code: 544476

Sub: Intimation to Stock Exchange – Transcript to the Conference Call for the quarter ended 30th June, 2025

Dear Sir/Madam,

Pursuant to Regulation 30 read with Schedule III and Regulation 46 of the Securities and Exchange Board of India (Listing Obligations and Disclosure Requirements) Regulations, 2015, as amended, we hereby inform you that the transcript of the Conference Call held on 18th August 2025 to discuss the Financial Results of the Company for Q1 of FY 2025-26 has been made available on the Company's website.

The aforementioned transcript to the conference call is available on our website at:
<https://jeenasikho.com/wp-content/uploads/2025/08/Jeena-Sikho-Q1FY26-Earning-Call-Transcript.pdf>

Kindly take the above intimation on your record.

Thanking you,

Yours faithfully,

For Jeena Sikho Lifecare Limited

Manish Grover
Managing Director
DIN: 07557886
Place: Zirakpur, Punjab
Date: 21-08-2025

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"Jeena Sikho Lifecare Limited
Q1 FY '26 Earnings Conference Call "
August 18, 2025

MANAGEMENT : MR. MANISH GROVER -- MANAGING DIRECTOR -
JEENA SIKHO LIFECARE LIMITED
MR. NANAK CHAND - CHIEF FINANCIAL OFFICER -
JEENA SIKHO LIFECARE LIMITED

MODERATOR : MR. RANVIR SINGH – NUVAMA WEALTH AND INVESTMENT LIMITED

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Moderator: Ladies and gentlemen, good day and welcome to Jeena Sikho Lifecare Limited Q1 FY '26 Earnings Conference Call hosted by Nuvama Wealth and Investment Limited. As a reminder, all participants' lines will be enlisted on remote. There will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star, then zero on your touchtone phone. Please note that this conference has been recorded.

I now hand over the conference to Acharya Manish Grover, Managing Director of Jeena Sikho Lifecare Limited. Thank you and over to you, sir.

Manish Grover: Namaste and welcome everyone. I am very happy to tell you that this month, on 11th August, our company has successfully migrated to the main board of NSE and BSE. Along with that, I am happy to share that Jeena Sikho Lifecare has achieved a strong financial and operational performance in Q1 FY '26. Our continuous efforts of excellence have yielded excellent results, and we have seen strong revenue growth in both services and products segments.

Revenue from operations in Q1 FY '26 was INR174 CR, while in Q1 FY '25 it was INR100 crore and in Q4 FY '25 it was INR139 crore. This means that our growth was 74% year -on-year and 25% quarter -on-quarter. Our profit after tax (PAT) was INR51 crore, which saw an increase of 218% year -on-year and 88% quarter -on-quarter, and our EPS was INR4.13. This growth is mainly due to the strong traction of private Panchakarma services and medicines.

Patient volumes are increasing continuously.

In OPD, we have recorded an increase of 70% year -on-year and in IPD, we have recorded an increase of 45% plus year -on-year growth. Our strategic focus has been to increase the expansion of products and hospital, clinics and daycare footprint. We have seen an increase of 74% year -on-year growth in both segments.

This exponential growth is a proof that we are committed to providing quality, alternate healthcare services to people on a large scale. Our services footprint has also increased significantly. Our current 2,180 operational beds have been added to 390 beds, and the total capacity has increased to 2,570 beds.

Our focus is to contribute to the growth of corporate health and India, which will be our main growth. We will take the health insurance of corporate companies together. Our reach has now spread to 100 plus cities and 23 states. And, the patient response from recently opened hospital has also been very good.

Our EBITDA has been INR79 crores in Q1 FY '26, which shows 220% year -on-year and 69% quarter -to-quarter growth. Our profitability margin has also been around 45%. These results underline our ability to maintain operational efficiency and drive business expansion.

Finally, I would like to thank our team, patients, shareholders, and all of you and all the people who trust us. We are committed to making India's most respected Ayurveda -based healthcare platform, which will help the society, stakeholders, and the country move forward. For both, I want to work as a value driver.

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I would like to invite our CFO, Mr. Nanak Chand.

Nanak Chand: Good afternoon, everyone. Before I present Q1 FY '26 financial performance, this is to bring to your notice that we have adopted Indian AS with effect from FY '25 and hence FY '25 financial number has been restated as we have migrated to main board Stock Exchange . We have started reporting quarterly performance from this quarter.

Now, on the Q1 FY '26 performance, revenue grew by 74% Y-on-Y basis to raise INR174 crores in Q1 FY '26. As each of the products and service segments grew approximately 74% Y-on-Y basis. Services segment contributed 54% on revenue during the quarter.

The EBITDA during the Q1 FY '26 jumped over 3x to 220% Y-on-Y to INR78.8 crores by implying the EBITDA margin 45%. The Q1 FY '26 PAT grew by 3x or 218% Y-on-Y to raise INR51 crores .

With this, we open the floor for a question -and-answer session. Thank you.

Moderator: Thank you very much. The first question is from the line of Sucrit D. Patil from EyeSight

FinTrade Private Limited . Please go ahead.

Sucrit D. Patil: Good evening to the Jeena Sikho team. I have two questions, one for Mr. Grover and one for Mr. Ch and. Sir, I have a question for you. Jeena Sikho is expanding its Ayurvedic portfolio and wellness reach. So, what is your next big step , in which direction are you thinking? Especially regarding personalized diagnostics, AI -led treatment protocols, or any subscription -based preventive care integration, do you have a plan to mak e Jeena Sikho a tech -enabled, outcome -driven wellness platform that can go beyond just products and clinics and plan innovation and business development? Thank you.

Manish Grover: Namaste, sir. Thank you very much. Have you seen our PPT?

Sucrit D. Patil: Yes, I have seen it. But I want to understand...

Manish Grover: So, you must have seen in the PPT that I have submitted 44 -48 research paper. I have given clinical trial results to Mr. Nanak today. We have conducted clinical trials of a lot of products that we are going to launch in OTC. And on 22nd, 3 days from now, we are launching our own OTC in Lucknow. The name of the product is PET. So, we are launching a platform for OTC. We have launched a product called PET Liver Spleen Shuddi Kit. For the first tim e in India, we have started the arrangement for a healthy person to stay healthy. We have started the arrangement for a healthy person to stay healthy.

We have launched a product for INR960. And the population of India is 140 crores. And there are about 90 crore people in India. And it is a product for every person. We have used a shloka from Ayurveda. We have used a shloka from Ayurveda. I will tell you. Everyone listen.

Ayurveda says, Sarve roga, api mandagni hi. That the main cause of all diseases is a weak fire. That is, weak digestion, weak liver, weak spleen, weak pancreas. That is, slow metabolism.

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Ayurveda says, when the metabolism is slow, the food is not digested properly, then there is a weak fire. And the body collects waste. So, the shloka is , Sarve roga, api mandagni hi. Ajarne kupit mal sanchate. That is, it becomes indigestion. And it becomes sanche after being kupit mal. So, this is the root cause of every disease. So, when we treat this disease, then the solution will come out.

You asked the tech -based question. So, now we have enabled the entire Salesforce software in our company. We have changed our entire CRM. And by working on the AI chatbot, we are making some such small apps. We are making a BP app for the patient. We are making an app for sugar.

Apart from this, we have also made a product of depression and anxiety. We are also making an app for that. So that the patient asks questions in it , get the answer in it and the medicine is recommended from there.

Apart from this, we have tied up with two new labs. In India, in cancer tests, there was only biopsy, PET scan and mammography now we have tied up a new technology HRC and blood -based biopsy with a company in Mumbai where cancer testing is done without tearing. So, we have tied u p with them to do more such researches.

You find such things that the patient's testing is without side effects. Now people do biopsy and spread cancer. Now people increase cancer after mammography. So, we are working on prevention in this. We are also wo rking on tech -enabled system. And we are also working on treatment.

And I will make a request to all of you. That today the results a

re very good. I am telling you a

number. Everyone note it down. 7528975283. Everyone put your address on WhatsApp of this number. So, the clinical trial -based product that we have launched. We will courier it to all of you s o that all of you, investors and shareholders, we will send it free.

So that you use Ayurvedic products a nd see the quality of Ayurvedic products. And th e Shloka that I told you. Sarve Rogapi Mandagini . And see the Shloka practica lly. How you will use that kit a nd you will get the results. And what treatment did Ayurveda tell? Sarvesha Naam, Roga Naam, Nidanam Kupitamala. That all the diseases have to be treated.

And all these products are evidence -based and research products. And all AI technology, software. We are starting all this. We are doing all the homework. Every month there is R&D team.

Sucrit D. Patil: Thank you very much Mr. Gr over ji. This was a very good guida nce. And just to touch the base t he number that you just shared. So on this number y ou should just send the WhatsApp?

Manish Grover: Yes, put WhatsApp on this. So that we will courier the product to all of you. We will do the delivery; home delivery of the product will come to your home. This is a small kit. It has a powder that needs to be taken for 6 days. You have to eat powder for 6 days. You don't have to eat powder on the 7th day. You have to drink a shot of 30 ml on the 7th day.

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Which is made of A maltas, Amla, Harad and Ajwain w hich is important for our digestion, fire and digestion b ecause some people do not feel hungry. Some people feel more. Some people have liver fatigue. Some people have constipation. So all the products work t ogether on all diseases.

Sucrit D. Patil: Thank you very much for the guidance and congratulations to you and your team or the future progress.

Moderator: Thank you. The next question is from the line of Akshay from AK Investment. Please go ahead.

Akshay: Hello. Good evening . First of all, congratulations f or the best ever first quarter per formance.

My first question is t he Ayurvedic medicines that we have launched recently t he combination of stomach and liver. There were two, three medicines. What was the traction in that? I think we have launched it a month ago. What was the traction in that?

I think it was the first fraction and the quarterly performance in the first quarter w e have done a very good performance in EBITDA margin. It has improved signific antly. So will EBITDA margin be sustained for this year and next quarter, w ill it be sustained for the next year? Will it be sustained for the next year a nd will it be sustained for the growth and our products, which was my first question, what is the reac tion to that ? That is my first question?

Manish Grover: I will answer both of your two questions. Number one, we launched the product exactly 15 days, 17 days ago and what we did in the first one hour, we got about 2 ,200 orders. And in four days, our first lot of product was completely over. So I deliberately put a gap of 20 days for the second batch.

The second batch reached me yesterday. I deliberately did not give any product again for 20 days, for 18 days. And what I did, the people who had taken the product, first they took their feedback. We had already done its clinical trial, but still I called everyone again and took feedback. So we talked to about 650 people.

Out of 650 people, about 25 -26 people made their video testimony and sent it. They say that till date, this is the best product of life that we have used. This is the feedback of pe ople and doctor Neha Sharma is also sitting with me, who is in our main Board team. So the feedback of the product has been super -duper . And som e ministers, some IPS officers, some big officials, I also gave.

So that we spread Ayurveda all over the country. So

now we are launching official on 22nd. So

our first distributor, which is a super distributor of UP of Lucknow became. And we have kept dinner there a nd super schools of every state of India are coming. So we are calling about 60 people from all over India on 22nd in Lucknow.

On the morning of 22nd, there is a press conference at 12 o'clock. All the newspapers are coming from all over India . At 4 o'clock, we have kept an influencer meet a nd we have called a lot of big influencers in it. In the evening fro m 7 to 10, we have kept dinner t he super distributors who have to become.

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And from them, we have the first che que of INR1.01 crores came 1 5 days ago. Now about 16 parties have been finalized, which will become super. So this material will be available at every medical store and there will be three, four variants will come . Now we have taken out the first variant of INR960. One such will be o f about INR1,900.

One such will be of INR2,400. Along with this, medicine of BP, medicine of sugar, medicine of kidney, med icine of liver, m edicine of d epression, medicine of anxiety, medicine of sexual wellness w e are going to launch all this. We are als o going to launch blood purifier. This is one thing. Now the second.

I would like to tell you that the allopathy hospitals in India i ts market is of INR7 lakh crores.

And the pharma market is of INR2 lakh crores, and I am planning a turnover of INR700 cro res this year a t the same run rate, which is running at a run rate of INR60 crores. I am planning to maintain this with my hospital and clinic business.

So my turnover is of INR700 cror es. So let's see, this is 0.1% for t he who le company, the whole indust ry. You think in front of you growth is there a nd the second INR2 lakh crore s market is of pharma, I will take it to the turnover of INR500 c rores in the first 1 to 2 years of medical business, w hich will be 0.025% of the entire market.

So you yourself think how much growth can be. We have not started anything yet. W e are at the tip of the needle a nd we have to go on the ship. Now you said that we will maintain this

margin. So if you look. So what is my bed ticket per bed which is my revenue it was INR8, 200 last year after H2 and in H1, it was INR 8,100.

In 2024, it was INR7,900 and in 2023, it was INR6,100. So financial now in quarter 1, this is INR8, 280. We did not increase this. But the occupancy has increased. In our FY25, I had an operational bed of 1600, and the occupancy was 850, i.e. 53%. Now in quarter 1, my operational bed is 2,180. So according to that, the occupancy is 57%, but if I look at the last occupancy, I will take it at 1,600. So I have 80 occupancy. So now if I just increase the occupancy then also my profit will increase.

But I started a new work. I don't want to take a lot of profit. I have to maintain the profit for 20% to 25 %. So I started a new work in every hospital, we started a weaker section for the poor. 10% free treatment. So in the last three months. The number of patients we treated in the hospital.

So we did not take 10% money from the patient. We started free treatment for the poor.

Because they are becoming our mouthpiece, they are becoming our spokesperson, they are telling people about us, their videos are being made. The footfall of the patient is increasing.

So our ad expenses are reducing. So we helped the society in a way.

By treating 10% people for free. And we also arranged the ad for ourselves for free so that our ad budget also reduces. And in this quarter, we have also reduced the budget of the ad in this quarter compared to the last quarter and the ad budget that we have to use the ad with these patients and prepared the videos which is free of cost. So we are planning to maintain 20 % to 25% which is an old existing business. If it comes more,

then it is very good for all of us.

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Akshay: Sir, I am very happy to know this. So sir, we are doing a very good performance in medicines and what is being made in lots. So we have to scale up that. So we have enough partners for that manufacturing partners like we do contract manufacturing. So we have special arrangements for that?

Manish Grover: Sir, all the contracts have been made. Agreements have been made. We have made agreements in four different states so that we do not have to spend like I have to deliver in Punjab and Haryana. So I will get it made in Punjab, like if I have to deliver in UP, then it will be made in UP. If I have to deliver in Maharashtra and Gujarat, then the product will be made there. We have made this kind of planning and the language there will also be different. Like in Maharashtra, we will write Marathi together on the box. In Gujarat, yesterday our Gujarat has also been made what is it called super Distributor. And they have given a cheque of INR11,01,000 in advance yesterday. Now we will pick up a product worth INR1 crore s from them.

So by doing this, in the next 15 to 30 days, we will launch this product in three states. And in the next 2 months, in about five to six states. We will launch this product. First, we will do one product and in our pipeline, there are a total of 15 products. So in the next 1 to 1.5 years, we will launch a total of 15 products. And this business, my target is INR500 crore s turnover. And I will maintain a margin of 20% in this also.

So as big as Jeena Sikho is now such a big margin will come from this OTC market only. And OTC and our whole business is clubbed together. For example, the one who will buy a product in OTC. The hospital details will come out in his box. The clinic details will come out. So that clinic will move from there.

Our whole business will complement each other. That is the patient here and there, the patient there and here. All together because the purpose is the same. So that no one gets sick in India. And if someone is sick, then his first choice should be Ayurveda, naturopathy and home kitchen.

Akshay : Okay Sir. And Sir, my second question was that in the Ayurvedic sector , like all the hospitals, Generally, we have seen that the old age people like people above the age of 50 years to 60 years, they come more. So according to your knowledge, what is the data of our hospital? In that, people above the age of 50 years, 60 years come more or young patients like me, people of 25 years, 30 years such people also visit, and such people also adopt Ayurvedic therapy. So what is your opinion in that, sir?

Manish Grover: Sir, now people of 25 years to 30 years do not come. They come about 2% to 4%. Now we have people above the age of 40 years and mostly we have chronic patients. Who have been taking blood pressure pills for many years, taking diabetes pills, taking thyroid pills. They come to get their pills released and come to detox.

And now the wellness that we had earlier it was 2%, 3%. Now it has become 10%, 12%. That is people have started coming to us for wellness even without disease. That now our data has

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become 8%. And the biggest thing because of being in health insurance, as people are getting to know that Ayurveda is also in health insurance.

So the patient of health insurance has also increased. Due to which our profitability has also increased. And the patient footfall has also increased. I will tell you the number. We had a total of 5,700 patients admitted in 2023. In 2024, a total of 13,000 patients were admitted.

In 2025, a total of 24,500 patients were admitted. Now we have admitted 8,600 patients in the quarter 1. That is, if you look at it in this way then an average of about

34,000, 35,000 is made.

And in the OPD, In 2022-23, 1,57,000 patients took medicine. In 2023-24, 2,52,000 people took medicine. In 2024-25, 3,37,000 people took medicine.

Now in the quarter 1, 1,25,000 people took medicine. In 2023, we had 230 doctors. In 2024, there were 300. In 2025, there were 480. Now there are 532 and the number of employees was also 2400 in 2023. In 2024, there were 2,500. And in the financial year 2025, there were 3,700. Now there are 4,000 employees.

And our hospitals also increased from 50 to 55 in 2025. And our NABH was 41 in 2025. That also increased to 45. And the number of operations is 2,180. By the way, the total number of beds has become 2,570. And I had talked about doing 2,850 beds by 31st March. I have already done 2,570 in three months because we have tied up with Ayurvedic colleges. So we have started getting hospitals without spending.

For example, our Mohali hospital has started. The Saraswati group of college that we took in Khara r. And our second hospital that is being built in Vrindavan 150 beds that will also start next month. And the one in Aurangabad will start in about three months. After that, we will take the hospitals of Ayurvedic colleges.

Akshay : Okay, sir. All right. Thank you so much sir and all the best for the future.

Moderator: Thank you. The next question is from the line of Abhishek from AB Capital. Please go ahead.

Abhishek: You said in the last quarter that you will tie up with medical colleges. So how big is that business? I mean, how many medical colleges have been empanelled so far?

Manish Grover: We have tied up with three so far, sir. The first is Saraswati College, Mohali, Khara r. The second is Sanskriti University, Vrindavan. The third is a hospital of 100 beds in Aurangabad. My plan is to build 10,000 beds in the next five years, sir. We will build 7,000 to 10,000 beds in three to five years.

Now there are 19 lakh beds in India, sir. Of which 11 lakh are private, and 8 lakh are government. In which the average occupancy is 60%, 70%. And I have only 2,180 operational beds. So now the whole ocean is left, sir.

Abhishek: So how big business will you make in 2-3 years, sir a broad view?

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Manish Grover: Sir, you have to calculate the money. I will tell you that in 3 to 5 years, there will be 7,000 to 10,000 beds.

Abhishek: Okay, sir.

Moderator: Thank you. The next question is from the line of Nilabja Dey from [inaudible 0:26:27]. Please go ahead.

Nilabja Dey: Hello, sir. Good afternoon. First of all, thank you for the very good results. Actually, I just have two questions. First question in the concall of 2024, the first call you made, you said that you will be hiring CEO and COO. Do you have any update on that?

Manish Grover: Sir, we have already promoted COO internally from our company. Whatever we did yesterday, we will update it on NSE soon. We are looking for a CEO. Till then, I am the only one.

Because I need a dangerous person with energy like me. The one who sleeps at night, keeps waking up all day like me, keeps running all day.

One day Mumbai, one day Lucknow. If I get such a person, if you have anyone, sir, please suggest. I have taken many interviews like this. So, we are hoping that we will finalize one of them soon. If you have any reference, please give it to us. The one who has the knowledge of this field, has a growth mindset, and can handle all the operations well. We have already appointed COO.

Nilabja Dey: So, actually another thing, sir, regarding corporate governance, I think you are on the right path. You have hired one very good auditor. So, you just mentioned about the sales force right now, but for your financial transactions, do you use ERP or any other homegrown software?

Manish Grover: Absolutely, sir. We have a whole team of in-house

use software. We have a development team.

We have an internal system. We call it ERP. We have our own ERP system, sir. If you think about it, in the last three months, 1,25,000 patients have taken medicines . 86,000 patients have been admitted. So, we do everything with software, sir.

And our internal software is very solid. Earlier, we did it outside. So, there is a fear of data leakage, sir. Someone else starts calling the patient. So, internally, we have our own call center. We have a call center of around 900 children. We have our own data bank; cloud and we have our own in-house storing capacity. So, we have an internal team.

Nilabja Dey: I will take just one more question. Sorry, it's a hard question. In terms of internal auditor, if E&Y is still your internal auditor, I would just like to know. What is your internal auditor?

Manish Grover: Right now, we are bringing in a new auditor and we have an agreement with them. We will announce it in the AGM. We have finalized it with Grant Thornton.

Nilabja Dey: So, Grant Thornton will be your internal auditor?

Manish Grover: Internal auditor and signing authority auditor.

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Nanak Chand: Sir, I would like to clear on this part. Actually, right now, we have already internal auditors. I have a CA firm, which is Deepak K Garg and Associates. And [inaudible 0:29:38] is a statutory auditor. So, in the future, we are also going to change the internal auditor while we think it according to the size of the organization.

Nilabja Dey: Okay. Thank you.

Manish Grover: And E&Y has been connected with us for 1.5 years, right? On management and performance review. They have been supporting us for 1.5 years. They have been helping us a lot. They have helped us with all the success.

Nilabja Dey: We sincerely wish you success. And you maintain the highest level of corporate governance so that you will get the confidence of the investors in India very soon. This is my last question.

Are you still maintaining your guidance of 25% tax...

Moderator: Sorry to interrupt, sir. We would request you to come back for a follow-up question. As there are several participants waiting.

Nilabja Dey: Yes.

Moderator: Thank you. The next question is from the line of Keshav, an Individual Investor. Please go ahead.

Keshav: Your run rate of INR60 crores was running. So, your new products are also coming. So, ideally, your INR700 crore s guidance is a bit conservative. And how did you do it in July?

Manish Grover: Sir, in the first quarter, we have kept a run rate of INR60 crores . After that, a month has passed. July has also passed, August has also passed. We are maintaining the same run rate.

Because on 22nd, OTC will be launched. So, the result of OTC will start coming next month.

So, we are maintaining this run rate, sir. And we will also show the growth gradually. The hospital expenses will remain the same. As the bad occupancy increases, the benefit will also come.

Keshav: Right, right. I was saying that INR700 crores will come from here. And OTC will come more ...

Manish Grover: OTC will be extra. I did not count that -- I did not count OTC. And I did not count online marketing in this. We have made our own store jeenasikho.com in which we have listed our products. So, in the last month, our sale of INR3 crores has come from our store.

Keshav: Got it. And how much OTC are you expecting this year and next year...

Manish Grover: In OTC, I am expecting a turnover of INR500 crores in 1.5 to 2 years. Because there are 140 crores people in India. If I sell 10 lakh pieces of my kit, which I have launched now. Pet Liver Shuddhi Kit . Which is being printed on the front in Dainik Jagran and Amar Ujala on 22nd and 23rd.

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If I sell 10 lakh pieces of that

kit, then my turnover is INR55 crores per month. That is, a turnover of about INR600 crores. If I hold a 20% pad, then you can get an idea of on margin .

And this is on e product. And I am bringing 15, 16 products like this. I am bringing a BP Tablets /Capsules , I am bringing a Sugar Tablets /Capsules .

Medicine of Kidney, medicine of Liver, medicine of Depression and Anxiety. Medicine of Sexual problem, medicine of Thyroid, medicine of Migraine. All such clinical trial researches are going on.

If you want, then I will share the screenshot. If you check the website of the Indian

Government ICMR, CTRI. So my 12 clinical trials are visible there. Out of which 3 have been completed. And everything is visible there. Clinical trial is ongoing.

And I will keep increasing it continuously. Because my aim is to move India ahead from Ayurvedic. Modi ji's vision is to Make in India, Vocal for Local. Move India ahead from Swadeshi. So our mission is the same that India will move ahead from Swadeshi.

Keshav: Thank you so much. Congratulations sir on good set of numbers and good vision what you have for the company. Thank you.

Manish Grover: Thank you sir.

Moderator: Thank you. The next question is from the line of Kush Kothari from Kothari Investments and Infrastructure. Please go ahead.

Kush Kothari: Sir, first of all, you are very lucky that you have shown such good results.

Manish Grover: Thank you sir.

Kush Kothari: And sir, I wanted to know that when I came in the last con call. You told that you have a hospital in Dubai. It will be open till the next quarter. That's why you said. So can you tell something about it?

Manish Grover: Sir, we are not opening 2 hospitals. We are opening clinics. 2 have been opened there. But actually, there is a problem in the company's registration. It will be final approval in 2 or 4 days. Clinic and hospital has been opened. 2 have been opened in Dubai, Abu Dhabi. And we are planning 4 more there. And the Nepal one is also operational. This time in 3 months. We have received a profit of around INR10 lakhs from Nepal. The Nepal one has also started.

Kush Kothari: Sir, I wanted to ask this. That when you told in the last con call. That you will take more care in taking hospitals of the colleges. That you have avoided this medicine business. So what growth do you see ahead in that this year? Are you talking about your target with other hospitals?

Manish Grover: My target is clear. Before 31st March. My target is from 2850 to 3000 bed this year. So that's why I will increase the occupancy in this only. Then in 3 to 5 years. My target is 7000 to 10,000 bed. This business is done. I have told about OTC. That in 1.5 to 2 years. I have a plan to bring a turnover of INR500 crores. And I have a plan to maintain a 20% PAT margin.

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Kush Kothari: Sir, how much does your cost fall in this medicine business? Sir, what you make?

Manish Grover: What we do now. Our gross margin is 85% now. And what happens in this OTC counter. What happens in this. Like I am selling medicine for INR960. So we are billing for INR555.

Distributor, dealer, super distributor. So if the medicine is for INR1000. Then its 45% discount commission goes to the whole network. In which super distributor. Distributor, dealer, and medical store. Means it spreads in all of them.

And schemes etc. So we have to give about 45% discount. From the print rate. And now there is very good news. Now we are getting to hear that. GST will also come to 5% instead of 12%. What Modi has announced. If it comes to 5%. Then it will be very good for the whole country. It will also be good for Ayurvedic.

Kush Kothari: And sir, I want one last question. Sir, like I am in the south. So your clinic and hospital ...

Moderator: Sorry to interrupt. We would request you to come back for a follow up question.

As there are

several participants waiting.

Manish Grover: Sir, we are having a meeting this week, for future expansion. We will open a clinic franchise.

We will also open a hospital and daycare franchise. So south, east, west, north. We will spread everywhere.

Kush Kothari: Okay, sir, thank you sir.

Moderator: Thank you. The next question is from the line of Prerana from Equity Research Program. Please go ahead.

Prerana: Sir, your occupancy. I know occupancy cannot be compared. Because we have added all the beds this time. I think almost 700, 800 beds now. But still if you can give an occupancy number. Like you know. Maybe matured hospitals and non-matured hospitals. What would it be?

Manish Grover: Actually mam. Like in the last year. When 31st March ends. We had 1600 operational beds. And occupancy was 53%. If I still consider 1600 operational beds. Then occupancy is 80%. But now the beds are 2180. So the occupancy will be 57%.

Actually we are adding beds every 2, 3 months. So the occupancy will be like this. Means you will have to see both the ratios. If you calculate the old beds. Then it is coming 80%. But if you calculate the new beds. Then it will be 57%. Whereas in 2022...

Prerana: Sir, next time ...

Manish Grover: I have a plan to add 2850, 3000 beds by 31st March.

Prerana: Yes sir. I remember that in your last con call. I will just give you one suggestion. Maybe in

PPT, means the beds which are less than 1 year. You give separate occupancy for that. And the beds which are more than 1 year. You give separate occupancy for that ...

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Manish Grover: In PPT, we do bifurcate, we will give both the figures. What is the percentage of existing beds. And what is the percentage of new beds. We do this.

Prerana: Yes sir. And I had the same question. But I want to say , like you are great. I always listen to your con call. I wait to interact with you. And I pray to God. To make you the number 1 in India.

Manish Grover: Thank you very much. And let me give you an information. The products which I am launching in OTC. So I am giving a number along with it. I am giving a link to people. Like Modiji talks about mind. I am talking about health. That every Saturday , at 4 pm in the evening. And everyone who will buy the product. I will connect them online. And it will go on Facebook to the people. Who have purchased the product. We have converted them , we are going to start a new topic of health . In 1 , 2 months that will also start.

Prerana: Thank you so much sir. Have a nice day.

Moderator: Thank you. The question is from Anjali Bajaj from Naredi Investments. Please go ahead.

Anjali Bajaj: Good afternoon, sir. Congratulations for good set of numbers. You're actually a model for higher education colleges like Saraswati College, Sanskriti University, since promising for faster expansion and better marketing. Can you share how this partnership will work in practice? Who managed the operation? How revenue is shared? And also, how many more such college tie-up are planned this year? Thank you.

Manish Grover: Madam, now we will plan all the colleges. We will not open our own hospitals. I told you in the last con call that earlier when we used to open hospitals, we used to spend INR3 lakhs, INR4 lakhs per bed for one bed. Now it will be INR1 lakhs, INR 1.5 lakhs because we are spending from the hospital itself.

For example, there is Sanskriti University in Vrindavan. They are preparing a hospital of 150 beds. They are spending all the money. And we will get another benefit there. Our operational cost will also be reduced because in the college, there are already 50 -60 professors who teach. So we will get that on a very low salary because the salary is given by the college.

Secondly, when we had to find a doctor, there was a problem. Now we will g

et a benefit that

when we tie-up with colleges, we will open a hospital there. So the doctors who will come out there, every year 100 doctors will come out of a college. We will get trained doctors from the beginning. So our training cost of 6 months to 3 months to give to the doctors, that will also be finished because training has already been done in those hospitals by those doctors. So this will be a win-win situation for both.

And the hospitals that we have tied up with, they have done minimum rent and revenue sharing. For example, there is Sanskriti University. We have got INR2 lakh s rent from them and 6% revenue share has been finalized. And there is Sar aswati University in Mohali , we have got INR5 lakh s rent from them and 7%, which is highest, has been finalized.

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And Aurangabad College, we don't have to pay any rent there. And they only have 7% of revenue sharing. There is no rent there. So we will get such colleges like this. And I have approached about 16 colleges who are ready to work with us. But we will finalize one by one because we have to fill the beds, we have to do the operation, and we have to bring different projects too. For example, we will bring a heart project in one hospital, we will bring a joint pain project in one hospital.

So we will make different hospitals specialized in different ways. If you have kidney disease, go to that hospital. If you have kidney disease, go to Aurangabad. If you have liver disease, go to Chandigarh. This is how we will do it.

Anjali Bajaj: Okay, sir. Okay, sir.

Nanak Chand: Thank you.

Anjali Bajaj: Congratulations to you, sir.

Nanak Chand: Thank you.

Moderator: Thank you. The next question is from the line of Divya Agarwal from Ficom Family Office . Please go ahead.

Divya Agarwal: Hi, sir. Thanks for taking my question. Sir, Manish ji, I had a question. Our receivables from the government were INR92 crores in the last quarter. Our total was INR98 crores . Of this

INR98 crores , INR92 crores was from the government. So how much is our receivables from the government now?

Manish Grover: Sir, let me give you some information. Last time, I was asked this question in a call. So I brought the government business which was running for 25 minutes to 8% in this quarter. And out of the last revenue, we have received around INR25 crores. So how much was the revenue from INR92 crores now?

Nanak Chand: Now, our...

Manish Grover: Now, the 3 -month business which is worth INR15 crores, we have received around INR70 crores -INR75 crores.

Nanak Chand: From the government.

Manish Grover: The receivables from the government. And we have clearly put a letter to the government that if you correct your payment, then we will work for the government or else we will not do it. In the last 3 months, I have only worked for 8% of the government. So the answer from the 12 governments was that we are building a new portal. And as soon as the portal is built, we will turn the payment circle within 2-3 months. So now, we have received INR25 crores. And now, in these 3 months, I hope we will receive INR15 crores -INR20 crores more.

Divya Agarwal: So now, we have INR75 crores.

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Manish Grover: We have around INR70 crores.

Divya Agarwal: Okay, because...

Manish Grover: I don't know the exact figure, but I have this idea that INR24 crores came in the last 3 months.

Divya Agarwal: Okay, because the last conference that took place on 25th June, it was said that the average of 3 months is INR60 crores from the government. But now you are saying INR70 crores. So what will happen in 5 months?

Manish Grover: No, sir, it adds up every month. The sales that I do every month, it adds up too.

Divya Agarwal: Okay.

Manish Grover: I have not stopped the sales yet. I

am still doing 8 %-9% business with the government.

Divya Agarwal: And will it remain the same or will it increase?

Manish Grover: No, why will we increase it? When the government corrects the payment, then we will increase it. When the government's portal is ready, the government has made an NHA portal. When the government's payment system improves, then we will start. Now there is one more good news.

The Bihar government and the Maharashtra government, the Bihar and Maharashtra governments have also started passing their employees' bills in Ayurveda.

Earlier it was UP, then Punjab, -- earlier it was UP, Punjab, and what was it? Haryana. Now we have the letter from the Bihar government and the Maharashtra government has also started passing its employees' bills. They have passed the mandate in the Vidhan Sabha. Mandate. And the health insurance companies have also improved. Now the cashless has increased and the reimbursement has decreased.

Divya Agarwal: Okay . But sir, what you are saying that we are doing new sales on the government, we are still collecting receivables from that too, right?

Manish Grover: Sir, what actually happens is that the payment keeps coming. This is a circle. But I am not dependent on this circle now. 92% of my business is private now. Last time, some of your people said that if you focus on the government, it shows a receivable pending. So I reduced the government business and made it private. Now the government business is 8% in 3 months.

Divya Agarwal: Yes, yes. What is the average data base from our government? How much money are we getting in a month?

Manish Grover: Payment comes in about 4 to 6 months.

Divya Agarwal: Okay, okay. And sir...

Manish Grover: The CAPF portal is not made by the government. That's why the CAPF is being delayed. TGSS is getting a very good payment. So we have also reduced CAPF. CAPF is the Central

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Armed Police Forces. It has BSF, CRPF, TISF, Assam Rifle, NSG Commandos. Their portal has not come yet. So the government is saying that the portal will be made by next month.

Divya Agarwal: Okay . Understood. Sir, I wanted to ask you about the auditor. You had appointed Walker Chandiok as an auditor. So now you are saying that it is Grant Thornton. So I just wanted to know about him. Who is our current statutory auditor?

Manish Grover: Sir, right now Grant Thornton has been appointed as a statutory auditor. From the second

quarter onwards, we are signing new alliances.

Divya Agarwal: So is Walker Chandiok temporary?

Manish Grover: Yes, he is a statutory auditor.

Divya Agarwal: So he is temporary. Later on, he will become Grant Thornton.

Manish Grover: Yes, yes. No, no, no. Right now, GT is the -- GT has come, sir. We have been appointed. This up balance sheet will come in the next quarter. They have also checked this balance sheet, sir.

But they have not signed the authority. They say that they will sign it from the next quarter.

Divya Agarwal: So is Walker Chandiok and Grant Thornton the same?

Manish Grover: Walker Chandiok ...

Management: Yes, yes, yes. They are the same. They are the same, right?

Divya Agarwal: Okay, sir. You were going to bring EY ?

Manish Grover: Didn't you bring EY? Sir, we didn't bring EY because EY was giving its fees with our revenue. They say that according to the increase in revenue, the fees will be given.

Divya Agarwal: Okay. Okay, okay. Understood. Thank you. Thanks a lot.

Moderator: Thank you. The next question is from the line of Gaurav Singhvi, an Individual Investor.

Please go ahead.

Gaurav Singhvi: Hello, sir. Am I audible?

Manish Grover: Yes, sir.

Gaurav Singhvi: Hi, sir. First of all, congratulations for the results.

Manish Grover: Thank you, sir.

Gaurav Singhvi: Sir, I wanted to ask you something. Why are you not ending the revenue of the OTC

counter in

your guidance till the end of FY '26 ?

Manish Grover: Sir, I -- you are saying that I should show the expected revenue?

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Gaurav Singhvi: Yes. I mean, how much will the OTC contribute at the end of FY '26 ?

Manish Grover: Sir, let me launch it on 22nd. I told you. I launched it on 22nd. In the first hour, I got 21 orders.

And in 4 days, all my goods were finished. So, I bought more goods. First, I took the feedback of people. So, I am launching it on 22nd. Sir, I -- 2 months, 4 months , July is still going on. By September-October, its distributors will be made. In November -December, it will be launched in Gujarat and Maharashtra. So, what will be left? 2 -3 months will be left for this year.

Gaurav Singhvi: Okay, sir.

Manish Grover: And if its revenue comes and there is no profit, that's why I am not claiming anything.

Whatever will come, your and my luck will fulfill its efforts. I have told you. A cheque of

INR1,01,00,000 crores. We got a cheque of Super before UP. We got a cheque of

INR11,01,000 for Gujarat yesterday. People from Maharashtra are coming to have dinner on

22nd. So, I am launching it. Whatever will happen, it will come in front of you, sir. I have told

this from the old business that is going on.

Gaurav Singhvi: Okay, sir. Ideally, sir, it should be more than 700 plus. Let's see what happens.

Manish Grover: Sir, you -- Look, I will put all my efforts. I will build a rail. I want to do the revolution of

Ayurveda in this country.

Gaurav Singhvi: Okay, sir. My second question was that what is the minimum net margin you expect in FY '26 ?

Manish Grover: Sir, I am in favor of 20 %-25% margin that this much is a healthy margin.

Gaurav Singhvi: Okay. Because we have already done 29 %-30% of this quarter.

Manish Grover: So, this margin -- we consider it as a lottery.

Gaurav Singhvi: Okay. It was only for one quarter , right, sir?

Manish Grover: Sir, we are putting all our efforts. But I want that 20 %-25% is a healthy margin for everyone.

Because I don't want to take too much money from the patient. My main aim is to cure the

patient. And check the ticket size , the ticket size is the same. Look, my ticket size was

INR8,200. Now it is INR8,280.

Gaurav Singhvi: Okay. Okay, sir. Thanks.

Manish Grover: Sir, our margin was good last time as well. We got some provisions in the end, and we spent a lot in our expansion. So, we got that benefit this year, right?

Gaurav Singhvi: I understood, sir. I just wanted to know whether this 30% will be continued or not. So, you have given the answer. Okay, sir. Thank you.

Moderator: The next question is from the line of Neeraj Agarwal, an individual investor. Please go ahead.

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Neeraj Agarwal: Acharya ji, I wanted to understand that your operating expense in the last quarter was

INR7,943 lakhs, this time it is INR7,714 lakhs and you have added additional beds. So, what is the reason for this? And can you tell us the mix of Q1 service and product, what is going on and what should we believe in INR700 crores, what will be the break-up of this?

Manish Grover: I will tell you right now, sir. The sale of INR174 crores that we did just now, you note it down. INR80 crores came from the medicine sale; INR78 crores came from the private panchkarma sale and INR15 crores came from the government panchkarma sale. So, if you see the average, it is a 54% panchkarma sale; 46% is medicine sale. If you ask about the last quarter, it was INR139 crores.

So, the government panchkarma sale in the last quarter was INR28 crores; private panchkarma was INR47 crores and in Q4, the medicine sale was INR63 crores. So, INR63 crores came to INR80 crores. INR47 crores came to INR78 crores and from INR28 crores, I have increased the government business

from INR28 crores to INR15 crores. So, I increased that and increased that because the bed is the same. So, we focused more on the private business.

Neeraj Agarwal: Okay. And my question was that your operating expense has increased from INR79 crores to INR77 crores. Whereas, the number of beds you are telling is 2,570, it is around 2,100. So, what is the reason for that?

Manish Grover: I don't know. Please tell me.

Nanak Chand: Hi, sir. So, we are reducing our operating expenditure as compared to the last quarter by 1% and our monthly expenditure right now is INR30 crores per month. So, we are maintaining the sustainability of the same and we are getting the benefit of the operating margin. So, we are continuously in the path of reducing the expenses and increasing the operating leverage.

Manish Grover: Sir, we have made new tie-ups from where we used to get bedsheets, from where we used to get to wels, from where we used to get oils we negotiated with them. We made the best deal with them. We said that we are increasing the sale of your product, we will buy things from you together, so we got the benefit in rates from that.

And our operating expense, the oil that used to be spent, the expenses that used to be there, they were a little maintained. We made a whole committee -- we made a whole committee of cost control in our company. We made a whole department to control the cost.

Neeraj Agarwal: Very good. So, sir, can we expect this 45% EBITDA margin to remain the same for the whole year or are you expecting a reduction in it?

Manish Grover: EBITDA margin, sir, what is EBITDA? Okay, the total gross margin. Sir, what I told you, I understand the net profit because in EBITDA, it also adds some depreciation. I don't know what it adds. I don't know what the balance sheet is. So, I know that according to me, the profit that I gave in H1 last year, I gave 21% and it was left in H2, it was 19.5%. So, according to me, I will increase 2%, 3% free patients to treat more. But I want the margin to be 20%-25% healthy margin. It will come on its own -- if the bed occupancy increases, it will come on its own.

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own because now it will show that the bed occupancy has increased and we told you that on April 1, we increased our rate by 15% which was mentioned in the last call.

Neeraj Agarwal: Yes, it has come to INR60 crores net profit. So, sir, as soon as we reach the top, we will sit with you.

Manish Grover: I am maintaining the net profit of 60 crores because what happens is that in the middle, there was a problem last time as well. I said it, then it goes around. For example, India and Pakistan got affected, my hospital patients ran away for 4 days then I will commit like this then that problem will come. You do not say anything to India and Pakistan, but you guys catch my words. So, I am promising to maintain that only. Now those 4 days the patients have become empty in the last month.

What is still in November, December, January here it is cold in North India hence patient numbers decrease. In the month of Diwali, 5-7 days the patient go home. Actually, no bottle is applied in our hospital, neither any glucose is applied, nor any injection is applied so they have a right to run.

In allopathy hospital, they hang the bottle, so the person cannot run. In our hospital, it is said that it is my anniversary, I have gone home. It is my wife's birthday; I have gone home. I have a business deal, I will do it. This is how our patients do it because we have a kind of wellness. They are having a kind of regeneration. So he finds his convenience.

Neeraj Agarwal: Yes. This is my last question. This year you are targeting 2,800-3,000 beds. What is your target for next year?

Manish Grover: Sir, I told you that in 3 years, 5 years, my target is 7,000, 10,000 beds.

Neeraj Agarwal: Okay, so there is no such firm target for next year. It is long term?

Manish Grover: Sir, if I am saying 7,000-10,000 beds in 3 years, 5 years, then it means that every year I am planning 1,000 beds. Actually, I told you that there are 19 lakh beds in India, 11 lakh private and 8 lakh government allopathic and I have 2,200 total. So the gap is only 0.00. Now I have an opportunity, I am getting a whole hospital of 1,000 beds.

But I will sit with my team and discuss that do I have to take a big mess of 1000 beds and I will sit with you and the people who are with me, CA, GTE, I will sit with everyone and discuss what project we will bring there. I mean, I don't want to be born from a child's operation, I don't even let people change their knees, that's my target, so my 10,000 beds will be less. By the way, there are 60,000 beds vacant in India today, which are vacant near Ayurvedic colleges, so that 60,000 beds are my target, but that is my long-term plan.

Neeraj Agarwal: You are doing a very good job Acharya ji, best wishes and you progress more, best wishes to you. Thank you.

Moderator: Thank you. The next question is from the line of Darshil Jhaveri from Crown Capital. Please go ahead.

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Darshil Jhaveri: Hello, thank you for taking my question, sir. Firstly, congratulations on a very good result, sir. Sir, in our last call, I think we said that our target is to do around INR140 crores of PAT, but within quarter 1, we have made a profit of around INR50 crores. So, are we revising some aims for this year or what can be done, sir?

Manish Grover: Sir, look, I will get down to my commitment whatever I have said, it will come useless, it will come more, it is good for everyone. We will put our full effort and now I have told you that I am maintaining my run rate and I will increase it a little every month. It is not increasing on its own, because the occupancy is increasing, so it is increasing on its own and now the OTC has also started, meaning it will launch on 22nd.

Sir, clear that calculation, my target is the same, that the turnover of INR700 crores and maintaining PAT between 20% to 25%. And I will tell you again, note a number, 7528975283, please put your address on this number, your full address and your name, so that we can courier you that our new product is being launched, that you also see the power of Ayurveda, that how Ayurveda works on the root of the disease.

Darshil Jhaveri: Correct. And sir I just wanted to know that in terms of the product launches, now we are doing a big product and then, I mean, in this year, what do you think, what revenue can be possible from the product?

Manish Grover: Sir, in this year, I will launch six to seven products before 31st March and in the next year, I will launch 15 to 20 products and I have told you that my target is that in 1.5 to 2 years, the turnover of INR500 crores from the product side and 20% with PAT margin. I have done all the calculations according to that.

Darshil Jhaveri: Okay. Fair enough, sir. So will there be any upfront marketing cost, due to which we will have to invest in the starting distribution and all.

Manish Grover: Sir, we don't want to do a loan business, I have said it clearly in the market, that I will not do a loan of INR1, pay the payment and take the goods, I will add it in the newspaper, I will make videos on social media, if you want to buy the goods, then come and take it, so till now I have about 125 distributors and franchises from all over India, who want to become super distributors, their numbers have come, in which we are inviting 60 in Lucknow for dinner, on 22nd evening from 7 PM to 10 PM, I will have dinner with them at night.

And even if I will have salad and fruit, they will have dinner, the next day morning I will have breakfast with them and I will spend

2 hours to 3 hours with them, so that they get the confidence that a new world, a new India, I need their support to make a new healthy India, because the world goes to the medical store a lot.

So they make push products, we will give them good margins, and they themselves guide the patient that a new product has come and it will work on the root of your disease, so that we also put effort and they also put effort. So my ad expense is not much in this. We will add it from the front page, newspaper and TV, and by adding the expense of ad, I am telling 20% PAT margin.

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Darshil Jhaveri: Okay. Fair enough, sir. Yes. Thank you so much, sir.

Manish Grover: Thank you.

Moderator: Thank you. The next question is from the line of Chirag Shah from White Pine Investment

Management. Please go ahead.

Chirag Shah: Namaskar, Acharyaji.

Manish Grover: Namaskar. I listen to on e person from the beginning in every call, Chirag Shah. Thank you very much sir. I talk to you every time, so I feel good.

Chirag Shah: Yes. Most welcome. Sir, Acharyaji, my question is a little different. I have two main questions. First, the entry that w e are doing in OTC, in that quality control and in general the space and speed that we are expanding, what is your view on quality control and quality check, especially on OTC? Why am I asking this? We know that a very large Ayurvedic company and two, thre e small Ayurvedic companies have lost the plot. Many notices and actions were taken against them because of which the quality was compromised. Now as you have said...

Manish Grover: No. Notice do not come of quality compromises, the notices that come are of fake claiming, quality notices never come, and the products that I have launched, I am taking all the batches of testing, clinical lab reports with them, which will make the product and give it to me. And my team, I always -- I had told you earlier also in con call before that whenever I have a product pack, my own team goes, packs the product in front of us, mixes the herbs in front of us, they have to stay for four days, seven days, everyone stays there.

And when the product is made, before that, we do Ma hamrityunjaya, I don't know if I have told you once before or not, we pray to God in every product and do the packing, that God, we are using these herbs of yours, whatever you eat, please make it right, make it good. So we will not compromise that quality , we have already made written agreements, whichever manufacturer we are making.

And next year I will buy a very large manufacturing unit, I will buy it, I will do it in my company only, when my production will be of 10 lakhs a month, 20 lakhs a month, I d on't want to enter the factory on purpose. And if you have seen, we have also taken a loan license. We are also making four products in the name of Jeena Sikho .

So our planning is that we can slowly take all the products with us, but now my own team does the packing, so there will be no problem of any kind. And from where I have activated and used Indian Herbs , those herbs are available in India in a very large quantity, that is, the herbs that are added to their products, they have no shortage in India.

Chirag Shah: No. Sir, this is a good thing, but on this clarification, when you will do this in the beginning, but on a regular basis, then how are you approaching for checking, because regular checking...?

Manish Grover: Sir, every lot's lab report will come with it, whenever I will buy a product...

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Chirag Shah: Okay.

Manish Grover: I will take it with the lab report.

Chirag Shah: Okay.

Manish Grover: Heavy metal testing, its toxicity report. If you want, then the clinical trial of my BP pill has come to me, the whole trial has come to me, the BP pill has come, the sugar pill has come, I wi

ll share it with you, it has come to me. And you yourself have sent your patients to us, you got the idea that how we are working. So we do not compromise on quality at all, si r.

Chirag Shah: Sir, the second question, now there is no 100% success rate anywhere, right, and in some extreme cases, it goes to God too. So now the patient inflow that comes to us, the success is a good thing, but where we are a little unsuccessful, what are we learning there, why is it happening, we are bringing some thought process on it, what improvement can we do with the next patient, what are you doing on this, sir?

Manish Grover: Sir, I have a whole R&D team, I have a proper training team, R&D team, we are actually Stage 4 cancer patients who are coming, last stage, who are given the answer of allopathy, we get them to sign the form first, that we will only try, whether the result comes or not, your fate will depend on your effect, in spite of t hat, 40 -50 patients live a long life, so this is a very big success rate.

And we get the patient to sign the form first, and do the clarity, in allopathy, it is not even told that there can be side effects in the future. We say it very clearly, you don't h ave any other option, we are ready to put all our efforts, but you will cooperate, first sign this form. We give all the clarity to his family, that we will put all our efforts, you do it 10 days before, if you get the result, then you continue, if you get the result, then you continue. So patient first...

Chirag Shah: No, sir, my question is little...

Manish Grover: So patient first take it for 10 days. If they get good result, then they continue otherwise they are at their home and we are at our home.

Chirag Shah: No sir. My question is little bit different. You are doing a good thing, but for example, if a patient's weight is reduced, some patients recover, some patients don't, their body is different,

so we are doing something here, if we can do something at this rate, why is this happening?

Manish Grover: I – one time, Dr. Neha Sharma will talk, tell them about it, madam.

Neha Sharma: Good evening, sir. Hope everyone is in a good health. Dr. Neha this side.

Chirag Shah: Yes.

Neha Sharma: The patients we have, that is very different from the normal Ayurvedic footfall, as we say in Ayurveda, difficult to treat diseases, where allopathy is also involved, I have 40% patient footfall, specifically in cancer. And sir, for these type patients we have all the trained doctors,

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the prognosis of the patient, the staging of the disease is explained, based on that, the treatment is explained.

And you will not believe, but I will surely send you the research papers also. For example, I had a patient, whose research paper was already published, Mahesh Prabhakar from Mumbai. That patient was given just one month to survive, when his wife came to us, and said that, ma'am, the allopathy said we have a month, so whatever we do, at least in a month, their footfall will come out, so that patient passed away last week, completing almost three years, CA Lungs with Liver Metastasis, Bone Metastasis, Brain Surgery, Immune Therapy, Gamma Knife, everything was done, but that patient, who was surviving for three years, taking a month, was sustained by our treatment, their video, their research paper, has already been published.

So it is not like that, apart from that, there are such patients, who we know, that now they are near to the death bed, in those patients, the quality of life improves a lot, and if there is a patient, who has any problem, after going to the hospital, so we have follow back teams, every month, the patient's data is taken out, it goes to the call center, to the IPD retention team, to the IPD follow back team. So it is not like that, if in the patient, we feel that the result is not coming, so again, our me

dical board, discusses with our senior doctors, we work on multiple research, and again, we come up with something, for the betterment.

Multiple research is going on, sir, as sir told you in the starting, HRC and [BQ 01:07:53]. This is a very different kind of technology, which is for our test, which is now, which most people do not know. So we are bringing new things. It is not like that, 80% of the patient is satisfied, 20% depends on the prognosis, that sometimes, the patient comes to us, in a very critical condition, when they are on the death bed, they are also recovered.

Manish Grover: And anyway sir, a small thing, now a days, people are dying while dancing, people are sitting on the horse, the groom dies. Now, no one asks questions there. Still, we try our best, that such things do not happen, that is the thing sir. One thing I will tell you...

Chirag Shah: No, sir. That is the case. We will learn from here...

Manish Grover: No one has an answer to this. Some say, we are dying due to the CORONA vaccine. Some say, we are dying tired, so sir. No one has a chance to die, but we try from our side, to guide everyone correctly, and put full pressure on everyone.

Chirag Shah: Okay sir. Sir, all the best. Rest all the best. Everything is going good. Thank you very much.

Manish Grover: Thank you very much, sir.

Moderator: Thank you. The next question is from the line of Nagi Reddy from [Ghost 01:08:57] Capital.

Please go ahead,

Nagi Reddy: Yes. Namaskar.

Moderator: Nagi Reddy, we request you to please ask the question.

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Manish Grover: Madam, maybe, his voice is not coming. You take the question of someone else,

Moderator: We move to the next question. The next question is from the line of Anurag Agrawal from Multify Wealth. Please go ahead,

Anurag Agrawal: Hi Acharya ji, Namaskar sir, I wanted to ask, that last, you said a while ago, that in this quarter, we have reduced the marketing spend, vis-a-vis last quarter, so what was the reduction amount?

Manish Grover: Sir, I don't know, we have reduced the quarter to quarter expense, so how much is it? We have reduced the expense, that is why the profit has increased, we have reduced the expense, that is why the profit has increased, we have reduced the expense by 3%, we have reduced the expense by 3%.

Anurag Agrawal: In marketing?

Manish Grover: Sir, we have started the free treatment of 10% patients, in our every hospital, if 100 patients

are being admitted, then we are giving free treatment to 9 to 10 patients, who have become poor, who have no money left, who have been robbed by the hospital, we have started their free treatment, that too in the near future, the same people will work for me as a patient, they will do mouth to mouth publicity for me all my life .

Plus the biggest thing is that we will get blessings, we will get blessings from people, now in hospitals, people's clothes are taken, there they say, if you don't give money, you won't even get a leave, we don't have anything like that, our patients say, we don't have money, we say, no problem, don't give, because we are people connected to the earth .

What are the Ayurvedic people, they are very sattvic, connected to the earth, and emotional people, so the patient says, I have become poor, we say, you don't give . I will put the rent from my pocket and send it, if required.

Anurag Agrawal: My question to ask was, to understand, how sustainable is the margin, which is our profit margin this time, because this time you said, that cost wise, we reduced the marketing expense

Manish Grover: We reduced the marketing expense, we also reduced our operational cost, we increased our performance review, we improved our staff training, we improved patient retention, the patient who was taking Lama earlier, my Lama reduced, I increased my stay this time, my average

stay was of 8 days, 9 days, I brought him to some hospitals for 12 -14 days, we did a lot in the last three months, and we will continue to do in the next nine months, we will put full effort every month .

Anurag Agrawal: Thank you, sir, all the best for the future .

Moderator: Thank you, the next question is from the line of Kush Kothary from Kothary Investment and Infrastructure . Please go ahead .

Kush Kothary: Sir, first of all, I want to tell you that, thank you very much for giving me a second chance, that you have given me a second opportunity . I had this question that, sir, all the clinics and Jeena Sikho Lifecare Limited

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hospitals I see, there are very few hospitals and clinics in the South, as much as there are in the North, and I belong to Chennai, so seeing that your clinic and hospital is not there, so what is your...

Manish Grover: Sir, we have opened hospitals in Chennai, Bangalore, Hyderabad, Assam, I have opened all places, and after three days, my team is leaving for Chennai, Bangalore, Hyderabad training .

Now we are fully focusing, and I told you, in the next 15 -20 days, we are going to launch a franchisee network offer, we will not open clinics, hospitals, and daycare centers in all the cities of India .

On the franchisee model, this will be ours, this will be different, and why will it be different, because when I will launch OTC, then I will have to add in the newspapers every week, full page, quarter quarter page, I will have to come on TV in the morning and evening, so the patient will see me . So, if my clinic is not there, hospital is not there, then why should I do business loss .

So I am in the plan of opening clinics, hospitals in all the cities of India, but we will make its model first, and launch it in the franchisee model, means we will not do our own investment, but that franchisee will be invested, it will be operated by a company, means franchisee is called FO CO, FO CO model, Franchisee Owned Company Operated, it is called FO CO, I also have to see all this,

Kush Kothary: Sir, this is the right thing, Sir, you have many medical colleges in the South, are you doing tie - up with them too?

Manish Grover: Sir, first of all, the three that I have taken, first of all, I will set a business model for that, because for other hospitals, I will set a revenue model, business model, so the first one has started, Mohali wala, 15 days ago . Similarly, my franchisee will start the second, third Aurangabad . So first of all, I will make a model of three and fill it, make a successful model, so that 5th, 6th college .

Sir, I have to find a patient, he himself copies our model, and gives it to me, and runs it, help me, now I will have to spend the ad, then he will do it himself, his children too . Sir, their college seats will also be filled soon, if their college hospital is running . So I will make an ideal model in one way, like first I made Chandigarh hospital and started in Derabassi , one model was successful. I opened Lucknow, that was successful, then I opened Mumbai, in 4th year, I opened Meerut, now there are 55 hospitals, so in the same way .

I will make a successful model, and that is why, till the end of this year, from 2 ,850, I have expected only 3000 beds, and you think, I have already done 2 ,570, when I made a commitment till 31st March, last year, which was a call for 2025, I told you 2 ,850 in the whole year, maximum 3 ,000, now I have done 2 ,570.

Kush Kothary: Sir, so now your focus will be, that all the beds you have made, first of all you will fill them

up.
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Manish Grover: First of all, we will fill those beds. We will make this first, and along with that, we will develop the model, then I will develop a team, that will copy the model everywhere,
Kush Kothary: Ok, so you are saying a straightforward

thing, that you will explain this model to the colleges,
so that model will automatically run .

Manish Grover: Absolutely , I have already got the offer of 16 hospitals, that you work with us, and I have already created many like me in my team . Now I am not alone, if you see my presentation, it has a photo of five people . Last week, there was an investor presentation in Mumbai, so I did not go, Dr. Ish Sharma went. Now , the answer that was given, was Dr. Neha Sharma . So now the whole team is ready, now even if I don't go anywhere, the work is going on,

Kush Kothary: Yes, sir, and what you have told, that you have opened clinics in Chennai, Bangalore and Hyderabad, sir, is it your own or how is it sir?

Manish Grover: It is our own , all the hospitals are our own, there is no franchise,

Kush Kothary: Ok sir, thank you sir, this was my only question, and thank you very much for the future.

Manish Grover: Our franchise is only 36, our OTC, we only have small clinics, and we used to have 60 before, three years back, I have reduced it to 36 . The old franchise model is a bit weak, I will close it, I will bring a new franchise model, and this old franchise, we used to spend on it, we used to pay for the clinic, now it is only 36 clinics, and their total contribution is INR3 crores, in the whole business model . All 36 -37 clinics, give INR3 crore sales . So the remaining INR57 crore s revenue, from the hospitals, day care center, and big hospitals, and the government business,

Moderator: Thank you . The next question is from the line of Pawan Kumar from Shade Capital . Please go ahead,

Pawan Kumar: Manish ji, thank you for the opportunity . Congratulations for very good results . My question was, you are launching OTC products, how is it different from the products, that are available in the market ? Can you shed some light on this .

Manish Grover: In the market, for stomach cleaning, there is Pet Safa, for stomach cleaning, there is Kayam Churna, for liver, there is Lip 52, there are 36 such products, but there is no solution product. These are all Sanai based products, which put the patient in habit. All their life, the companies want, that the patient should keep taking medicines. We have done clinical trials , and introduced such herbs and made a product, which works on stomach, liver, and spleen, all three together .

And as per Ayurveda, I had told you earlier, [Inaudible 1:18:18] , means Ayurveda says, that the reason for the disease, is that the mind is fire, food should not be digested, in Ayurveda, the juices that come out, the juices come out, they are also called fire, which are our digestive juices, which are in the body, different hormones are secreted, different liver, stomach juices are there, there are juices in the stomach, there are juices in the intestines, all that is fire .

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So that fire is your mind, it is weak, it is bad, and your pancreas, spleen, and stomach, and liver, are weak, due to which the metabolism is slow, your digestive system is bad . So sir, I am making a target of only 10 lakh pieces, for a month, and in that also, many patients will take repeat, who will continuously take that product, whose stomach is not clean .

I will give you a figure . In India, 10 crore people are taking BP pills daily, in India, 9 crore people are taking pills for high sugar daily, in India, 8 crore people are taking thyroid pills daily, in India, around 1.5 crore are heart patients, 1.5 crore are kidney patients, around 2.5 crore are liver patients, in India, around 1.5 crore are cancer patients, and in India, around 11 - 12 crore people's stomach is not clean .

Now what is the root of all these diseases, that mind is fire, food is not bad . So I have brought such a product, which will work on the root of all diseases, and how much is it, INR960. How much will I give the

distributor ? INR 545. So sir, if every person, is taking any medicine, will start taking this along, then gradually, it will work on the root of the body's disease, and the rest of the products, are only cleaning the stomach . They are not working on the root, they are putting sanai, they are putting jamaal ghota, you will get IVs, you will be happy, 4 IVs came . Kapji came, did not come for many days, my products, will not put IVs, my products, will not

put fire, digestion, deepening, all three, will be managed together, and I am not saying this, this is a clinical trial, research has been done, that when the stomach, liver, and the axis of the spleen, works together, once . Dr. Neha will tell, tell me once, madam,

Management: Sir, like the rest of the products, which you are saying, such as, cleaning the stomach, it is basically, only laxatives, which we say in medical terminology, that constipation, or for constipation, or to release the stool, but the product, which we have made, sir, it is on your liver, on your spleen, on your gut, basically, it works on metabolism, this is the multiple functioning, of this product .

If you, talk about , kayam churna it has got only function after taking kayam churna, again in the morning electrolyte imbalance, and it's a quite an important factor medically. But what we have made is for major organs in your body. If I talk about liver, if 80% of the patient make their gut and liver healthy, they can reverse serious illness. So this is one kit which works in all aspects on liver, gut, spleen. So after taking this base kit, 90% of the patient's symptoms, start reversing, which is not the same for others, Kayam churna is only laxatives .

Manish Grover: Dr. Saab, whenever one take this, they would say thank you , afterwards will take admission only in hospital. So my occupancy, also help, if ten lakh people, can take my medicine, then I won't have an empty bed in any of my hospitals because some of them have cancer and some have kidney failure, so I am doing a lot of hunting with one arrow by buying products .

I am basically supporting my old company whose business is going on whose occupancy is 80%. if I look at the old 1,600 beds so I have to maintain that 80% and 90% , but I am increasing the beds every month that's why the occupancy so all these businesses will supplement each other and all this business is in the Jeena Sikho company itself there is only one parent company, and everything is in that .

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Pawan Kumar: So if I am able to understand these products are not available in the market?.

Manish Grover: Sir, in this kind of market, no one has a brain people's brain is to earn money people's thinking is how to earn money , my thinking is how did people get well . People didn't fall sick , people didn't go for allopathy , people didn't leave the house in the name of hospital their first thought was kitchen their first thought was naturopathy .

Like there is a medicine for fever when you all have fever, you take medicine but if someone of us has fever we dip our feet in hot water along with that we drink lime juice our fever gets cured in 1-hour gets cured in 8 hours, 4 hours this natural way of curing fever is not a side effect because fever medicine is the worst medicine in the world and this is not my research this is the research of the world that fever medicine is hepatotoxic my kidneys fail, my intestines get spoiled .

So any medicine before eating we want India to adopt naturopathy so that we don't fall sick there is a diet chart in this kit this is not just a kit product there is a diet chart in this there is a free counselling doctor's number there is a number for my health which I will come live every month and talk about health , I will educate in that sit in a corner and drink water there will be 4 toilet seats in your house so I will

tell you to use one so that your knees don't get spoiled . I will give such training so that people don't fall sick .

Pawan Kumar: so I understood that the other 14 products that are coming are also based on the same theory that they will be completely different from the available products in the market ?

Manish Grover: Sir, the products that I am bringing in the world no one has even thought about it . Sir, I have told you that everyone is thinking to sell products and earn money , my product is to sell products and earn blessings , to sell products and make India the world guru to sell products and make Ayurveda the number one this is my thought money will come by itself .

Pawan Kumar: Yes Manish Ji, Thank you for replying in this way and I wish you a lot and I wish you a lot for your future .

Manish Grover: Thank you very much , sir.

Moderator: Thank you . The next question is from the line of Divya Agarwal from Ficom Family Office . Please go ahead .

Divya Agarwal: Hi, Manish Ji . I wanted to know that our ESOP cost is in FY '25 and how much will it be in FY '26?

Manish Grover: ESOP we have not launched any new old ones, the cost is the same Nanak Ji, I will ask him .

Nanak Chand: Sir, till now the total expense of ESOP that we have booked is around 50 to 60 lakhs intrinsic value calculation and it is still vested . IT will be bifurcated in the next 5 years it will be bifurcated in the next 5 years so it will have a minor impact .

Gaurav Singhvi: Are there any new in FY '26 ?

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Nanak Chand: We are planning to vest the new ESOPs, so we are planning. We are finalizing and the Board then will decide to vest more ESOPs.

Gaurav Singhvi: Sir, lastly, I wanted to ask that our volumes sold in medicines, how much was in quarter 1? Total volume sold in medicines/products?

Manish Grover: Sir, I will tell you in 2022, '23, the total 1,57,000 people took medicines from clinics and hospitals in 2023-'24, 2,52,000 people took medicines and in 2025 total 3,37,000 people took medicines in this year till now 3 months, the total 1,25,000 people took medicines. So if you multiply it by four it will be 5,00,000 if you maintain the same run rate.

Gaurav Singhvi: Sir, our other expenses will increase this year?

Manish Grover: Nanak Ji, tell, what you are saying?

Nanak Chand: No Sir, our direct expenses did not increase it will be constant.

Gaurav Singhvi: How much will it be constant? Can you quantify?

Nanak Chand: Sir, our direct expenses in medicines is 15% and the consumption is 9%. So we are maintaining this figure, we had this much in the last quarter. So we hope to maintain this in the future also.

Gaurav Singhvi: Okay, so our profitability will increase?

Nanak Chand: Yes.

Moderator: Thank you. The next question is from the line of Nagi Reddy from [inaudible 1:26:44] Capital, please go ahead.

Nagi Reddy: First of all, I am not only an investor, I got a treatment in your Bangalore Hospital for Panchakarma. So, I got cured well. Yes, I have one question. I see that corporate hospitals like Apollo, they are entering into Ayurveda. Do you see any opportunity to sell our products to them?

Manish Grover: Sir, if they demand, we will definitely give. We have done clinical trials, researched and spent. If anyone contacts us, we will keep our margin and give them the product. Because we have BP pills, we have sugar pills, we have kidney medicines. All these have been clinical trials. So, if any good company asks us, we will give. Because our aim is to serve the patient and serve the country.

Moderator: Thank you. Ladies and gentlemen, that was the last question. I now hand over the conference to Mr. Ranvir Singh from NuVama Wealth and Investment Limited. Thank you. And over to you, sir, for closing comments.

Ranvir Singh: Yes, thank you. Thank you, everybody. And just

to my apologies for inconvenience. A lot of participants. This is a huge number of participants. And thanks to all. And this participant, there was a long queue for a question and answer also. So, we had to curtail the question.

Jeena Sikho Lifecare Limited

August 18, 2025

Manish Grover: So, Mr. Ranvir, give everyone Mr. Nanak's number. If there is any financial question, give them Mr. Nanak's mail ID or mobile number.

Ranvir Singh: Yes, definitely. So, if anyone has any questions, or in case you want any other clarification, please do approach me or even Mr. Nanak. And then we'll try to get it solved.

Manish Grover: And if anyone has a question for me, the number on which they will send their address, the address on which you will send your address, where I will courier a kit for them, write a question there. I will answer them as you say.

Ranvir Singh: Yes, definitely. So, thank you. Thank you for participating. Would you like to say anything else, sir? Mr. Acharya, closing remarks.

Manish Grover: The thing is, sir, that in India, it's been 78 years since independence. People only knew one thing that a llopathy can only cure. With the help of all of you, we listed a company 3.5 years ago. And with the same purpose, that we show the strength of Ayurveda in India and the whole world. Show the strength of the health of naturopathy. And show people the strength that people can treat themselves with their own body.

By correcting diet without medicines, by stopping eating after sunset, by eating fruits first in breakfast in the morning, by eating salad first in lunch and dinner, the world can be treated. Just everything is not treated with medicines, chemicals, injections, and operations. Trust the self-healing capacity of the body.

All the animals in the jungle, no operation is done, no hospital is visited. Even if a dog gets sick, it leaves food and heals itself in three days. So, there was a lack of education in India. So, we are basically an education-based company. We don't sell any product. We are giving education to people, by which people are healing themselves.

We only have the right diet, lifestyle, food, because in Ayurveda, there is a shloka in Bhagavad

Gita, yukta har viharasya, yukta cheshtasya karmasu, yukta sokna bodasya, yogo bhakti dukha. It means that whenever you fall sick, apply yukti. And whenever you fall sick, correct your diet, lifestyle, the disease will heal itself.

And you know that there are many blue zones in the world. Even Japan is taking an average of 84 years in the world. So, I would request you all that whenever you wake up in the morning, eat a piece of turmeric, a small piece of ginger, and 10 curry leaves in the morning, for 4 -5 days in a week.

Don't eat it every day, don't eat it for a whole week, for 4 -5 days in a week, because the body becomes numb. Otherwise, the body gets used to it. Never drink water with food, half an hour before, half an hour after.

And after sunset, stop eating food. Or eat for 6 days, and fast for 1 day. If every person in India follows these small rules, they won't fall sick. So, sir, this is the purpose , Jeena Sikho , and name of the company is also Jeena Sikho and name of my TV program is also Jeena Sikho .

And I would like everyone to Jeena Sikho . Thank you very much, sir.

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Nagi Reddy: Thank you. Thank you, everybody. Now we can conclude the call.

Moderator: Thank you. On behalf of Nu vama Wealth & Investment L imited , concludes this conference.

Thank you for joining us, and you may now disconnect.

Call: Nov 2025

Date: 12th November , 2025

To,
Manager - Listing Compliance
National Stock Exchange of India
Limited 'Exchange Plaza '. C-1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai - 400 051
SYMBOL: JSLL To,
Head of the Department,
Department of Listing Operation,
BSE Limited
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai 400001
SCRIP Code: 544476

Sub: Intimation to Stock Exchange – Transcript to the Conference Call for the quarter ended 30th
September , 2025
Dear Sir/Madam,

Pursuant to Regulation 30 read with Schedule III and Regulation 46 of the Securities and Exchange
Board of India (Listing Obligations and Disclosure Requirements) Regulations, 2015, as amended, we
hereby inform y ou that the transcript of the Conference Call held on 07th November 2025 to discuss the
Financial Results of the Company for Q 2 of FY 2025 -26 has been made available on the Company's
website i.e. www.jeenasikho.com .

The aforementioned t ranscript to the conference call is available on our website at:
<https://jeenasikho.com/wp-content/uploads/2025/11/Jeena-Sikho-Q2FY26-Earning-Call-Transcript.pdf>

Kindly take the above intimation on your record.
Thanking you,
Yours faithfully,
For Jeena Sikho Lifecare Limited

Manish Grover
Managing Director
DIN: 07557886

Place: Zirakpur, Punjab
Date: 12-11-2025

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“Jeena Sikho Life Care Limited Q2 FY’26 Earnings
Conference Call”

November 07, 2025

MANAGEMENT : MR. MANISH GROVER – MANAGING DIRECTOR , JEENA
SIKHO LIFE CARE LIMITED
MR. NANAK CHAND – CHIEF FINANCIAL OFFICER ,
JEENA SIKHO LIFE CARE LIMITED

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Moderator : Ladies and gentlemen, good day and welcome to the Jeena Sikho Life Care Limited Q2 FY'26 Earnings Conference Call.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touch -tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Ranvir Singh from Nuvama Wealth. Thank you and over to you, sir.

Ranvir Singh: On behalf of Jeena Sikho Life Care Limited, I extend a very warm welcome to all participants on the Q2 FY'26 Financial Results Discussion Call.

Today on our call, we have Mr. Manish Groverji, popularly known as Acharya ji, he is the Managing Director and Mr. Nanak Chand, he is the CFO.

Before beginning with this call, I would like to give a short disclaimer. This call may contain some of the forward -looking statements which are completely based upon management's belief, opinion and expectations as of today. These statements are not a guarantee of company's future performance and involve unforeseen risks and uncertainties.

With this, I would like to hand over the call to Acharya Manish ji for his opening remarks Over to you, sir.

Manish Grover: Namaskar Everybody. “Om Arogyam Astu”, we say this in our hospital. I am happy to share Jeena Sikho Life Care Limited's Q2 FY'26 Financial and Operational Performance with you all.

Our continuous hard work and effort has given a great result. We have observed a good revenue growth in our product and services segment. Revenue from operations for Q2 and FY'26 stood at Rs. 190 crores. Whereas in Q2 FY'25 it stood at Rs. 114 crores , in Q1 FY'26 it stood at Rs. 174 crores . This way company has registered a growth of 66% year -on-year and a growth of 9% quarter -over-quarter. PAT has seen a huge jump and it stood at Rs. 59 crores registering an increase of 121% year -on-year and 15% quarter -over-quarter .

Our focus was to increase the number of beds in the hospital, to fill them, to work on products delivery chain and products, to increase the footfall of customers in the clinic, to increase the footfall in daycare and increase the delivery and e -segment that we have launched. And we have seen a great result because of this. We have registered 66% growth in both the segments year -over-year. This expansional growth shows that we are strongly motivated towards spreading quality alternate health care services to a lot of people. We have increased our service footprint a lot. Recently we have added 582 more beds because of which our total bed has increased to 2,802, which I promised to do 2,800 beds in the whole year. We did 2,800 beds in six months post the conference call of 31st March, 2025 and our percentage has also increased, it is 57%
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occupancy. We have our reach in 23 states and 100 plus cities of the country and the customer traction which our newly opened hospitals have got is really encouraging. We are seeing tremendous growth in both our product and services verticals. And it is proof that we are continuously increasing the free cash generation revenue generation. Our EBITDA has also seen a growth along with the topline. It has increased to Rs. 92 crores registering a growth of 129% year-on-year and 17% quarter -over-quarter. Our profitability margin has been strong in Quarter 2, around 48% , which shows that, I am talking about EBITDA, it has been successful in doing business expansion while maintaining operational efficiency. And I want to tell you that the financial report that has come this time has been given by Grant Thornton, which is one of the top four companies of the world. We

have also done that work because people always say that results should be authentic . When I brought this company, we were only present in online market. Then we added clinics, we added hospitals, added daycare and now we are going to add wellness center. We are also adding diagnostics , and we are also increasing our distributor chain.

With that, I now hand over the call to Mr. Nanak Chand who is our CFO to tell you about financial performance of the company in detail.

Nanak Chand: Good afternoon, everyone. Thanks Manish Sir. As mentioned by Manish sir we have achieved a healthy financial performance during the Q2 FY'26.

Here are the key highlights of Q2 FY'26 Results :

So, I am pleased to present before you the detailed financial performance of Jee na Sikho Life Care Limited for quarter ended September 30, 2025, Q2 FY'26. Before we move to the details, I would like to highlight that it was a second quarter of reporting under Ind -AS following our migration to main board in 2025. And our financial statements and disclosures are now fully aligned with the main board standard and show high transparency and comparability. The company continues to maintain strong balance sheet supported by the robust operating cash flow and prudent financial management.

Now on the Q2 FY'26 performance :

Our revenue grew by 66% on a Y -O-Y basis to Rs. 190 crores in Q2 FY'26 as product segment grew approximately 78% Y -O-Y and services segment contributed 50% revenue during the quarter. EBITDA during Q2 FY'26 grew at 129% Y -O-Y basis to Rs. 92 crores by implying the EBITDA margin of 48%. The Q2 FY'26 PAT grew 121% Y -O-Y basis to Rs. 58.78 crores .

With this we open the floor for question and answer session. Thank you.

Moderator: Thank you very much. Ladies and gentlemen, we will now begin with the question and answer session. The first question is from the line of Akshay from AK Investments. Please go ahead.

Manish Grover: Akshay ji namaskar. "Om Arogyam Astu"

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Akshay: Namaskar Sir . Thank you sir. Sir, my first question is, first of all the performance that you have shown in this quarter is mind -blowing. A big congratulations to you on that and keep this performance going forward also. It is a request from all the shareholders. Sir, my first question is, the Pet Shuddhi kit that we have launched recently, how is the traction that we can see? We were going to launch our medicine and kit in medical also. Right? Has it started already?

Manish Grover: Please repeat your question.

Akshay: Recently, we launched Pet Shuddhi kit in commerce also. Right?

Manish Grover: Yes, Pet Shuddhi kit's Distributors have been created. UP has been created, we have launched in Delhi, Bihar, Gujarat and Maharashtra. And sales are increasing, product sale has increased this time a bit.

Akshay: Okay. So, this year we are launching eight to ten products in next six to twelve months. Right?

Manish Grover: No, sir. We are going to launch at least 15 to 20 products, and we are also in talks with a big distributor network . They have more than one lakh Pharmacies in India and it is a tech -first company. And they promoted through B2B app. So, I have had two, three meetings with them. In one or two final meetings their distribution network will also work with us. And our products will sell there also. Medical trials of BP tablets, sugar tablets, kidney medicine, liver medicine , and depression medicine have been done. And these products will be sold on their platform too.

Akshay: Okay sir. So, going forward we can expect that our product revenue will increase a bit and service revenue share will decrease by around 50%?

Manish Grover: No, sir. Actually, we are increasing the number of beds continuously. So, we will take both of them equally. Our main focus is to increase the number of beds because I said that

in three to

five years, I will do 7,000 to 10,000 beds. And in the conference call of 31st March 2025, I told that we will do 2,850 in the year end and we have done 2,802 beds in six months only. So, we are very aggressive, and my purpose is to fill these beds.

Akshay: Okay.

Manish Grover: Because according to the last quarter, bed occupancy is 57%. So, we have to increase it to 80% from 57%.

Akshay: Okay. Right. Sir, my second question was regarding margins. Will be able to sustain this 40% margin in the coming quarters for next one to three years?

Manish Grover: No, sir. In every conference call I tell about my target. 20% to 25% margin is a healthy margin, and we have to maintain that and this extra which is coming is good and happy news for us.

Akshay: 20% to 25%, you are talking about profit margin.

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Manish Grover: Yes. 20% to 25% profit margin is a healthy margin.

Akshay: Okay.

Manish Grover: For every company. But if it is additional then we all should be happy for this.

Akshay: Okay sir. All right sir. Thank you so much and best of luck sir.

Manish Grover: Thank you.

Moderator: Thank you. The next question is from the line of Abhishek from AB Capital. Please go ahead.

Abhishek: Hello. Am I audible?

Manish Grover: Ji. Namaskar sir.

Abhishek: Ram -Ram Acharya ji. A big congratulations on the performance of this quarter.

Manish Grover: Ram -Ram. Thank you so much sir.

Abhishek: Sir, one notification came regarding your tie -up with Chandan Diagnostic. Could you please explain in detail how synergies will play out and what benefits we will get and they will get because of this tie -up?

Manish Grover: Sir, we will get huge benefit out of this. I do not know about them. They should think about their benefits. We will have huge benefits and I will tell you what benefits we will get. When we tie -up with Chandan, they had existing 40 lakh Chandan privilege card holders and now they will avail cash benefits from us. My discount will get over. Their card holder patient will come to us, I will use their cash back of their card. They were already in seven states and in a way their 34 centers have opened in my centers on this 1st November, separately. Those running centers were theirs, so now my 34 centers have started running. And they have given us the offer that they will do the first basic blood test like ESR, uric acid, CBC, lipid profile, LFT, RFT, and thyroid profile for free. This the first step towards my customers and they will do it under health package and because of which footfall will increase in my hospital, and it will increase greatly. And in three, four days I am making the system smooth, so I will start making videos of it in three to five days. So, the footfall will increase in my hospital and clinic. And they are setting up full -fledged clinics and diagnostics too. Along with that they are setting up liver fibro care, ultrasound machine, blood testing total and radio diagnostic. And I will also get the benefit of my chronic patients plus we have focused a lot on our health insurance segment. Our next revenue that will come, the next sale which will increase it will be because of health insurance. So, Chandan's benefit that we will get is, their lab is NABL, so, because of Chandan's testing lab. Health insurance claim does not get passed if patient does not do test. So, because of my own in -house testing, I will get a lot of help from Chandan to increase my health insurance business. So, there are a lot of factors. And secondly my patients who will get their tests done
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from Chandan and the commission which Chandan will give to us, which is 50% for the world from that I will give 30% to the patients as a cash back. So, patient will purchase from me, patient will get admitted

at my hospital and Chandan will pay me. So, this is a very big strategic deal for us and I will get huge benefits from this to fill bed, sell medicines and to benefit the patient through testing. Thank you sir.

Abhishek: Okay sir. And sir our plan to take over the management of medical college, how is it progressing sir?

Manish Grover: In total we have done three and one has become operational, two has not been operational. We were waiting for Ayur veda to come in Ayushman Yojana. So, I have stopped it on three for now. I am not doing more until these three run successfully. One has become operational, and it is houseful. We have 90% occupancy over there, the takeover we did in Mohali which is under Saraswati Group of Colleges, it is of 100 beds, and it is running at 90% occupancy in today's date. Two are ready, one is Saraswati Group of Colleges in Vrindavan and third is in Buldhana, which is under Aurangabad. Second one is fully ready. We are planning what to start there because I am going to start a completely different line there. Hiring is going on for that. As soon as hiring gets completed we will start that also.

Abhishek: Sir, is there any news that Ayurveda is going to come under Ayushman?

Manish Grover: I have internals that government meeting has been done and it will be launch really soon. So, at that time very aggressively I will pick up five to ten hospitals in India and I will admit patients in those hospitals. Because I have brought my government business to 3%.

Abhishek: All right sir. Thank you sir.

Manish Grover: Thank you sir. Namaskar.

Moderator: Thank you. The next question is from the line of Ashok Parekh an individual investor. Please go ahead.

Manish Grover: Namaskar Parekh sir.

Ashok Parekh: Namaskar sir. A big congratulations to you on a great set of results and on the numbers that you are giving.

Manish Grover: Thank you sir.

Ashok Parekh: Sir, the contribution of government that you have brought down to 4%, I have a suggestion regarding this, all the private hospitals in big cities all are doing treatment. So, why are we becoming so conservative and our occupancy is also less

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Manish Grover: Sir, actually there is no point of raising outstanding in the books. What happens is money comes in a circle

Ashok Parekh: All the big hospitals are running on RGHS and CGHS. There is no such big hospital that is not running on RGHS.

Manish Grover: Sir, you are the investors who asked me a question on 31st March. How did so much cash come up? How did so much lenders come up in the books? So, I said, first we will finish this, then we will do it again. So, I did not let the business come down, sir. I increased the focus on the private sector, on health insurance.

Ashok Parekh: Sir, I want to say that we are losing an opportunity that others have taken. We have such good services. And small people are utilizing it.

Manish Grover: Sir, you do not let people live. This investment community does not let us live. If I do this, then you raise that questions. If I do that, then you raise questions. So, I will have to do a meeting with you.

Ashok Parekh: Sir, there should be a balanced approach. Our occupancy is not 100%.

Manish Grover: Sir, the problem is that I had a discussion with the government last month. I had a discussion with the ministry. So, they are making a new portal. In that portal, the TAT is of two and a half months. So, as soon as the portal launches, I will do a trial. If their payment comes on time, I will make the rail again.

Ashok Parekh: Sir, this is what I want to say.

Manish Grover: But sir I am waiting for the portal. I am waiting for the last payment to come out and the portal to be fixed.

Ashok Parekh: Sir, I have a personal question. I watch your videos and you do such treatments that are not happening anywhere

in the world. There is a patient of ours who has come to a very chronic

constipation. He is not able to function without anemone. And he is also a patient of depression.

Manish Grover: Note a number. This is my personal PA's number. Give it to him. If god is willing, he will be fine.

Ashok Parekh: Sir, I want to hear from you. He is a critical patient of depression. And he has chronic constipation. And these two are linked.

Manish Grover: Sir, do not worry. Our body is made by God. It is very intelligent. It knows how to cure himself. Note the number. 9855487770. This is Ravi's number. This is my PA's number. If there is any medical problem, call him. He will tie up with the doctor and get it settled.

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Ashok Parekh: Sir, I want to hear from you. Is this possible?

Manish Grover: Sir, this is our investor's conference call. For that, add him to the webinar.

Ashok Parekh: I am also an investor. And for whom I am talking, I want to talk to you personally.

Manish Grover: There is no problem. Take my PA's number and talk to him. I will see what is the best possible I can do.

Ashok Parekh: Okay sir. Thank you very much.

Manish Grover: Thank you sir. Namaskar. Thank you.

Moderator: Thank you. The next question is from the line of Praveen Kumar an individual investor. Please go ahead.

Praveen Kumar: Hi, Can you hear me?

Manish Grover: Yes, namaskar.

Praveen Kumar: Namaskar Manish ji. I was eagerly waiting to get in a call with you.

Many congratulations on your Q2 numbers. I have a couple of questions Manish ji. How is the early trend of your OTC business? First question. And my second question is, is there any revenue sharing planned with Chandan Healthcare? What is the strategy of planning there? And my last question is, do you want to update your FY '26 guidance from previously quoted Rs. 700 crores? Thank you.

Manish Grover: Sir, it would be better if you ask in Hindi. I am a little inclined to promote Indian language.

Praveen Kumar: But I am not that comfortable with Hindi. But I can understand. But I cannot speak fluently.

Manish Grover: I heard your question. I will answer it. Number one, you are asking about OTC business. What is the future of OTC business?

Praveen Kumar: No, how is the initial trend, sir?

Manish Grover: Sir, our trend is very good. We have started getting repeat orders. In the last month, we have increased the sale of Rs. 2.25 crores online for OTC only. In just one month. Last month was Diwali, everyone knows. It was festival time. So, we had to jump down a lot. Because patients do not stay in the hospital at that time. Because no one can keep a patient tied up in our hospital. Neither any bottle is applied, nor any injection is applied. So, our patients took a leave and went

home. So, we got support from OTC business. With which we got a covering. You know that this is a festival month. And November and December are cold. Still, we will do our best to grow
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it. What was the second question? How will we share revenue with Chandan Healthcare? The tie-up we have with Chandan Healthcare. Whatever percentage we will get as a lab commission, so we will pass on a very large part of it to the patient. And we will not give cashback to the patient. If the patient takes our medicines, then he will get a discount. Chandan will pay us, then our conversion will increase. The patient's stay will increase. Authenticity will increase through testing. And the lab person is sitting in-house, we will get the benefit from that also. We had a system, people used to go out for labs. Some used to go to Max. Some used to go somewhere. We did not get anything. The patients also did not get any discount. With this, we have made a plan to give cashback to the patient up to 30%

the revenue which we will get from
Chandan.

Praveen Kumar: Understood, sir. Thank you, sir. There was one more question on the guidance of FY'26. Would you like to update that?

Manish Grover: This year, as I said in the end of 2025, you will remember when there was a conference call in 2025, there was a profit of Rs. 91 crores. There was a turnover of Rs. 469 crores. So, I said that we will grow by 30% to 40% which is about Rs. 650 crores. And as I said, the profit according to that comes up to Rs. 125 crores, Rs. 130 crores. So, in front of you, we are just over-delivering what we are saying. Understood. So, we will put full effort in the future. The better the result, the better.

Praveen Kumar: Alright, sir. Thank you very much sir for your answers. Thank you.

Manish Grover: Thank you, sir.

Moderator: Thank you. The next question is from the line of Gaurav Sanghvi, an individual investor. Please go ahead.

Gaurav Sanghvi: Hi, sir. Am I audible?

Manish Grover: Yes, sir. Namaskar.

Gaurav Sanghvi: Namaskar, sir. How are you?

Manish Grover: Great. You tell me.

Gaurav Sanghvi: Sir, I just wanted to know that till the end of this financial year, you had given guidance of Rs. 650 crores, Rs. 720 crores. So, now do you think that you will easily overshoot?

Manish Grover: No, no, sir. Actually, our third quarter is a bit weak. Because it gets cold in November-December. So, the patient is a bit affected. So, we will remain where we committed. We will put effort to move forward. But as far as we have committed, our October is also closed. So, it has been eight days in November as well. So, we are maintaining our average.

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Gaurav Sanghvi: Okay, sir. My second question is that let us say we do Rs. 650 crores with a net profit margin of 20% to 25%. But in FY '27, the OTC component is being added. Because of that, what will be the top line guidance of FY '27?

Manish Grover: Sir, I cannot calculate like this. I had told you last year that in the next one, two years, my plan is to bring turnover of Rs. 300 crores, Rs. 500 crores from OTC, online marketing, and products. I had told in the last conference call that I am planning to bring a turnover of Rs. 300 crores, Rs. 500 crores is from OTC and product marketing.

Gaurav Sanghvi: Okay, sir. Thank you.

Manish Grover: Thank you.

Moderator: Thank you. The next question is from the line of Shrenik Mehta from Indo Alps Wealth. Please go ahead. Mr. Shrenik Mehta, please go ahead with your question. Your line is unmuted.

Manish Grover: Namaskar, Shrenik Mehta ji.

Shrenik Mehta: Namaskar, ji. Can you hear me?

Manish Grover: Yes, sir.

Shrenik Mehta: Sir, my question was for FY '27 which you have already answered. I just wanted to know the growth momentum that you have seen in the last two quarters. Do you see this momentum in the next year as well?

Manish Grover: Sir, you can ask yourself. If you listen to all my conference call from the start of this company, you will automatically get the answer. I had told all of you that we will do 2,850 beds by 31st March, 2026. Now, our beds have reached 2800. And our operational beds have reached around 2200 plus. In the next two, three months, these will also become operational. And I had committed to you that in three to five years, I will do 7,000 to 10,000 beds. And in allopathy, there are a total of 19 lakh beds. 11 lakh are private and 8 lakh are government-owned. Even if

we do 10,000 beds , we will still become 0.5% in front of them, sir. We are nothing, if you see. Now, we have to write the entire Ramayana. It has just started. I have told you clearly that my goal is to become India's No. 1 healthcare provider company. After that, I want to become Asia's No. 1 company, then the world's No.1 naturopathy and Ayurveda company is my plan. It ma

y

take five years, eight years, ten years but we will continue.

Shrenik Mehta: Very good, sir.

Manish Grover: Thank you, sir.

Moderator: Thank you. The next question is from the line of Deepak Poddar from Sapphire Capital. Please go ahead.

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Deepak Poddar: Am I audible, sir?

Manish Grover: Yes, sir. Namaskar.

Deepak Poddar: Namaskar. Sir, my question is that we have already achieved our bed of 2,800, which was our year-end target. So, at the year -end, at the end of FY '26, or at the end of FY '27, what will be our revised target in terms of bed ?

Manish Grover: Sir, actually, what happens is not all quarters are equal. You must know that my majority of the network is in North India. So, in North India, Deepavali is a big festival. In December, it gets cold. In January , it gets cold. In February, two, three days are less. So, if you see las t year, our H1 was good. H2 wa s slightly different. So, that is why I do not want to give a revised target. I will maintain the same old target that I s aid, between Rs. 650 crores, Rs. 700 crores, we will close this year.

Deepak Poddar: Okay. No, I was ask ing about the bed. Number of bed s is 2800.

Manish Grover: Bed. Sir, I have slowed the speed of the bed . First, I will fill this. In the next six months, I will increase it by 200, 300 more. But first, I will increase its occupancy rate.

Deepak Poddar: Okay . I understood. So, you can keep your target around 3000, 3100.

Manish Grover: For this year it is 3000. In three to five years, my target is of 7,000 to 10,000 beds .

Deepak Poddar: Okay. And will there be any target for FY'27 as well?

Manish Grover: Sir that is why I said it thickly. In three to five years, 7,000 to 10,000 beds . Actually, sir, people are contacting us. I have at least 50 mails from the people who want to give us the resorts for it .

Very soon, I am bringing a VIP premium center. All our cen ters are normal now. That bed count is not there in this . I am opening a premium center for investors, for people with money . We will take money from you b ecause you people want from us to take money but give the best facilities. We do not want to go to the economic class. We want to go to the business class, the premium class. So, I will s oon announce a center within two to four months. I am talking about. We are taking a fort of an old king, i n which we are planning this.

Deepak Poddar: Understood. Than k you, sir. And in this OTC business, you are saying that we can try to gener ate a revenue of around Rs. 300 crores, Rs. 500 crores. Right? Yes. So, are you targeting FY '27 next year?

Manish Grover: Sir, actually, we have only one product in the market righ t now. Right now, our R&D is going on. And when I will launch three, four more products, then I will be able to give more clarity.

Because I told you in the last conference call that all my products are for the benefit of the customers. For example, we hav e BP pills. Now, in India, 10 crores people take BP pills. So, how many customers wi ll you acquire, the sale will come automatically. For example, we have

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diabetes pills, kidney medicines, liver medicines, or depression medicines. All these are unique in themselves. Because there is no other player in Ayurveda who has undergone clinical trials and who has launched this product. So, we are first. So, according to the government guidelines, I am thinking of a plan on how to promote this.

Deepak Poddar: Okay.

Manish Grover: Because I have to take care of rules and regulations. Once people in the market get to know that we have BP pills, diabetes pills, then people automatically start buying them. For example, we have a pet liver shuddhi kit. Whoever is buying it, everyone is posting a review on Google and saying t

hank you. That it is a very good product and it has started working on the roots of our diseases.

Deepak Poddar: Correct. Thank you, sir. So, you have launched one product and y ou sai d that you will launch 15, 20 products. So, is the plan to launch it by the end of FY'26? Like in five months?

Manish Grover: By the end of this year, 10 products will be launched.

Deepak Poddar: 10 products by FY '26 end?

Manish Grover: Yes, it will be launched this year. By the way, our R&D has been done for 15 to 20 products.

Deepak Poddar: Okay. And just one last final thing. What can you think of your margins in OTC?

Manish Grover: It will be the same margin. Between 18% to 22%.

Deepak Poddar: PAT margin?

Manish Grover: Yes, PAT margin. We will keep the same margins. Between 20% to 25% I plan for the whole company and I keep it.

Deepak Poddar: Between 18% to 20%. Okay. It was very helpful. I wish you all the best. Thank you, sir.

Manish Grover: Thank you, sir. Thank you.

Moderator: Thank you. The next question is from the line of Prateek Tandil from Code Advisors LLP. Please go ahead.

Prateek Tandil: Yes. I wanted updates on the numbers in terms of Q2 FY '26. So, how many hospitals are there with us? In Q1, we had 55.

Nanak Chand: Now, we have 58.

Prateek Tandil: 58. In terms of clinic, we had 61 as of Q1.

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Nanak Chand: Now, we have 59.

Prateek Tandil: Right.

Nanak Chand: We have 35 franchisees.

Prateek Tandil: 35 franchises. Okay.

Nanak Chand: We have NAVH centers now. 46 NAVH and 5 is in pipeline.

Prateek Tandil: 46, 5 is in pipeline. In terms of IPD and OPD, if you could give the updated numbers for Q2.

Manish Grover: Sir, IPD is 58 hospitals. Rest are OPD and day care.

Prateek Tandil: No, sir. I am talking in terms of Q1, we were at 8,616 and OPD was 1.24 lakh.

Manish Grover: I will tell you.

Prateek Tandil: Yes.

Manish Grover: The volume of IPD in Q1 was 8,616 which came to us in three months. Now, we have 9614 patients admitted.

Prateek Tandil: Sir, can you give the numbers in English?

Manish Grover: 8,616 in Q1 and 9,614 in Q2. And total OPD volume is 1,24,364 in Quarter 1. Now, it is 1,40,144 in Quarter 2.

Prateek Tandil: Okay. Perfect. And our net profit margin guidance is 20% to 22%, right?

Manish Grover: Yes, yes. Our average revenue per bed on Quarter 1 is 8,287 and in Quarter 2 is 8,324. Means, Rs. 8,324. Our occupancy is same 57%.

Prateek Tandil: 57%. It was similar as Q1, right?

Manish Grover: Similar as Q1. But, if we calculate from last financial at 25 beds, it is more than 80% because 1600 beds total in 25. If we look at the same number of beds, it becomes 80%. But, we increase the number of beds every month, so the percentage changes.

Prateek Tandil: So, number of beds as of Q1 was 2,179, am I correct?

Manish Grover: Yes, yes. And, as of now, it is 2,220.

Prateek Tandil: 2,220, these are open ed. The remaining of 2,800 will open in two to three months, right?

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Manish Grover: Yes. Actually everything is ready but some beds have not received fire NOC, some beds have not received pollution NOC, some doctors have not been hired, somewhere hiring is going on, so it is in the process.

Prateek Tandil: Right, so the operational ones as of this quarter is 22?

Manish Grover: 2,220.

Prateek Tandil: Okay, perfect. Thank you.

Moderator: Thank you. The next question is from the line of Raunak Duggar an individual investor, please go ahead.

Raunak Duggar: First of all, thank you very much sir, you have got very good results.

Manish Grover: Thank you sir.

Raunak Duggar: I have a question that the medicines you were selling on the website for stomach, liver and sugar, you were selling those on your website. And now you

are bringing in on OTC. So, what is the

difference between those medicines? Were the earlier medicines less effective and these ones are more effective? What is the difference?

Manish Grover: No, these are the same medicines and clinical trials are being done of them only. We have changed the packaging of those and we will sell it ourselves as well as through OTC. We will sell at both the places. These are the same medicines, earlier those were Ayush approved and

now clinical trials for them has also been done. You can see on the ICMR website of Government of India that five, six clinical trials have been updated. Total nine clinical trials are going on. It is going on prostate cancer, breast cancer, prostate problem, migraine, weight loss. These new clinical trials have started.

Raunak Duggar: Okay sir, you have told about the inject device for water. What is the traction going on for that?

Manish Grover: The same sale is going on for that. Around Rs. 1 crores, Rs. 1.5 crores sale per month is going on for that. We have not given focus on it for now but in the next quarter our admit patients, we are bringing a cash back scheme. We are launching a card, referral system card. Till now we did not have any system in which if the patient sends a patient to us and he gets benefitted from that.

So, many patients used to say that I have sent you 15 patients, give me a discount. So, we are bringing a point system that whatever patient's card will be made, even if he gets tested by Chandan Healthcare, even if he takes medicines from us, and the consultation fees that he will give, all the points will be added to his card. And if any patient comes through that card, then his points will also be added to his card, and that patient will buy medicines from us, or will be admitted to me, he will get a discount. So, all this will be card driven. So, this is an integrated model, which will increase the patient's footfall. Because for now we do not give anything to the patient. If a doctor gives us a referral, we give him incentives. But if the patient says that I am

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sending 25 patients, I have sent four, give me some benefit. So, we did not have such a system for on software. So, we did a tie-up with Salesforce a while ago. So, we got the benefit of bringing Salesforce software. One, our reporting has improved, our data has become very segregated, and it has helped us a lot in bringing the card system. Because in the old software, all this integration was not possible. Now we will have integration in Salesforce. So, we will also have a tie-up with Chandan's software. So, that the customers will be able to benefit each other. Our customers will get Chandan's tests cheaply, first test will be free, and Chandan's patients will also come to us and take medicines.

Raunak Duggar: Sir, I had one more question, on 31st March 25th, and in Q1 of this year, you had said very positively about your hospitals tie-up with colleges. So, my expectation was that you will pay more attention to it. But in today's call, it seems that you are paying less attention to it. Because last time too, you had three tie-ups till Q1, and there are still three tie-ups. So, why do you feel that we are paying less attention to it? Because the way you told about the benefits of this, you will get good graduate doctors and there is a bed from the beginning, and your CAPEX will also be less. Then why are we paying less attention to it?

Manish Grover: Sir, it is not that we are not paying attention to it. But the time that is taking, actually there are a lot of formalities of the government. What is that, bring NOC, bring fire NOC, and bring NOC of pollution in it, it is taking time. And these government colleges, they are all in trust. They are NGOs. They cannot run a profitable organization. So, that is also a big c

hallenge. They cannot

take income. We are taking it from them on rent. Earlier we had offered a rent and revenue sharing model. So, the 10, 15 more pipelines that we had, we stopped them so that first we can practically run two, three efficiently. And earlier we had to give results in six months. Now we have to give results in three months. So, the planning that we are doing, it does not change so quickly in three months. Now like the college in Aurangabad, it is getting ready now. Now next week haven will be performed. Then they will start the work. So, the one who has to prepare the college for me, he takes the speed, he takes the time. The one in Vrindavan is ready, of 150 beds. But we have not added its beds yet. The day I start acquiring, I will add it.

Moderator: Sorry to interrupt Mr. Raunak. May we request you to please rejoin the queue? We have participants waiting for the turn. Thank you. The next question is from the line of Amit Joshi, an individual investor. Please go ahead.

Amit Joshi: Sir, Namaskar.

Manish Grover: Ji, Namaskar.

Amit Joshi: Sir, first of all, congratulations to the entire team.

Manish Grover: Thank you.

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Amit Joshi: I think everyone has asked my question. So, I am asking a new question. Sir, you said that you want to become India's number one company, right? And then Asia and then the world, right?

Sir, what are you doing differently from others so that you can become number one?

Manish Grover: Sir, we are teaching the patient how to live. We are teaching them how to eat food. We are teaching them lifestyle. We are telling them why to fall sick. We are focusing on prevention.

Right now, the whole world is treating the sick. Whether it is any country, America, India, Japan,

everyone is treating the sick. Whose duty is to not let them fall sick?

Amit Joshi: Okay.

Manish Grover: So, I am focusing on that. Why to fall sick? If you do not fall sick first, then why do you have to go to the hospital? The Ayurvedic sutra says, "sarve roga api mandaginihi ajir maline kupit mal sanchate". This means that the cause of all diseases is mandaginihi that is weak stomach, weak liver, weak pancreas, low metabolism, and poor digestion. So, I make videos to teach people how to live on social media and YouTube. So, that people do not fall sick. And if we keep this approach, then we will be able to make a healthy India at least.

Amit Joshi: Right. It is good sir.

Manish Grover: Thank you, sir.

Amit Joshi: Sir, you also told a number. I could not note it down.

Manish Grover: 9855487770.

Amit Joshi: Fine, sir. We will contact you on this number if there is any requirement. Thank you.

Manish Grover: Thank you, sir.

Moderator: Thank you. The next question is from the line of Kamal Sharma from Srishti Wealth. Please go ahead.

Kamal Sharma: Namaskar.

Manish Grover: Ji, Namaskar.

Kamal Sharma: Sir, I had two questions. The first question was that you were expanding internationally in Dubai and Nepal. How is it going there? Do you have an expansion plan there going forward?

Manish Grover: Sir, Nepal has already started. The profits of Nepal have also started adding up in the book. And in Dubai, Abu Dhabi, one is 100% operational. And it has also become profitable. And work is going on there on three. But that company only made it last month. The process is so hard to make that company. So, the company was formed 20, 25 days ago. So, our payment will be

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transferred this month. So, three clinics will be handed over to us. And on three, we will start working on the new one.

Kamal Sharma: Okay, sir.

Manish Grover: But the investors has told me, you people have told me to focus more on India. That is why I have put the US project on the shelf.

There is a lot of potential in India.

Kamal Sharma: Yes, that is right. And my second question, sir, was this. I was reading the PPT. So, our H1, the PPT in the last year, we had a PAT of total Rs. 47 crores of H1. And the PPT of Quarter 1 and Quarter 2 that you have given this year. So, when I saw the PPT of Quarter 1 and Quarter 2 of last year. So, by adding it, it is coming to Rs. 43 crores or Rs.42 crores. So, am I calculating something wrong or am I missing something?

Manish Grover: Look, I do not know. Mr. Nanak will be able to tell. Mr. Nanak, please tell.

Nanak Chand: Hi, sir. You are seeing it right. Actually, we have shifted to Indian -AS. So, there are some adjustments that are impacting the previous.

Manish Grover: Sir, I will tell you. The GT that has come, the GT that has come in our books. So, we have taken Grant Thornton. So, the accounting system that we had has been made indigenous by Grant Thornton because of it coming to the main board. So, Indigenous has made some calculations again. Like our rent agreements. Like we used to do rent agreements for five years, seven years.

Now we have started doing it for 10 years. Because our depreciation has increased. Otherwise, the depreciation comes first and ends. Is there any problem with this call?

Kamal Sharma: No. Okay. I am listening.

Manish Grover: Okay. So, due to Indigenous, those changes are visible. Otherwise, everything is the same.

Kamal Sharma: Okay, sir.

Manish Grover: Actually, the accounting practice has changed because of main board and GT.

Kamal Sharma: Okay. So, that is the main reason. Okay, sir. Thank you so much. And very, very congratulations on a good set of numbers sir.

Manish Grover: Thank you very much, sir. Thank you.

Moderator: Thank you. The next question is from the line of Devi Agrawal from Fimcom Family Office. Please go ahead.

Devi Agrawal: Namaste, Manish ji.

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Manish Grover: Namaste, ji.

Devi Agrawal: Sir, I wanted to know that our gross margins are now 85%. Whereas it was 91% in Q2 FY '25. So, what is the reason for it to be 87%?

Manish Grover: Mr. Nanak, please go ahead.

Nanak Chand: Sir, first of all, the financial restate that we have done. There was a part of consumable that was being booked in OPEX earlier. So, now it has been included in the gross direct expense above the line. So, that was the first reason. And slightly, our consumable part has increased by 0.5%. So, this is the reason that we have got a little variation in GP.

Manish Grover: Sir, it is because of the adoption of Indigenous. It is because of the changes that GT has made in the books, sir. Our margins are the same as before. We have increased it in front of you. In the last quarter, there was 29% profit. In this quarter, it is 31%. Yesterday, I was telling GT that why does not the Indian government start Indigenous to the companies from the first day? Why does it apply later when they come to the main board? Now, I will write a letter to the Indian government.

Devi Agrawal: Thank you, sir. Sir, I wanted to know that we have tied up with Salesforce. So, are we anticipating any cost savings from that?

Manish Grover: Who did we tie up with?

Devi Agrawal: Salesforce.

Manish Grover: We have benefited a lot from tying up with Salesforce, sir. And you can see from the last two quarters. Our per-lead system, the generation system, earlier, we used to generate leads by running TV and social media. Its journey has become very easy now. Now, it has become good for us to extract data one by one. Earlier, we used to get confused patients. We used to get mixed data. Now, we get the data in a very segregated way. Reporting is very good. With this, it has become very easy for us to make our policy, to give feedback to the customer, to send videos, to tell them about a p

articular disease. So, we have benefited a lot from Salesforce. And I will also tell you to use Salesforce. Because we just held a press conference on the part of Salesforce.

Devi Agrawal: Sir, I wanted to know that we have appointed an internal auditor. Deepak Garg. So, which auditors were there before this? And why did they resign?

Manish Grover: It was the same, sir.

Devi Agrawal: Okay, it was the same before.

Manish Grover: Now, we will change it. We have not changed it yet. We will bring an internal auditor from a different firm. But our main signing firm is now GT from this quarter.

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Devi Agrawal: I am talking about the internal auditor, sir. You had appointed Deepak Garg for FY' 26, right?

Manish Grover: He is still the same.

Devi Agrawal: You are saying it was him only earlier? And he is the one right now or will you change in the future?

Manish Grover: Yes, he is the same. Sir, why do you make me speak on the call? Deepak ji will get angry.

Devi Agrawal: Okay sir. Okay, I understood. And sir, you said that you have 2,800 beds. So, this has just been made. In operation it is only 2,200.

Manish Grover: Sir, everything is ready. Their approvals are there. Government's NABH is there. Pollution is there. Fire NOC is there. Hiding is there. All that is going on regularly, right? Sir, on 31st March, 2025, there were 1,600 beds operational. Now, there are 2,200 beds operational. In six months, 600 beds have increased, sir.

Devi Agrawal: Okay, I understood. And sir, I just have a request. Like we have restated our financial statement in Ind -AS. So, can we get its schedules as well? Because in FY'25 annual report it will come or not?

Manish Grover: Nanak ji will tell you. Nanak ji please tell.

Nanak Chand: Yes, sir. We will share the reconciliation. Yes. So, sir, we have already given disclosure of its resulted reconciliation. So, if there is any further requirement, we will share with you.

Devi Agrawal: Sure, sir. Thanks a lot. All the best.

Manish Grover: Thank you sir.

Moderator: Thank you. The next question is from the line of Sanjay Nandi from VT Capital. Please go ahead.

Sanjay Nandi: Namaskar, Manish ji.

Manish Grover: Ji Namaskar.

Sanjay Nandi: Sir, I have got the answers of my questions. Thank you so much sir. A big congratulations to you.

Manish Grover: Thank you.

Moderator: Thank you. The next question is from the line of Akhilesh Rawat from Vedant Vision Private Limited. Please go ahead.

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Akhilesh Rawat: Namaskar, sir.

Manish Grover: Namaskar.

Akhilesh Rawat: Sir, first of all, I would like to congratulate you on a good set of results.

Manish Grover: Thank you, sir.

Akhilesh Rawat: Sir, my first question is that we have an operational capacity of 2220 plus. So, what is the utilization target are we aiming in the future and how much impact are you seeing on our EBITDA margin in the coming quarters?

Manish Grover: Sir, look, I do not know how to calculate this percentage. All I know is that my earlier target to take colleges in that also my average was one lakh per bed. And even if I do it myself, I get three lakhs to four lakhs per bed. So, as soon as its speed decreased, I increased the speed here. So, I am managing on my own. I am running at my own speed. And my goal is to bring 70% to 80% occupancy in the next six to eight months.

Akhilesh Rawat: Okay.

Manish Grover: Which is 57% now.

Akhilesh Rawat: Sir, my second question was that, as we have seen in this quarter, we have seen a lot of revenue and growth expansion from private. So, in the future, what is our revenue target is private and government business.

Manish Grover: Yes. Look, I will start the government business only when the timing of government payments is set

ted. Because in allopathy hospitals, there are fraudulent bills and operations. We do not have anything like that. We have a fixed rate of Rs 8000 per day for the general government. So, we cannot even take Rs 8100 from that. But to earn 20% or 15% I stuck Rs. 8,000. After that, I have to pay 25% income tax in advance. So, until the system of government money is not made, I will not increase the government business. My next target is to increase private insurance. And I have increased the government business by 20% from the first quarter to the second quarter. Sorry. Private insurance business. And I am focusing more on that.

Akhilesh Rawat: Okay. And sir, I have one last question. Sir, as we can see now, the network of our two hospitals is mostly your north side location. So, do we have any plan in the future that we will go to another part of the country?

Manish Grover: Of course, sir. We have already started. Our Guwahati hospital has started. Hyderabad, Chennai, Bangalore, we are looking for a place in Bhubaneswar. A building has been finalized in Calcutta yesterday. So, we are spreading all over India, sir. And now we have started a new work. The patients who used to call us earlier, we only had a Hindi and English team. Now we have started to keep children of all languages. Assamese, Bengali, Punjabi, Marathi, Tamil, Telugu, Kannada. Jeena Sikho Life Care Limited

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We have started to keep children of all languages. Which will be very beneficial for the future.

Because the patient from there will also start coming. And our big hospitals will open there.

Akhilesh Rawat: That is too good. Okay, sir. That is it from my side. All the best to you.

Manish Grover: Thank you, sir. And I have started one more thing. I have dubbed my voice from AI. I have started making videos in Assamese, Bengali. My team and my team's main doctors their videos have also started going in dubbing. And very soon we will be in Kazakhstan too. There is a simple health center there. Our doctor and therapist will be assigned there. So, we have a tie-up in Kazakhstan too. We are sending a doctor and therapist there too.

Akhilesh Rawat: Okay, sir. Thank you.

Manish Grover: Thank you.

Moderator: Thank you. The next question is from the line of Chirag Shah from White Pine Investment Management. Please go ahead.

Chirag Shah: Namaskar, Acharya ji.

Manish Grover: Namaskar, Chirag ji. How are you?

Chirag Shah: I am doing good.

Manish Grover: You are our lucky charm. You join us in every call from the very start. Thank you very much.

Chirag Shah: No, sir. You are doing a great job. You are also taking care of the investors. You are doing a great job.

Manish Grover: Thank you, sir.

Chirag Shah: Sir, I had a question about the tie-up regarding Chandan. Is this tie-up only for 34 hospitals? Or as you increase the number of hospitals they will add their own diagnostic centers?

Manish Grover: Sir, it is for 100% hospitals. 100%. They will set up when any new opens. They will make all the investments. Whether it costs Rs. 50 lakhs, Rs. 1 crore or Rs. 1.5 crore. It will be their investment. In Delhi, they bought a lab worth Rs. 4 crore to Rs. 5 crore just after our tie-up.

They made a deal. I do not know the exact date of handover. They bought a complete diagnostic center. There are 19 centers in Delhi NCR. And their employees are sitting in all 19 centers. And I will start making videos this week to give blood samples for the first blood testing in these 19 centers. You will get a free test. And the payment will be done by Chandan. The patient will come to me. Reporting will be done at our place. Apart from this, in Prashant Vihar they have set up a full-fledged VIP where they have X-ray, MRI, ultrasound, CT scan. They have set up

everything. And what is the biggest benefit? In Delhi, they are

setting up cancer marker tests.

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They are also setting up expensive machines. Because for cancer markers, we had to send all the testing to Mumbai. And that test cost at least Rs 75,000. And the patient was not so comfortable.

Now what we did was we took cashback from them. It will come in the card. And the patient will get discount on medicines from us. Because if the rate is reduced, the entire market will go down because the commission in the lab business is very high.

Chirag Shah: Right.

Manish Grover: There is a 50% to 60% commission business in the market. And I was unaware of this line. And I had to learn this. So, I thought the best way was to increase my business because of diagnostics.

Because I will get the benefit that my footfall has also increased. The patient will also come to me. Because they will put 30% cashback in the card. It will run with us. And they will also give cashback to their own patients that will also work with us.

Chirag Shah: Okay. If they give Rs 100, then Rs 30 cashback will go to the card. When they get treatment, they will get to use Rs 30.

Manish Grover: And they will pay us completely. That means I got all the money. And the patient got a discount. And what will happen? My patient repeat will increase. And the patient's trust in us will increase that it is completely different from the world.

Chirag Shah: Okay. Sir, what we have invested in the company is one time, right? We will not invest any more in the future. Correct?

Manish Grover: Sir, I did not have any plan to invest in that company. I was only asking for 51%. No company is ready to give its 51% right now. And without any investment, even if you do a little, if we do a little then we will share your data otherwise why will they share data? Then how can I lose their 40 lakh patients? They say, when you do not trust us, you will make an agreement with someone else in the future. That is why they have put this condition. So, we have agreed for a minimum investment. And that is also Rs. 10 crores and more than that, they have also spent for us.

Chirag Shah: Right.

Manish Grover: They have bought two labs in Delhi. One is of Rs 5 crores, Rs.6 crores and another one is of Rs 12 crores, Rs. 15 crores, they were investing in that from before. At our request they added machinery worth Rs. 5 crores in that. And we will get all the benefits.

Chirag Shah: Sir, this happened really well. But we have committed one time money to them, we do not have to give anything more than that,

Manish Grover: And now what I have been advised by the consultant, everyone has said that if a company gives 51% stake then we will take it. We will not make small investments in any company.

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Chirag Shah: No, sir, I have a request to you. Jeena Sikho's franchise is very strong. It has a very good reputation. Do not undervalue it. You have thought this well for the long term. But I have a request to you that please do the value of Jeena Sikho's franchise really well. You are doing a good job.

Manish Grover: We are going to launch Jeena Sikho's franchise in two, four days, sir. We are planning to open at least 50 new hospitals. And Jeena Sikho will open in a franchise model. I did not have to tell this in this conference call. You said it, so I told you. People put a lot of expectations and then pressure comes on me.

Chirag Shah: No, sir. Do not increase our expectations. Keep it lower.

Manish Grover: That is why I did not want to say it. When you said it, I had to say it. We are going to open 50 new franchises. Which we will launch next month. So, we are planning 50 new hospitals in a franchise model. We will not invest in that at all.

Chirag Shah: Right. Sir, you had explained that earlier

also.

Manish Grover: So, actually when GT came, they told us a lot of things to improve this and that. So, it took a lot of time in that.

Chirag Shah: No, sir. It is a good thing.

Manish Grover: Look sir, earlier, people had doubts that this company is showing profit. Now, with the GT's stamp it is like that we are a genuine company. People had doubts about our company.

Chirag Shah: Sir, I am just clarifying on one thing. In Chandan, we have invested a total of Rs. 10 crores and balance Rs. 30 crores we have to pay.

Manish Grover: We have not given them a single rupee yet.

Chirag Shah: You will pay after the warrant approval. That is right.
Moderator: Sorry to interrupt but Chirag sir, your voice is breaking.
Manish Grover: Ma'am, I cannot hear Chirag sir.
Moderator: Yes, sir.
Manish Grover: Chirag sir, let us discuss about this and find the best solution.
Chirag Shah: Yes, sir.
Manish Grover: Thank you, sir.
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Moderator: Thank you. The next question is from the line of Somyajit Biswas from Amaze Power. Please go ahead.
Somyajit Biswas : Namaskar, A charya ji.
Manish Grover: Namaskar. How are you?
Somyajit Biswas : I am very good. I congratulate you on a great set of numbers and to achieve before commitment date. Rest of my questions have been answered but I have a small question. You are planning to expand OTC drugs. Do we have any plan to go through quick commerce and increase our sales through quick commerce?
Manish Grover: Let me tell you. We are discussing with a company and it is at a final stage. A company like Entero. Entero is a listed company. They have one lakh pharmacies in India. Entero is a tech-first company. It is tech-based and has a B2B app. They have 2700 plus healthcare products. They have 100 plus warehouses and 500 plus districts. They will make an exclusive agreement with us for Ayurveda. They will not join any other company in Ayurveda. We will supply the Ayurveda products they need. This will be a big plus point because we will get one lakh medical stores on the first day. We will get a distributor network for our product and it will be promoted by Entero. The acquisition cost to bring customer, we will transfer it to Entero.
Somyajit Biswas : My question was regarding quick commerce like Blinkit.
Manish Grover: We have started . We have started on Zepto, Blinkit and everything. Last month we had a sale of Rs. 2.5 crores plus.
Somyajit Biswas : I see. I had a small question regarding healthcare , East and South market is a very big market but you do not have much presence here. You just told us about a hospital in Kolkata in this conference call.
Manish Grover: No. I told you that I am going to start a franchisee chain where I will focus there and invest their money . I will not invest money and we will do revenue sharing model with them.
Somyajit Biswas : Okay. All right. Thank you very much.
Manish Grover: Thank you.
Moderator: Thank you. The next question is from Akshay from AK Investment. Please go ahead.
Akshay: Thank you so much ma'am for the follow-up. Sir, I had a question , in the last conference call we discussed that the luxury resort and luxury services do not earn much money and are all loss-making. We are entering in the next one, two quarters. What is our benefit and will we be able to sustain the profit margin like our business ?
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Manish Grover: Yes, sir. We will be able to do it that is why I am opening in my neighborhood. That is why I did not want to go into the luxury business but I am opening a center for everyone and I am opening it in my neighborhood around Chandigarh so that I can manage it and it will give the same profit as the others.
Akshay: Okay, sir. That was my question and I have got the answers for other questions. Thank you, sir.
Manish Grover: Thank you.
Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to the management for closing comments.
Manish Grover: Do we have any questions ma'am or not ?
Moderator: No, sir. I have handed over to you for closing comments.
Manish Grover: Okay. So, I want to tell everybody that I have a request to all of you do not wait to fall sick. Clean your body before falling sick. Like, Diwali has gone, we clean our house every day. We do sweeping and mopping in our house every day. We do dusting in our house everyday but still in Diwali there is some extra trash in Diwali. Just like that we clean our body through potty, urine and sweat but still we fall sick. So, I will request to all of you in every six months every person should get panchkarma done and try to get it done from us. So, that you know how to live and detox your body through panchkarma. Now, in every big cities we have panchkarma centers. And in Panvel, Mumbai a big centers of 200 beds have started and is running and it has lawn and all because majority of you all are from Mumbai in this call. So, I will request that you

all should take some time out and we will facilitate you in Panvel and we will get your panchkarma done there. You all will stay healthy so our country's wealth will also remain wealthy because you are the wealth managers of the country. Thank you very much. I will say an Ayurveda sutra in last, "Sarveshanam roganam nidanam kupitah mala, which means in Ayurveda there is one solution to every disease that is to detox the body, throw kupitah mala out. So, you all should detox, stay healthy, stay busy and stay cool. Thank you. Thank you very much. Thank you ma'am.

Moderator: Thank you. On behalf of Nuvama Wealth and Investment Limited. That concludes this conference. Thank you for joining us and you may now disconnect your lines.

Call: Feb 2026

Date: 13th February, 2026

To,
Manager - Listing Compliance
National Stock Exchange of India
Limited 'Exchange Plaza'. C-1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai - 400 051
SYMBOL: JSLL To,
Head of the Department,
Department of Listing Operation,
BSE Limited
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai 400001
SCRIP Code: 544476

Sub: Intimation to Stock Exchange – Transcript of Earnings Conference Call on Unaudited
Standalone and Consolidated Financial Results for the Quarter and Nine Months Ended 31st
December, 2025

Dear Sir/Madam,
Pursuant to the Regulation 30 read with Schedule III and Regulation 46 of the SEBI (Listing Obligations
and Disclosure Requirements), Regulations, 2015, as amended, please find enclosed transcript of
Earnings Conference Call held on Monday, 9th February 2026 at 03:30 p.m. (IST) in respect of
Unaudited Standalone and Consolidated Financial Results for the Quarter and Nine Months Ended 31st
December, 2025.

The aforementioned transcript to the conference call is available on our website at:
<https://jeenasikho.com/wp-content/uploads/2026/02/Jeena-Sikho-Q3FY26-Earning-Call-Transcript.pdf>

Kindly take the above intimation on your record.

Thanking you,
Yours faithfully,
For Jeena Sikho Lifecare Limited

Manish Grover
Managing Director
DIN: 07557886

Place: Zirakpur, Punjab
Date: 13.02.2026

Encl: a/a

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Q3 FY '26 Earnings Call "
February 09 , 202 6

MANAGEMENT : MR. MANISH GROVER – MANAGING DIRECTOR -
JEENA SIKHO LIFECARE LIMITED
MR. NANAK CHAND – CHIEF FINANCIAL OFFICER -
JEENA SIKHO LIFECARE LIMITED

MODERATOR : MR. RANVIR SINGH – NUVAMA WEALTH

Moderator: Ladies and gentlemen, good day , and welcome to the Jeena Sikho Lifecare Limited Q3 FY '26 Earnings Call . As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star and then zero on your touch -tone phone.

I now hand the conference over to Mr. Ranvir Singh . Thank you , and over to you.

Ranvir Singh: Yeah . Thank you, moderator. So on behalf of Jeena Sikho Lifecare Limited, I extend a very warm welcome to all participants on the Q3 FY '26 financial results discussion call. Joining us today on the call are Mr. Manish Grover ji, popularly known as Acharyaji. He is the Managing Director . Mr. Nanak Chand, he is the CFO.

Before we begin, I would like to draw your attention to the standard disclaimer. This call may contain certain forward -looking statements which are based on management's current expectations and beliefs. These statements are subject to risks and uncertainties and actual results may differ materially.

With that, I would now like to hand over the call to Acharya Manish ji for his opening remarks. Over to you, sir.

Manish Grover : Namaskar. I hope everyone is well and healthy. Thank you, Ranvir ji. I am very happy to inform you that in quarter 3 FY '26, Jeena Sikho Lifecare's financial and operational performance has been very strong. It is the result of our hard work and constant thinking to improve, that we are seeing good growth in both our service and product businesses. In this quarter, our total operational income has been INR221.7 crores , which is 92% higher than the same quarter last year and 17% higher than the previous quarter. This increase has come from the growth in patients and sales in both our businesses ; Ayurvedic Healthcare Services and Ayurvedic Products . And from people's trust , people are sending more patients to us.

Our EBITDA this quarter stood at INR100.8 crores , and for the first time, we have crossed INR100 crores in sales within a quarter. This is 240% higher than the same quarter last year, and the EBITDA margin has been 45% plus. That means, despite continuous investment in expansion, the company has earned good profits. Despite all this, while we are opening new centers, investing in facilities, doing new things, our operational efficiency has remained strong.

Our profit after tax stood at INR66.7 crores, which shows an increase of approximately 400% year-on-year. The PAT margin remained at 30%. Talking about operational performance, the patients for OPD and COD video consultation this time were 4.34 lakhs, meaning a growth of 247%. IPD patients increased by 84%. Day care patients increased by 139%, and video consultations grew by 214%.

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These figures show that our hub and spoke model for Ayurvedic treatment across the country is proving successful. And what is the plan ahead? Our focus is to grow our service and product businesses just like this. We currently have around 2,700 to 2,800 beds under development. Our bed capacity will increase further.

We are soon going to launch the Jeena Sikho Health Card, which will give patients a sense of Swadeshi and Indianness. Patients will send more patients to get the referral bonus. Because of this, patients will benefit by promoting Ayurveda, and they will get more facilities, better value, and easy access. They will get discounts on our tests. We will provide information about this in the coming time.

Regarding our Ayurvedic products, we had previously launched one OTC product. We are launching our second product this month. The demand for new product launches and the strength of old products is because we are working well on o

ur quality. And by the 31st of last month, we have published approximately 180 research papers. Because the language the world understands , Research and Evidence -Based Work , we are doing that alongside.

In conclusion, I would like to say Jeena Sikho is fully ready for continuous growth. I have been ready to give better value to all my stakeholders for a long time. Now I hand over this call to our CFO, Shri Nanak Chand ji.

Nanak Chand : Thank you, Manish sir. Good afternoon , everyone. I will now take you through the key financial highlights for Q3 FY '26 prepared in accordance with the Ind AS following our migration to the main board in August 2025. For the quarter, our revenue from the operations

stood at INR221.7 crores, up by 92% year -to-year and 17% quarter -to-quarter. Gross profit increased to INR197.5 crores with an improved gross margin of 89%.

Our EBITDA came in at INR100.8 crores, recording 240% year -on-year growth with margins at 45%. Profit after tax stood at INR66.7 crores , reflecting a 405% year -on-year growth. Our basic EPS for the quarter is INR5.37. The improvement in the profitability was driven by the higher operating leverage from the increased patient volume, strong contribution from the high margin products and sale, controlled operating expense despite our network expansion.

Our balance sheet remains strong with prudent financial management, improving cash flow and capital -light expansion strategy supporting the superior ROCE. With this, we open the floor for a question -and-answer session. Thank you.

Moderator : Thank you. We will now begin the question -and-answer session. We have the first question from the line of Priyanshu Jain from Growth X Infinity. Please go ahead.

Priyanshu Jain: Sir, can you hear me?

Manish Grover : Yes, Namaskar sir.

Priyanshu Jain: Namaskar sir. First of all, congratulations to you for this quarter, it has been a very good quarter for us. I have joined some con -calls before, but this is the first time a question has

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come up. Sir, I had a few questions. First, regarding our product launches, the new ones we haven't done yet , can you tell us from which product the majority of revenue is coming from among those we recently launched? That is my first question, sir.

Manish Grover: Sir, we have launched only one product so far. My target was that when the first product crosses INR10 crores in monthly sales, I will launch the second product. So now, this very month, my second product will be launched. The first product was my Pet Yakrit Pleeha Shuddhi Kit , whose sales crossed INR10 crores monthly. This month, my second product NutriRoz will be launched. When the sales of this product also reach around INR5 crores monthly, I will launch the third product. In the next three to five months, about 6 to 8 of my products are ready. In total, I have taken a target of 16 products within this year.

Priyanshu Jain: Sir, in this year?

Manish Grover : In this year.

Priyanshu Jain: This year , FY '27?

Manish Grover: In 2026 itself, yes. Not March 31st, but by December 31st, 2026.

Priyanshu Jain: Yes, yes sir. That's what I was saying. Okay sir. Sir, what is our current occupancy of beds? And going forward, what are we thinking regarding the plan to take it to what level?

Manish Grover: Our occupancy running currently , in the last quarter , it was 57%, this quarter it has been 58%. It was 57% in the quarter before that too. For three quarters , we have maintained 57% plus.

This quarter is usually the worst quarter for the healthcare industry. Despite that, we maintained 58%.

Priyanshu Jain: Absolutely sir. Sir, you mentioned in the last call that the government portal was getting ready.

So sir, would you like to give any update on that? And sir, regarding our occupancy...

Manish Grover: We have greatly reduced the government business and focus

sed primarily on recovery. So in

the last few months, we have recovered approximately INR20 crores, INR 21 crores.

Priyanshu Jain: Sir, my question was that, as you mentioned, once their portal launches, because we face problems with payments, do you have any update regarding that? When will it happen?

Manish Grover: Sir, I cannot control the government, right? That is the government's sector. When will the government do it? And why do I need to do government business? There are problems upon problems in government business. Money doesn't come on time. There are many challenges, their protocols. When I am doing so well with private business, doing such good work with health insurance companies, I don't think I should increase the government business right now until their portal is fully settled and until the government starts giving payments on time.

Priyanshu Jain: Absolutely sir. Sir, I have just two more questions. One is, what is our capex per bed currently? And going forward, if it remains the same, could you shed some light on that?

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Manish Grover: In the last quarter, it was 8,300 -- 8,324. In this quarter also it is 8 ,337. The amount of money large hospitals spend just to acquire a patient , spending INR8,000, INR 9,000 as patient acquisition cost , we treat the patient for that amount.

Priyanshu Jain: Sir, I was asking about capex. How much does it cost us for one bed? It was INR3 lakhs previously...

Manish Grover: Our capex is very clear in every call. We set up one bed for INR3 to INR4 lakh. If we have to open a 100 -bed hospital, we open it in INR3 to INR4 crores.

Priyanshu Jain: Okay sir. So for next year, what is our plan? How many beds are we targeting for FY27?

Manish Grover: Sir, I don't work on a yearly bed basis. I had said that in the next 3 to 5 years, we will reach 7,000 to 10,000 beds. Currently, we are around 2,800 beds. Out of which 2,290 are operational.

Priyanshu Jain: Okay sir. And sir, just one last question. Nowadays, there is a huge market for supplements. So sir, like Omega 3 or Magnesium, do we have any plans regarding these products? Because...

Manish Grover: We have launched them. Last month we launched 8 food supplement products. And we launched them on social media/online. The one product I launched was in OTC. On social media, we have already launched products. Supplements launched, beauty products launched.

Sir, we have just started, there is still a lot to do.

Priyanshu Jain: Absolutely sir, absolutely. Sir, those were my only questions. Just one last thing, sir. Last time you gave Ravi sir's number. I spoke to him. I am currently a 3rd -year student at Delhi University. I wanted you to come once and guide our students a bit. I spoke to him and he gave a date in April?

Manish Grover: If the date is fixed, I will come there to give a lecture and take a session.

Priyanshu Jain: Absolutely sir. The date is in April, sir.

Manish Grover: 98554 87770 is Ravi sir's number.

Priyanshu Jain: Absolutely sir. The talk happened sir. Okay sir. All the best sir.

Manish Grover: You fix the time. I will definitely come.

Priyanshu Jain: Absolutely , sir. Thank you , sir.

Manish Grover: Thank you. Namaskar.

Moderator: Thank you. We have the next question from the line of Akshay from AK Investment. Please go ahead.

Akshay: Namaskar , sir. How are you?

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Manish Grover: Namaskar. How are you?

Akshay: Just fine. Sir, last time you mentioned that we will do around INR750 crores in FY '26. So do we have any plan to raise our guidance now? Because we have already done INR586 crores in 9 months of FY '26.

Manish Grover: To be honest, I don't pay attention to numbers. I only focus on curing patients, patient valuation. I don't know the calculations, that is your job. I won't be able

to speak on numbers

because -- when the third quarter is a weak quarter and we perform so well in it, then this is a normal quarter. You do the calculation yourself sir.

Akshay: Okay , sir. And sir, what was the capacity utilization of operational beds as on Quarter 3 of FY '26 till December?

Manish Grover: Sir, I deliberately did not increase operational beds in quarter 3 because this is a weak quarter. If I had increased them, my expenses would have increased. So in the third quarter, when Diwali comes, winter comes , all hospitals remain weak. At that time, if I increased operational beds, I would have harmed myself, reduced my profit margin. So in the last quarter, I did not increase operational beds. They are built. In this quarter, we will make them operational.

Akshay: I understood that sir. But out of our 2 ,290 operational beds, how many were occupied till Quarter 3?

Manish Grover: 1,325. We had 58% occupancy in the last quarter. In the quarter before that, it was 57%. In the quarter before that also, 57%. In the concall on 31st March 2025, I had said we would do 2 ,850 beds in the whole year. I have completed that. 2 ,850 beds are ready. But out of that, 2 ,290 are operational. Because as soon as we make them operational, all costs start -- Nurses, GDA, Doctors, Therapists. So I focused on increasing the occupancy of those already operational first.

Akshay: Okay. Okay. And sir, regarding our international expansion, you mentioned in the commentary that we are starting new hospitals in UAE?

Manish Grover: We have done it. We have loaded the stake in the UAE company on the NSE website. We have made two international day care centers operational. One in Abu Dhabi in Khalifa City, and one in Khalidiya. Currently, four are being built in Dubai. And we have started Kazakhstan also. We started Nepal also. Planning for US is ongoing. Planning for other countries is ongoing. Now we are emphasizing international expansion.

And in India too, now we are bringing a new chain of "Super Specialty Clinic Day Care Centers." In which obesity, joint pain , such diseases which are not chronic , meaning mid - section diseases . For those we are bringing a whole new chain in cities. Which will have infertility, ladies problems, joint pain, obesity, hair and skin. There will be separate cabins for

separate doctors. We will name them "Super Specialty Clinic Hospitals." And all of them will have Panchkarma. And they will all be VVIP, made very beautifully. Team hiring is going on for that. Approximately, I will open them in every state capital of India.
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Akshay: Okay. Okay. Understood sir. Thank you so much sir. And all the best.
Manish Grover: Thank you sir.
Moderator: Thank you. We have the next question from the line of Abhishek Sen Gupta from AB Capital. Please go ahead.
Abhishek Sen Gupta: Ram Ram Acharya Ji.
Manish Grover: Ram Ram Ji. How are you?
Abhishek Sen Gupta: Just fine. I was asking, how is the Chandan deal going?
Manish Grover: Currently, we have reached 3 lakhs per day in diagnostics. 3 lakhs per day is coming from Chandan. In this quarter, you will see it.
Abhishek Sen Gupta: Okay, okay. So how much is coming into the total revenue from that 50 -50 deal?
Manish Grover: Multiply 3 lakhs per day for 365 days. That's 10 crores right there. And it is increasing every month, every day. Because currently 34 centers have become operational. In 6 months, 64 centers will become operational. My target is to increase turnover by INR10 to INR15 crores just from diagnostics.
Abhishek Sen Gupta: Okay, okay. And in this quarter, the OTC business has done very well. Is there any specific reason?
Manish Grover: Sir, we put in effort, that's why it did well. We are working on the root cause of diseases of the people of India. Teaching people how to live, teaching people what to eat and
d drink. And my mission in life is that humans shouldn't fall sick.
Abhishek Sen Gupta: And sir, the medical college one -- any update on that?
Manish Grover: We have tied up with only three medical colleges so far. The first one has started. And for the first one, the other two we deliberately delayed because the last quarter was a weak quarter. If I had started, my expenses would have started. My profit would have reduced and expenses would have increased. Whereas that quarter was a weak quarter, so what was the need? In India, until February, hospital business remains down. From March to September, hospital business remains on a boom. So now I will make it operational this month.
Abhishek Sen Gupta: Okay. And this new Health Card business we will start. What will be the advantage for us? What is the scheme?
Manish Grover: Sir, the Health Card will basically be a "Proud Card," a card of self-respect, that you are connected with Swadeshi. In this Health Card, the patient will get a bonus whenever they buy any medicine, buy any product, take any services, get any diagnostic done, they will get points. And the value of each point is INR1. So the patient can redeem that much in their treatment. And they can use it for their family members too. Plus, as many patients as they refer, they will
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get the benefit of that too, of the same amount. So with this, our mouth publicity will increase with speed and it will be easier for us to fill beds.
Abhishek Sen Gupta: Okay. And in the PPT, it showed 35 franchise Centers. So is there any revenue coming from franchise centers?
Manish Grover: Sir, we are not focused on franchises. Franchises are standing as they were from the beginning. Franchises are only clinics. No hospital is a franchise, all are our own. We are slowly reducing franchises. If you see continuously over 3 years, we have reduced franchises and started our own centers. Franchises are only clinics. Clinics contribute very little sales. Our main contribution is from products, our hospitals, and day care centers.
Abhishek Sen Gupta: Okay. And would you like to give any guidance for next year? How much sales will we do?
Manish Grover: Sir, we will put in full effort. You see our consistent performance over the last 4 years, then you do the calculation yourself.
Abhishek Sen Gupta: It will continue like that according to your estimate?
Manish Grover: I will put in full effort sir. Now a full team is being built with me. In fact, I have found such good people now, we have brought a COO in the company. Currently 4 -5 corporate people have joined the company. We are working on corporate governance. Our team of professionals is increasing. So it is fun working, sir. Going forward we will do the same.
Abhishek Sen Gupta: And the new India...
Moderator: Sorry to interrupt Mr. Sen Gupta. Request you to kindly come back in queue for follow-up

questions. We have the next question from the line of Akhilesh Rawat from Vedanta Vision.

Please go ahead.

Abhishek Sen Gupta: Okay, yeah. Thank you.

Moderator : Thank you. We have the next question from the line of Akhilesh Rawat from Ridhanta Vision .

Please go ahead.

Akhilesh Rawat: Greetings, Acharya ji.

Manish Grover : Yes, greetings, how are you?

Akhilesh Rawat: I'm fine, are you well, Acharya ji?

Manish Grover : Just fine, I have one request for all of you if you would accept one appeal —just as you get your car serviced, or your scooter, or your bike, start getting yourself serviced too. We have centers for that; you can contact us whenever you wish. In Ayurveda, this is called Panchakarma. Yes sir, now ask your question, otherwise I forget to say this line.

Akhilesh Rawat: Yes, so sir, my first question is, what is our current average revenue per bed?

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Manish

Grover : 8337 for the last quarter. 8337. Okay. And the number... how many have been admitted in the quarter? 11313 in the quarter. In the previous quarter, it was 9600, and 8600 in the one before that. In total, over 9 months, 29,000 patients have been admitted, whereas 3 years ago, a total of 5700 were admitted. And those who took medicine totaled 1,56,000 in this quarter. In the last quarter, it was 1,40,000, and 1,24,000 in the quarter before that. In total, in three quarters over 9 months, 421,000 people have taken medicine.

Akhilesh Rawat: My second question is actually —you mentioned in the last quarter that we were going to make an exclusive agreement with a B2B distributor, with Entero, for Ayurveda, so what about that...

Manish Grover : We finalized the agreement with Entero last week.

Akhilesh Rawat: What impact are you seeing from that on our revenue? Like...

Manish Grover : Sir, I'll give you a figure: there are 9 lakh medical stores in India, 9 lakh chemists. And I want to enter the OTC market. Now, how do I reach those 9 lakh chemists? So I contracted with people like Entero; Entero has a 10% share of all medical stores in India, and we have become their exclusive Ayurveda partner.

This means if they need anything in Ayurveda now, they will demand it from us, and we will develop the product for them. Yes. Like BP pills, diabetes pills, besides that, medicines for depression and anxiety —we will provide all that to them. We have already started giving them the Stomach -Liver -Spleen Purification Kit.

And this month, we'll launch our first product for nutritional deficiency in India, called 'Nutri - Roz', right in this month of February, and that will be our second OTC product. After that, products will come in quick succession because once our network is created, as sales increase, I will bring the third, fourth, and fifth products.

Akhilesh Rawat: So currently we have only given them the Stomach -Liver -Spleen Purification Kit .

Manish Grover : Right now there is one in OTC; we have 300 products in total, which sell both online and offline.

Akhilesh Rawat: Yes. Yes, I was asking about Entero first.

Manish Grover: At the medical store, only the 'Stomach -Liver -Spleen Purification Kit' is available, and I also said you all can call or message the number and I will have it sent to you.

Akhilesh Rawat: Yes, yes. Yes. sir, I had one final question, a final question —like now regarding our -- as we saw that this quarter we achieved a very good EBITDA margin, you gave a guidance last time that we are targeting around 20 % to 25%, so are you still sticking with that guidance or would you like to improve it a bit?

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Manish Grover: That is our margin; that is what should be maintained, and if it comes in higher, it's God's grace. Yes sir, thank you sir, thanks. We will put full effort into maintaining it. And we've been showing it to you for three quarters now, sir.

Moderator: Thank you. We have the next question from the line of Shubhanu from Three Head Capital. Please go ahead.

Shubhanu : Sir, you said that in Q3 we didn't increase operational beds due to weak season. But from Q1 to Q2 also we only increased operational beds by 60. Like from 2179 beds to 2220 beds in Q2. And that's why our operational occupancy is also very low. And in the last call, you said you would focus a lot on increasing occupancy. How do we see occupancy at the end of this year?

Manish Grover : Sir, I am planning to reach 70 %-80% occupancy very easily so that it maintains at 70 %-80%. I will put full effort into that and I will increase beds at the same speed as I can gain occupancy. My target is 7000 to 10000 beds in the next three to five years.

Shubhanu: Okay.

Manish Grover: Because in India, there are 20 lakh beds in total. You understand one thing: there are 20 lakh beds in India, 9 lakh are

government, 11 lakh are private. Compared to that, our number doesn't even show up anywhere. We have only just begun our journey, so to speak. So now we are increasing occupancy, bettering our performance, and moving into products as well because all these things help each other. Our patient helps the product, and the product helps in increasing our patients.

Shubhanu: Okay sir.

Manish Grover: In the last call, for visibility, I also went to Masterchef last week if any of you saw it; in the Masterchef episode, I promoted Ayurveda, and soon I will also appear on Laughter Chef. So I am promoting in every way so that somehow our Ayurveda becomes global and India is known by the name of Ayurveda.

Shubhanu: Very good. Sir, my last question is , what you said in the last call about bringing 50 franchise hospitals, in how many years will those 50 franchise hospitals happen?

Manish Grover: Regarding the opening of new hospitals, we have just finalized the model. We have finalized a team for it which will go and open them. Hiring for that team has started; I didn't do it in the last quarter intentionally, otherwise my expenses would have increased.

Now, first I must finalize the model I want to bring in. Now we have finalized the model: super -specialty hospitals. So super -specialty clinics —now they will be within cities. Our initial plan was to do it through a franchise network, then we thought, why should we do franchises? Why divide the margin? Why shouldn't we start it ourselves? So first, we will do 5 to 10 ourselves, and after that, we will decide whether to do franchises or not.

Shubhanu: First 5 to 10 super -specialty hospitals will be our own.

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Manish Grover: First, we will bring them into a running model, make them successful, they'll start showing profit, a running model will be ready, then we will plan whether to franchise or do it ourselves because we have capital now. We already have the money, so why do we need money from franchises? And I have kept money aside because I am looking for some high -level acquisition or tie -up.

Through which we can easily reach double or triple X from here as well, so I'm in talks in several places and they are at a good stage. But these days I don't know what happens with you people -- people start asking for erratic multiples. So it takes some time to set them straight.

Shubhanu: Fine, sir. And sir, my only last request will be...

Moderator: Sorry to interrupt Mr. Shubhanu...

Shubhanu: It's not a question, it's a request, sir.

Manish Grover: Yes, go ahead.

Shubhanu: My request is just to maintain this kind of growth; a lot of investors' money is tied up with you, so please maintain this growth.

Manish Grover: Alright sir, thank you sir, thanks.

Moderator: Thank you. We have the next question from the line of Pravinkumar from Pravin Investments . Please go ahead.

Pravinkumar : Sir I have two questions. Can you tell me what percentage of your patients are recurring? That's the first question. And second question is the follow -up on your previous comment . You said you are in the ...

Manish Grover: Voice is very low, can you speak a bit louder? I couldn't hear your question, please repeat.

Pravinkumar : Yes sir, can you hear me properly now?

Manish Grover: Yes, yes.

Pravinkumar : Okay, so my first question is what is the percentage of your recurring patients?

Manish Grover: Do you mean repeat patients?

Pravinkumar : Yes, yes, yeah.

Manish Grover: Sir, we are not a hospital for repeats, sir; we teach the patient how to live —now you go home and do your homework yourself. We don't work on repeats, sir. We say, come to us, learn how to live, get trained, get educated, and then take care of yourself, and just come for Panchakarma after 6 months. So we have implemented Salesforce software, but currently, I don't have that data.

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If you want, send a mail to Nanak ji and I'll extract it for you; currently, I haven't extracted such data. And you can get the answer from a common sense question: the amount we spent on ads 3 years ago, I'm spending the same amount on ads today. 4 years ago we were working at a 6% profit; today we are working at a 30% net profit, so you yourself are smart enough to know that it is only through repeats and word -of-mouth publicity that we have reached here. But I haven't extracted that specific number; I will get it. Okay, understood.

Pravinkumar : Sir, my second question is on your previous comment. You said you had piled up so much of cash and planning for some acquisition or big engagement, right? Can you comment more on that please? Where are we? And what are those plans please?

Manish Grover: We will work on our similar business itself, sir. I don't want to go outside the line at all. I don't want to go beyond Ayurvedic products, services, international business, and this diagnostic business right now. I want to stay in this field itself and become the market leader of Ayurveda -- I want to make it the number 1 company in India, then Asia, and then the world.

Pravinkumar : All right sir, understood and I wish you all the best sir, to you and your...

Manish Grover: Thanks. And currently we are working heavily on research so that we can talk to the world with validation. So now more than 180 of our papers have been published.

Pravinkumar : All right sir, thank you.

Moderator: Thank you. We have the next question from the line of Deepak Pruthi from Wealth with Wisdom. Please go ahead.

Deepak Pruthi: Pranam Acharya Ji. How are you?

Manish Grover: Namaskar sir. How are you? Everything well?

Deepak Pruthi: Acharya ji, yes, very basic questions. First, regarding the products launched in OTC as you mentioned, like Pet Shuddhi, and you are going to launch many more.

Manish Grover: This month you are going to launch Nutrition Roz.

Deepak Pruthi: Nutrition Roz. Okay. This diet supplement...

Manish Grover: There is a nutritional deficiency in Indian food, because carbon has decreased in the soil, pesticides are being used heavily, and someone in India is deficient in Vitamin B12, another in D, another in Calcium, another in Iron. So I have created such a simple product with pure herbs, adding protein and all that, it's made with 33 herbs. Every Indian just needs to eat two spoons of it daily.

Deepak Pruthi: Very good sir, Indians really need this...

Manish Grover: You can't even believe how large the market size is, and the price is set so that everyone, rich or poor, can afford it.

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Deepak Pruthi: No, that's absolutely right, sir. Just looking at Vitamin D in Delhi -NCR, I think 80% of people must be deficient.

Manish Grover: Yes, so my product is a general product, it's for every Indian -- children, adults, and the elderly can all take it. Its name is Nutritoz. Meaning daily nutrition -- complete your nutrition every day, eat two spoons daily, and it will be an OTC product like our Pet Yakrit Pleeha Shuddhi Kit, available both online and offline. After this a new blood purifier will come, then BP pills, sugar pills -- we have completed nine clinical trials in total like this, which we will launch one by one.

Deepak Pruthi: Very good sir, very good. Sir, I was asking one thing -- our distribution model, is it a super stockist model where you appoint super stockists, then distributors under them, and then pharma retail outlets?

Manish Grover: Yes, we have already established super stockists, under them is a state distributor, then a city one, and then the goods are sold at the medical store. And about 40% of our margin goes into this whole network, based on the printed rate. And despite that, you see, this m

onth and this

quarter we increased product sales, yet we maintained the same profit margin as what was coming from my services.

Deepak Pruthi: Yes, yes, I saw that, yes.

Manish Grover: And that is our plan going forward too, and we will maintain it.

Deepak Pruthi: So Acharya ji, in how many states is this available now -- meaning how many super stockists do we have?

Manish Grover: Currently, the product is in about 11 states and is available at approximately 2000 to 2500 medical stores according to my estimates -- I don't know the exact number, but as soon as we join with Entero, that number will straightaway reach 1 lakh, goods will start being sent to

Entero in the next 15 days or so.

Deepak Pruthi: Very good sir, very good...

Manish Grover: So they have 1.25 lakh stores -- the reason it took so long with Entero was because our rate negotiation was going on, because they weren't agreeing to work on this margin. In medical allopathy, these people work on very high margins, don't they? So I said, I am Ayurveda, if I give you so much margin then what will be left for me?

Deepak Pruthi: Yes, and it would become expensive for the consumer too.

Manish Grover: That's why we didn't increase the rates, on the contrary, we passed on the entire GST cut to the customer. For example, if a product was priced at INR960 and the GST decreased, we made the price INR900. We transferred the GST cut for every one of our products to the consumer.

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Deepak Pruthi: Very good sir. Sir, tell me one more thing -- the notifications the government made in the budget, I couldn't hear very clearly but they have taken a very favorable stance regarding Ayurveda, they want to -- what is it, sir, and what will be the impact?

Manish Grover: Because of that, tourism from the whole world will come to us. The centers I've opened abroad will now act as feeders for me to fill my hospitals. Now I am planning VVIP hospitals and we are also working on better wellness. We have started getting 15% wellness patients, which was only 5% in the previous quarter. Meaning, earlier we were working on illness, now we are working on both illness and wellness.

Deepak Pruthi: You're moving toward Ayurveda?

Manish Grover: Exactly. And that's what Ayurveda says. "Swasthasya Swasthya Rakshanam, Aturasya Vikar Prashamanam Cha" the main objective of Ayurveda is to protect the health of the healthy and to solve the ailments of the diseased. So first, we won't let people fall ill. All the products I launch in OTC will be products that won't let a person fall ill -- they will be preventive products. For example, the Pet Yakrit Pleeha Shuddhi Kit is not for an ill person, it's to prevent falling ill. If you want to avoid illness, you have to take it first.

Deepak Pruthi: True. Acharya ji , about opening a resort -- is that the one in Meerut? I saw on your Instagram handle that you've involved many influencers, is that the same high -end resort you mentioned you would build...

Manish Grover: Instead of opening a new one, we are doing an upgrade of 28 rooms in our old one in Meerut, Muradnagar. Those are being made VVIP -ready. And we have brought in the head of a very large company -- which is almost number one or two in India for wellness so that we can open VVIP premium centers as well. Actually, we aren't finding the right person, so it's taking time. Otherwise, we will build our own.

Deepak Pruthi: Acharya ji, I have more questions, if you allow, I'll ask, otherwise others will and then I'll join later...

Manish Grover: You mail Nanak Ji, CFO mail ID is given. Put it on that, we will answer. I am in Mumbai tomorrow for Nuvama conference.

Deepak Pruthi: Okay fine, fine Acharya ji. Acharya ji, it feels great talking to you, your energy, I think your treatment is secondary, your energy heals people. All the best sir. Very good.

Moderator:

Thank you. We have the next question from the line of Omkar G from Shree Investment. Please go ahead.

Omkar G.: Sir, my question was that this time, more revenue has come from the products compared to the healthcare service, but looking at the last 2 -3 quarters, it was the opposite, so is there some...

Manish Grover: No it wasn't the opposite, you see my service sales have also increased. It's just that the previous quarter is a weak quarter for hospitals, so I focused more on products this month. I

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have to make the quarter better, don't I? Now, for this main Board company, we have to give results every 3 months. So I have to better my performance from quarter -to-quarter.

Omkar G.: No, I asked this question because in the product business, there is over 130% growth because your new OTC products were also added, so that also has a contribution, but in healthcare, there is 50% growth. I'm not saying this is low growth, but if compared, I asked this question because of that?

Manish Grover: Sir, actually, if you take my quarters from the last few years, like my quarter in '22 -'23, my total sales were INR60 crores for the entire quarter in that same period of '22 -'23. The following year, the sales for that same quarter -- my third and fourth quarters used to run the same in '22 -'23 and in '23 -'24.

But now in '24-'25, you'll notice that my fourth quarter increased by about 20% from the third quarter, while the second and third quarters were the same. So this time we made a strategy to change our planning beforehand for whichever quarter runs weak. Because we have to improve every quarter, right? And if we want to become the number 1 company in India, we have to think differently from the rest of the world. We have to bring new products too. Now I am bringing products in Ayurveda that people will use at home, use daily, and money will be made for the company every day. All the daily-use items in India -- I am putting Ayurveda into everything now. So my R&D is ongoing, it will be complete within 2-3 months, so we will launch those products too. They will all be Ayurvedic.

Omkar G.: Yes, thank you sir. I had one or two more questions, so I'll ask them. You mentioned these 7,000 to 10,000 beds -- are you just talking about setting up the beds or will these be operational beds? That's one thing?

And secondly, you mentioned you are trying to become India's, World's big company in Ayurveda. So with this, if you can issue any long-term guidance -- not immediately, not next quarter or next year, a 5-year guidance if you can mention, that would be good?

Manish Grover: Look sir, right now this year our profit will be approximately INR225 crores plus. You would have guessed. After this, my next target is straight INR1,000 crores. Now for that, if I have to work hard for 3-4 years, I am ready.

Omkar G.: Yes sir.

Manish Grover: Because if an Ayurveda company earns this much in India, then the whole world's attention will definitely come towards Ayurveda. And I want that. That the whole world sees Ayurveda through India's eyes and India becomes number one in the world because of Ayurveda. When we talk about Vishwa Guru, I think it can only happen through Ayurveda. And currently we are the biggest clinic and hospital chain in India.

Omkar G.: Yes sir. And regarding the beds, if you could tell, the question asked earlier?

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Manish Grover: Sir, actually this growth will run equivalently. We have to work simultaneously on beds plus products according to beds. We can't separate them from each other. Because the patient uses the product later. And products bring the patient. So I have clearly said that my target is 7,000 to 10,000 beds. After this -- and along with that, if I give a very long vision right now, then you people

also start calculating accordingly, right? So that's why I am saying that right now I am planning to make the profit 4x to 5x from here.

Omkar G.: Thank you sir. But if you can tell -- good sir. No, if you can tell, what will be the occupancy rate for these 7,000 to 10,000 beds if in the next 3 to 5 years?

Manish Grover: Sir, first I am increasing regular beds. Regular occupancy is going on. Just now, in the last quarter, I did not make 500 beds operational deliberately, 1 BECAUSE expenses start as soon as they become operational.

Omkar G.: Sir, I am not talking about just this quarter. I am talking about the 7,000-10,000 you are saying.

Manish Grover: Sir, I had told you that from March to September, beds fill up in hospital business. Now I will make them operational this month.

Omkar G.: Yes, yes, yes sir. I am asking about the same thing. This 7,000-10,000 bed target of yours in next 3 to 5 years, in that, what level of occupancy do you see then? I am asking this?

Manish Grover: Sir, my plan for occupancy, now and later, is 70% to 80%. So we are launching some cheaper packages too. For knee pain, back pain, heart. We will launch their packages, then our bed occupancy will increase. And now health insurance has done very well. That if a patient gets admitted for one or two days also, afterwards they can take day care treatment. All claims are passed.

So earlier day care was not allowed. Now day care is also allowed. So our policies have become very clear. If a patient is admitted even for a day, 2 days also admitted. Then afterwards they can get their treatment in day care. Their bill also gets reimbursed. Now one more big thing happened. Day care treatment has also started getting reimbursed. Earlier it wasn't. Now reimbursements have started.

That is why our growth in the last quarter was very good because Health Insurance is a very good sector for us. We reduced Government and increased focus on Health Insurance. That is why our patient occupancy did not decrease in the last quarter. Whereas routinely it decreases in every hospital. But in Ayurveda, health insurance is new, so people are taking time to understand. But our promotion is going very well. So now in this quarter also we will do better than before.

Omkar G.: And your target for OTC is INR500 crores by next year, right sir?

Manish Grover: No sir, I said 2 years. You people reduce 1-year.

Omkar G.: No, no. I mean, by the time this year ends, it will be INR300 crores right? With one product ?
Manish Grover: Okay. Now the second product is to be launched this month.
Omkar G.: Okay.
Manish Grover: Sir, now con call happens every 3 months. Earlier I had to speak after 6 -6 months.
Omkar G.: Yes, yes, yes. So meaning , in two financial years, you will have INR500 crores OTC revenue. Correct?
Manish Grover: Easily. Easily.
Omkar G.: Okay. Thank you , sir. If I want to know anything else, I will mail. Thank you.
Manish Grover: Thank you , sir.
Moderator: Thank you. We have the next question from the line of Amit Jeswani from Stallion Asset . Please go ahead.
Amit Jeswani: Ram , Ram Ji. Very happy with the numbers.
Manish Grover: Ram , Ram Ji. How are you?
Amit Jeswani: Just very good.
Manish Grover: Send Gopi kaka to get admitted. It's been 2 years.
Amit Jeswani: Done, done sir. 100%. 100%.
Manish Grover: He came to me 2 years ago for detox. After that, he disappeared.
Amit Jeswani: No, his health became fine, right? So didn't need it.
Manish Grover: If health is fine, then car service has to be done every 6 months, right?
Amit Jeswani: Right. I myself come. I come myself. My service is needed more.
Manish Grover: Then you come, what's the problem. I am coming to Mumbai tomorrow for Nuvama's conference.
Amit Jeswani: I'll see you sir. I'll see you there.
Manish Grover: Thank

you , sir. The way you guide us so well , I have to say thank you to you too. If we have reached here, your guidance has a very big role.

Amit Jeswani: Thank you , sir. Thank you so much , sir. Sir, Manish ji, wanted to understand this INR1,000 crore s goal you set today. How will you get there, Manish ji? Because from here, 200 -250 -- between INR225 crores to INR250 crores this year's profit will be. That is entirely free cash
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flows. How will this journey of INR1,000 crores happen according to you? Just thinking , how big is the opportunity and how will you reach there?

Manish Grover: See sir, I have identified 15 products. 15 products identified which are the need of every human. Now, with one product, I am taking INR10 crores monthly sales. And those 15 products are such that , now my idea is such, I didn't want to reveal it on this con call, I am going to launch a product which every single person in India will have to eat for 3 months in a year. How will I market it? Because that thing is so needed in India.

So like th is, 12 to 15 products are there and there are 7,000 to 10,000 beds. If I give 7,000 beds, my 50% occupancy is done. And I get a sale of about INR1,000 crores from 10 products , so my INR500 crores profit is being made like this, sir, 12 x.

Amit Jeswani: Right, right, right.

Manish Grover: So 500 has to be increased further, 750 has to be done, 1 ,000 has to be done.

Amit Jeswani: Right. So basically...

Manish Grover: So I have identified 24 products in total in India. Like there are 180 crores eyes in India.

Amit Jeswani: Right.

Manish Grover: So I am bringing products for their production. There are about 10 crores BP patients in India. And there are 13 crores pre-diabetic, pre-hypertensive. There are about 11 crores sugar patients in India. And there are about 14 crores pre -diabetic.

So what is put in the mind of pre -hypertensive, pre -diabetic? That you are going to have diabetes in the future. You are going to have blood pressure in the future. I have done the first product clinical trial for them.

Amit Jeswani: Right, right. So basically...

Manish Grover: I have to send it to the market to people. So the profit will come automatically, sir.

Amit Jeswani: Got it. You are the master of execution.

Manish Grover: When you were connected to me 2 years ago , whatever you told me to do, don't do this, don't do that. Tell me honestly, am I doing any mistake?

Amit Jeswani: No, no. One thing you did the best is that you kept a Big 5 auditor. We are very happy about

that.

Manish Grover: GT coming benefited us a lot. They pull us up a lot.

Amit Jeswani: Because what happens sir, you are number one in execution anyway. Now you have become number one in corporate governance too. Your Big 5 internal auditor. ..

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Manish Grover: I'll give you one more news. We have appointed a firm Forvis Mazars. It is at number 7 in the world. We appointed them as internal auditor.

Amit Jeswani: Wow. Wow. Wow.

Manish Grover: So Forvis Mazars internal auditor and Grant Thornton our main auditor. Apart from this, we brought Salesforce software earlier. Now work is going on Oracle. So international level software, international level agencies. Because we have to take Ayurveda to the international level. That's why I started expanding international business too.

Amit Jeswani: Right.

Manish Grover: And let me tell you, all health insurances run in UAE.

Amit Jeswani: I call you Rocket Man and I don't say it wrongly. I say it correctly. You are absolutely Rocket Man. You just keep at it Manish Ji.

Manish Grover: Just keep at it. Because you people guided so well.

Amit Jeswani: Just keep guiding us like this on what mistake not to make. That we won't do. No, the best thing you did Manis

h Ji, look, you have not gone on opening new stores. You did the best thing, deal with Entero. Our job is not to open stores, not to retail. Our job is to sell medicine and keep researching. Which new medicine to sell?

Manish Grover: Earlier a very big company made a very big mistake by opening its own stores. I won't name names. I won't make that mistake. The stores that are open, I will give my goods to them, right? Why should I increase my capex by opening my own store?

Amit Jeswani: Right. Right. Right. That's a very smart way of thinking. In that, our ROCE becomes unlimited. Neither goods are ours , manufacturing is also not ours, right?

Manish Grover: Manufacturing is our third -party. But this year I will make manufacturing my own.

Amit Jeswani: Okay.

Manish Grover: So with that our margin will also increase. Because manufacturing must be taking some margin, right? And quality will come more under control. Because now I need a huge quantity of OTC products, so I have to set up a big factory. So I am looking for a tie -up with a ready - made factory or I will set up my own. Talks are on at a couple of places.

Amit Jeswani: Keep at it Manish ji. Super -duper. Had fun. Seeing the numbers, heart is happy. Thank you so much.

Manish Grover: Thank you sir, thank you. Do send Papa -Mummy without fail now.

Amit Jeswani: Done, done sir. Done sir. Thank you sir.

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Manish Grover: Thank you.

Moderator: Thank you. We have the next question from the line of Ankit from Fusion Capital. Please go ahead.

Ankit: Ji Namaste. Voice coming?

Manish Grover: Yes, yes coming.

Ankit: I wanted to ask about Chandan. How is the partnership with Chandan going? Are all centers covered?

Manish Grover: Sir, I told you, right? We have reached 3 lakhs per day. 7 days ago we were at 2 lakhs per day. Day before yesterday we were at 3 lakhs per day. Soon we have made a target of 5 lakhs per day. Per day testing is happening this much from my 34 centers here. And these will become 64 soon? And they will become on all our centers. And all investment is being done by Chandan.

Now 6 new Fibro scan machines are coming. Which involve an investment of at least INR3.5 crores s. Chandan has done that too. With that, my footfall will increase and patients will increase. Now full -fledged centers of Chandan have opened in my Dera Bassi, in my Meerut, in Chandigarh. And test collection is going on from 34 centers.

So I have sales from the last 3 days -- it was 3 lakh per day plus. 7 days ago it was 2 lakh. Now my next target is 5 lakh per day. And Chandan gives us a benefit giving free first tests to our patients here , which is increasing my footfall.

Ankit: So, how much profit have we and Chandan made so far in this quarter?

Manish Grover: Sir we have just started before two months, so the number will come next quarter. First, we used to just talk, we just gave them the money now, just before 5 years. But they had invested in advance.

Ankit: Do we have to do any capex on them?

Manish Grover: We don't have to do any capex on them. Not a single rupee. Capex is theirs. All money they are investing. Their headache, their job. All money, manpower is theirs. We just have to give them whatever collection happens a percentage of that they give to us. And in healthcare also they give that discount claim, clearing. Whatever patient is ours, cashback bonus comes, right? They clear that too.

That Chandan deal is a very good deal sir. With that our footfall became very good. Despite the third quarter being weak, our footfall increased because of that. Because first testing is absolutely free. And after the second, whatever commission goes to the patient, it comes in the patient's card itself.

Ankit: Ji, ji. Great sir.

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Moderator: We have the

next question from the line of Harish Kumar from Nirmal Bang.

Harish Kumar: Sir, I wanted to know the treatment happening through our video conferencing, has the facility started out of India too? And have patients started getting treated from there too? Or is it only in India?

Manish Grover: Yes, yes sir. Started in both India and outside.

Harish Kumar: Very good, very good sir. Very good.

Manish Grover: Government of India is promoting it itself. Government of India has launched an AYUSH portal where the doctor treats via video conference and it is allowed, valid in India. And our VOPD number is continuously increasing. If you want I can give you the number. I think I spoke the number just now. VOPD number was in quarter 1 total VOPD done was 41,000. In quarter 2 it was 56,000. In quarter 3 it was 61,000.

And the E-com product sold in quarter 1 was 38,000 pieces from e-commerce. And in quarter 2 sold 74,000. In quarter 3 sold 1,99,000 pieces. And 131% growth happened quarter-on-quarter in E-com. And in IPD, 8,600 in quarter 1. 9,600 in quarter 2. And 11,300 in quarter 3. And in OPD, 1,24,000 in quarter 1. 1,40,000 in quarter 2. 1,56,000 people came in quarter 3. We are growing in every segment.

Harish Kumar: That is true. And hearing this from your mouth felt very good and very satisfying today. That if India can become Vishwa Guru, it can only be through Ayurveda. And I am sure...

Manish Grover: Wait, I ask you a personal question. You tell me, other than Ayurveda, what does India have that the world doesn't? Our education itself is ruined. We are studying the British Macaulay education system. Our justice system is British. We have only one thing left -- Ayurveda. If we save this, India can become number one in the world.

Harish Kumar: Sir, if you remember, I met you and specially came to meet you in Chandigarh if you remember. And I told you, make yourself Vishwa Guru.

Manish Grover: Okay. Thank you, sir. Thank you very much. Thank you.

Harish Kumar: And you have said it, so I have trust that you will do it too.

Moderator: We have the next question from the line of Pratik Tandel from Qode Advisors LLP.

Pratik Tandel: In this PPT, we gave the breakup of Government, Private and Medicine in the revenue mix. Can you give that for FY25, '24, '23 annual numbers too? Revenue break.

Manish Grover: Meaning -- CFO sir will tell. Tell Nanak Ji what he is saying.

Nanak Chand: Yes sir, we are maintaining the same proportion of the product and service as well as government scenarios. Like we are maintaining the 3% --

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Pratik Tandel: 41% and 54%.

Nanak Chand: Yeah, yeah. So we are sustaining on the same margin.

Pratik Tandel: Okay so...

Manish Grover: Government business doesn't have margin and money doesn't come in government business, so we are reducing the government business. You look, in 2024-'25 our total turnover from government business was INR118 crores. I have reduced it to INR8 crores in the quarter.

Pratik Tandel: Ji, that's what I am trying to understand. Can you give the breakup of March FY25 numbers revenue? Government, Private and Medicine?

Manish Grover: You write it down, note it. March '25 INR215 crores was medicine sale. INR136 crores was

my hospital services. And INR118 crores was my government services. So total INR469 crores s. If I do it proportionately, healthcare was 54%, 46% was my medicine sale.

Pratik Tandel: Correct. Okay. One last question. You said that till Feb, for healthcare sector, the period is bad. Can you explain the logic why cyclical remains in healthcare business in this period?

Manish Grover: Sir, actually in India, there is a festival called Deepawali. After Deepawali, weddings start. If you know, Saya starts. So Deepawali, Saya, and then cold of November -December. And 25 December, New Year celebrat

ion. If you check, the quarter of all hospitals remains weak, this

one. But we knew about this trend from last year's quarter. So we changed our strategy this year and increased focus on products. So that our number remains maintained in this quarter and we become plus too.

Now we have developed teams -- I have divided teams. And put both teams on competition with each other. Given product to one team, healthcare to one team. And international business to one team. Now all teams are fighting among themselves. And we did ESOPs again in the company. So that everyone becomes profit partner. So that they don't feel they are working for the company, they feel they are working for themselves.

You must have seen our new ESOP policy also came. Scheme came. Earlier scheme was a bit weak scheme. Because in that ESOPs were to be reimbursed in 5 years. Now we brought a new scheme, 3 -year one. First year 20% will reimburse. Second year 30%, third year 50%.

Due to new ESOP scheme, good manpower has come to our company. And when new manpower comes, they bring new brains too. New ideas come. So those also get executed.

Moderator: We have the next question from the line of Amit Agarwal, an Individual Investor.

Amit Agarwal: Sir, first of all congratulations for the results. And sir, one question was that like we are curing diseases. And Allopathy also cures. So both are running together, business of both is increasing. So in coming 5 years what competition do we see and where do we see ourselves?

Manish Grover: Sir, to be honest, I am not in the mood for competition. My mood is that no person in India should fall sick. So for the last few months, I have focused on training and education. I go to Jeena Sikho Lifecare Limited

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schools, give lectures, go to universities. To teach humans how to eat and drink. I have written a letter, mailed the Government of India. And sent a proposal that you appoint an Ayurveda Doctor in schools. So that children learn from childhood itself so they don't fall sick.

Main purpose of Ayurveda was never treatment of disease. Main purpose of Ayurveda was "Swasthasya Swasthya Rakshanam" protect the health of the healthy. So I want that just like you service your scooter, bike, car every human should get their service done every 6 months. So in a way, we should make our name "Service Center". So they won't fall sick. And as far as sir I want that humans shouldn't face the need for operation.

But for that they will have to focus on prevention. Otherwise Allopathy is there, doing its work. And I have kept Allopathy doctors in my hospitals. In emergency, they help. So I am in favor of the integrated model. Ayurveda, Homeopathy, Naturopathy, Allopathy all work together. So that disease is cured in India.

But what is happening is , as hospitals are increasing, diseases are increasing. Because adulterated food, eating habits , all these are spoiling us. So you must have noticed last week I was on invitee in Master Chef. If you saw that episode , there I emphasized on education. Picked up bitter gourd karela, taught how to eat. Taught that stomach is not getting cleaned. People are not free f rom gas and acidity. What will they contribute to GDP? Someone is getting dialysis done, someone eating BP pill, someone getting operation done. So India if it grows, it will grow with healthy people. Because a sick person worries about his illness. So my focus is purely on wellness. Then illness won't even occur.

Amit Agarwal: Sir, one thing I want to say. Four months ago , in August , I came myself, me and my father, to Manesar branch . So we got Panchkarma done sir. And reimbursement also happened for our claim.

Manish Grover: Was it your Health Insurance?

Amit Agarwal: Yes, claim happened easily in Health Insurance.

Manish Grover: So this is very good news, right? Look, if people's claims are happening in Ayurveda, why

won't people come towards Ayurveda? Why were people going to Allopathy earlier? Because it didn't cost them money. I told you, right? Reason for our sales increase in last quarter.

Patient was getting admitted for one or two days, rest getting treatment in Day Care. So our business of Health Insurance got a jump. And right now we haven't even started , health insurance is less than 25% for us. Think how much percent it can grow now. So now our journey has started.

Amit Agarwal: Yes, sir. Thank you.

Moderator: Thank you. We have the next question from the line of Mohan Agarwal, an investor. Please go ahead.

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Mohan Agarwal: Hi sir, Radhe Radhe.

Manish Grover: Radhe Radhe. How are you?

Mohan Agarwal: Sir fine. I am speaking from Lucknow. First time connected in a conference call.

Manish Grover: I was born there, studied there, UP's product.

Mohan Agarwal: Sir, I have been following you for a long time. My brother's kidney failed, so the need for dialysis came. So I came to your hospital, following you since then. And I found out this year that your company is listed. So the journey started from a follower and has come to an investor. So it has been a very good journey. And sir, biggest reason to join the conference call is that you speak in Hindi. Sir, understood, felt good talking. Sir, will ask questions quickly.

Manish Grover: This country's misfortune is what? Those who know Hindi, still talk in English. They think we will look cool, we will look wow, we will look educated. But our mother tongue is Hindi, right?

Mohan Agarwal: Sir, in coming time you will fall short of time in conference call, I give this to you in writing. This many investors are going to come to talk to you. Sir, question is, you said 3 lakhs sale is coming from Chandan. Is it sale or is it commission that Chandan was giving you?

Manish Grover: No, 3 lakhs sale.

Mohan Agarwal: Okay, so in this our share...

Manish Grover: We don't take commission. I have never done commission business in my life. Whatever sale is coming, we are reimbursing the patient. Discounting that much money in patient's treatment.

Mohan Agarwal: That 30% commission that is coming?

Manish Grover: Whatever commission is coming, we reimburse the patient.

Mohan Agarwal: Okay, okay. And the card, the card being made...

Manish Grover: It goes into the card. They get discount in their treatment. But our business is growing from that, right? Discount to patient is separate in medicines.

Mohan Agarwal: Okay, okay. So the card, is it the same Chandan card or our Jeena Sikho card?

Manish Grover: No, we have made a new card which has logos of both Jeena Sikho and Chandan on it.

Mohan Agarwal: Okay.

Manish Grover: I won't be able to show it here. If you message on the number, will put in some group.

Mohan Agarwal: Yes, okay, okay. Sir, one more suggestion was there. As you said I have been your follower for a long time. The shorts you put on YouTube, earlier I used to see, in shorts the thumbnail

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used to have written, like "you can take this medicine if you want relief from this thing." In that -- I might be wrong, just giving a suggestion, views come more in that.

Manish Grover: Do one thing. This is investor presentation call. You call on the number I spoke, 98554 87770. This is my PA, Ravi's number. You call on that, I will talk to you. So whatever suggestion you have, we will definitely apply.

Mohan Agarwal: Thank you, sir. Okay sir one more was there. You said you send product. Shall I send my address on this?

Manish Grover: Yes, send address. Will send you a kit. If you send address on the number, product will come to you. I said in the last concall too, whoever wants, our investor or whoever is in concall, they all put me

message for product. We will send them free product so they can use. What is Ayurveda? People don't even know, many people don't know what Ayurveda is.

Moderator: Thank you very much. Mohan, I'll request you to come back for a follow-up question. Next question is from the line of Sampee Barloti from TechnoFunda. Please go ahead.

Sampee Barloti: Yes, Namaskar. Good afternoon, sir.

Manish Grover: Yes, Namaskar.

Sampee Barloti: Sir, a question. A partnership was going to happen with a distributor, right? Pharmacy distributor. What is the status of that?

Manish Grover: Sir, it happened. Partnership happened. Distribution happened. Last week only it happened. So this week it will be uploaded on NSE. It was a percentage commission problem. It got sorted out, so it happened, our agreement.

Sampee Barloti: Good, congratulations. This is the Entero partnership, right?

Manish Grover: Thank you. I'll tell the name. Entero. 1.25 Lakh distributor network is theirs. And in India total

9 Lakh medical stores are there. Meaning our goods will reach 10%.

Sampee Barlota: Okay. Congratulations sir. Second question was, on the OTC , just wanted to understand one thing. Your OTC products , are they -- like I know they go through the ICMR route . Are they patented? Is it...

Manish Grover: Yes, yes. Whatever product we launch in OTC , whatever products we are doing , first we complete clinical trials on all of them. So the product which is our "Pet -Yakrit -Pliha Shuddhi Kit", its clinical trial is already completed. That's when we launched it.

Sampee Barlota: The competitor cannot replicate that product, right?

Manish Grover: No sir, a competitor -- a competitor can make whatever they want; after all, it's been going on in India for so many years -- Kayam Churan was selling, Pet Saffa was selling, Jhandu Nityam

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is selling, Dabur's is selling -- so many products are selling, but they were only clearing the stomach. They weren't working on the cause of the disease in Ayurveda.

Samip Barlota: Okay.

Manish Grover: We worked on the cause of the disease. And do you know the biggest thing, sir? Anyone can launch a product, anyone can open an Ayurveda hospital, but winning the customer's trust. We have been winning the customer's trust continuously for the last five years , and we aren't making any mistake that would shake that trust in any way.

So we are focusing fully on the issues of corporate governance, on the quality of our doctors, and on our team, and our biggest strength is training. Our training is very special -- we train people repeatedly -- and if you look at it now, we had one and a half lakh patients in the entire last year.

In the entire last year, one and a half lakh patients received treatment, so what's the big deal in bringing in 10 -15 lakh patients when there are 20 lakh beds in India? And similarly, in the entire last month/quarter, we treated 15,000 , one and a half lakh patients, whereas 40 crore s people are ill in India.

So by treating one and a half lakh people, we aren't even at 0.00%. So sir, this is a very long journey that has only just begun, but we will not compromise on quality and we will not compromise on our principles.

Samip Barlota: Got it sir.

Manish Grover: And our strategy is blue ocean -- you yourself tell me, do we really have any competition with anyone?

Samip Barlota: No, no sir.

Manish Grover: And now we are bringing in a chain of super -specialty hospitals and clinics. We have done international expansion, we entered diagnostics, we've come into pharma, we are bringing new products in OTC, and we have the hospitals. So we don't have to give anything to anyone; our entire internal value chain is set up. We - we are complementing each other ourselves. Our integrated model is our own. Meaning, the medicines are our own, the doctors a

re our

own, we will do the treatment ourselves, and we won't let people fall ill either. So we are curing people by educating them as well. Because whoever is getting educated becomes our follower.

For example, day before yesterday , I gave a lecture at a Punjab University; so for all the students there, if anyone in their families falls ill in the future, they will say 'go to Jeena Sikho's hospital', even though I didn't take any money there. I taught them for free what to eat, what not to eat, what to eat more of, what to eat less of.

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So our followers are increasing on their own, and now we have started a new 'game' which will be implemented in the next two to three months. We have started making videos in other languages. Currently we only had North -based customers; until now we hadn't even reached Bengali, Gujarati, Marathi, and all those patients.

Now my OTC products are being translated into those languages and my videos are being made with AI, which will also reach there. I have recently started Russian -- my translation into Vietnamese has also started, and I've started in Russian. So everyone will see the effect of this over the next six months to a year , from what I have started now.

Samip Barlota: Sir thank you for the detailed explanation. Sir, one last thing: you had some discussions with the government about covering Ayurveda in the Ayushman Yojana -- is there an update on that?

Manish Grover: Sir, that is ongoing, and the Indian government keeps saying they are bringing it, bringing it, bringing it, that they will bring it in this budget; that's what they promised, but sir, it's the government, what can I say to them? But it doesn't make any difference to us, sir -- whether it comes or not, we are moving at our own speed.

Samip Barlotia: Got it sir, all the best.

Manish Grover: If it comes, it'll be very good because it will benefit the world. But until they bring it, we are putting in our own effort. And the biggest plus point now is that health insurance company policies have become very clear; now if a patient is admitted for even one day or two days, they can continue treatment in day-care after that, and those bills are also being reimbursed.

Now another big thing has happened: reimbursement for medicines has also started. Earlier medicines were not covered, but now medicines have also started being reimbursed.

Samip Barlotia: Good, good sir, all the best for the great Q3 results, congratulations.

Manish Grover: Thank you sir, thanks.

Moderator: Thank you very much. Ladies and gentlemen, that will be the last question for today. I now hand the conference over to the management for closing comments.

Manish Grover: Sir, was that the last question or is there one more coming?

Moderator: Sir, that was the last question. You can go ahead for the closing comments.

Manish Grover: Yes sir, thank you. So that part is done, I have to conclude now, right?

Moderator: Yes sir.

Manish Grover: So thank you very much to everyone, and I make one request that you all take care of your health. Wake up in the morning, sit in a squatting position, and definitely chew one piece of

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turmeric, a small piece of ginger, and 10 curry leaves because in India, all diseases occur due to stomach problems.

Ayurveda says 'Sarve roga api mandagnauhi, ajeern maline kupit mal sanchoe' -- that an uncleared stomach and improperly digested food are the main roots of all diseases. So please, everyone wake up in the morning, sit in a squatting position, drink hot water, drink herbal tea, or chew this ginger-turmeric-curry leaf mix, and every person should definitely undergo Panchakarma every six months.

Definitely do Panchakarma because you are all people from the investor world; you will earn plenty of money, but

if you ignore health, problems will arise in the future. So my advice to all of you is to keep health alongside you as you progress in life -- give priority to health because health is actually the real wealth.

And I thank Ranvir ji, Nanak ji, the entire team, Nuvama, everyone -- all the investors and all today's participants who listened to me carefully, and I hope you will definitely implement my last piece of advice. Thanks.

Moderator: Thank you very much. On behalf of Nuvama Wealth, that concludes this conference. Thank you for joining us and you may now disconnect your lines. Thank you.

Call: Jun 2026

Date: 08th June , 2026

To,
Manager - Listing Compliance
National Stock Exchange of India Limited
'Exchange Plaza'. C-1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai - 400 051
SYMBOL: JSLL To,
Head of the Department,
Department of Listing Operation,
BSE Limited
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai 400001
SCRIP Code: 544476

Sub: Intimation to Stock Exchange – Submission of Transcript of Earnings Conference Call for the quarter and financial year ended 31st March, 2026.

Dear Sir/Madam,

Pursuant to Regulation 30 read with Schedule III and Regulation 46 of the Securities and Exchange Board of India (Listing Obligations and Disclosure Requirements) Regulations, 2015, as amended, we hereby inform you that transcript of Earnings Conference Call held on Tuesday, June 02nd , 2026 to discuss the Audited Financial Results for the quarter (Q4) and Financial Year ended March 31, 2026 has been made available on the Company's website .

The aforementioned transcript to the conference call is available on our website at <https://jeenasikho.com/wp-content/uploads/2026/06/Jeena-Sikho-Q4FY26-FY26-Earning-Call-Transcript.pdf>

Kindly take the above intimation on your record.

Thanking you,
Yours faithfully,
For Jeena Sikho Lifecare Limited

Manish Grover
Managing Director
DIN: 07557886

Place: Zirakpur, Punjab
Date: 08.06.2026

Encl: A/A

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"Jeena Sikho Lifecare Limited
Q4 & FY26 Earnings Conference Call "
June 02, 202 6

MANAGEMENT : MR. MANISH GROVER – MANAGING DIRECTOR –
JEENA SIKHO LIFECARE LIMITED
MR. NANAK CHAND – CHIEF FINANCIAL OFFICER –
JEENA SIKHO LIFECARE LIMITED

MODERATOR : MS. SOUMYA CHHAJED – GO INDIA ADVISORS

Jeena Sikho Lifecare Limited
June 02, 2026

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Moderator: Ladies and gentlemen, good day and welcome to the Jeena Sikho Lifecare Limited Q4 and FY26 Earnings Conference Call, hosted by Go India Advisors. As a reminder, all participant lines will be in the listen -only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Ms. Soumya Chhajed from Go India Advisors. Thank you, and over to you, ma'am.

Soumya Chhajed: Good day, everyone, and welcome to the Q4 and FY26 conference call of Jeena Sikho Lifecare Limited. We have on call with us Mr. Manish Grover ji, the Managing Director, and Mr. Nanak Chand, the Chief Financial Officer. We must remind you that discussions on today's call may include certain forward -looking statements and must therefore be viewed in conjunction with the risks pertaining to the business.

I now request the management to take us through the business update, and post that, we will open the floor for Q&A. Thank you, and over to you, sir.

Manish Grover: Namaste and Jai Hind. Today, I will not just discuss the financial results; I want to tell you about the business model working behind our organization and our numbers, the health problems we are solving, and why we believe this growth will continue to increase and remain sustainable in the coming years.

I am not just reading numbers; I want to tell you what we do. Because when you fully understand the business, you are not surprised by the financial performance; rather, it feels like it was bound to happen.

India is facing a major crisis of chronic diseases. Crores of Indians are living with Type 2 diabetes, high blood pressure, kidney disease, thyroid, joint pain, psoriasis, obesity, and countless other diseases. Currently, the whole world works after people fall ill -- people get sick first, and then they are treated. Jeena Sikho is the first company in India created to prevent people from falling ill in the first place. We want to ensure people do not get sick. We have started a new mission in India ; we do not want to earn money from people's illnesses. By keeping people healthy, we will improve both our duty and our actions.

To solve this problem from a different perspective, I chose the medium of India's thousands -of-years -old science, which is Ayurveda. Not as a last resort or alternative medicine, but as the first choice -- an evidence -based, clinically delivered system that works on the root cause, not just the symptoms. Our primary treatment, Panchakarma Ayurveda, is a system of classical detoxification and rejuvenation.

Admitted patients in our hospitals receive Panchakarma therapies daily for a fixed period. We work with approximately 600 certified Ayurvedic and Naturopathy doctors and about 750 wellness experts. We have written about 550 papers last year, of which about 400 have been

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accepted, and more than 180 are already published and documented in peer -reviewed research papers. This is not just a story; it is a reality we have started on the path to changing India. We have two verticals , Ayurveda Healthcare Services and Ayurveda Healthcare Products. Both complement and strengthen each other. The service business model operates on a hub -and-spoke basis, including 58 hospitals and 59 daycare and clinical centers. Our centers are across 23 states and 100 cities. Large hospitals in big cities serve as hubs. Near Delhi and Meerut, we have about 600 beds.

In Mumbai and Panvel, we have four hospitals, including 200+ beds in Panvel and 150+ beds in Navi Mumbai. Patients from clinics in nearby small towns are referred to

these large hospitals.

Out of our 117 facilities, only 35 are clinic franchises. All other doctors are on our payroll, and the control remains with our organization. We supply medicines exclusively, meaning clinical quality and product revenue both remain in our hands without bearing the full capital burden. The capital efficiency of this model is remarkable. The setup cost per bed is only INR3 lakh to INR4 lakh. The break-even, even at 35% occupancy, ensures profitability for both small and large setups. The payback period for small facilities is less than 6 months, and the average ROCE for 3 years is 71%. Under the product business, we have more than 330 SKUs, gross margins exceeding 85%, and we follow AYUSH standards and Government of India regulations. We use third-party manufacturing handled by our healthcare centers, dedicated client support staff, tele-calling, and video consulting. Products are sold through e-commerce and now OTC pharmacy retail. In FY26, this vertical contributed INR416 crores, which is 52% of the total revenue.

The flywheel for both is very simple. Every patient who recovers and leaves our hospital takes a medicine regimen that provides recurring product revenue for months or years. Every medicine customer who calls our support center is a potential IPD admission.

The more healthy people come to us, the more they become mouthpieces for future patients. Satisfied patients and the common man are our marketing channel, word of mouth, completely free. For the full year FY26, IPD patient volume increased by 65% to 40,450. OPD patient volume increased by 69% from FY25 to reach over 6,00,000.

In Q4 FY26 alone, OPD, COD, and video consultation patients increased by 247% year-on-year. Daycare volume increased by 139%, video consultation by 214%, and IPD volume by 84%. These are not marketing numbers; they are the result of better utilization of these 58 wellness centers and hospitals. We have 59 clinics and daycare centers, client support centers, and healthcare camps with a 30% IPD conversion rate, generating direct business of INR30 lakh to INR60 lakh per camp.

On the financial side, revenue from operations in FY26 was INR801 crores, a 71% year-on-year growth. EBITDA margin in FY26 reached 44%, up from 30% in FY25, an improvement of 1,360 basis points in a single year. FY26 EBITDA was INR349 crores, up 149% year-on-year. FY26 PAT was INR222 crores, up 177% year-on-year, with a 28% PAT margin. There is absolutely no debt on the balance sheet.

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Currently, we have about 2,300 operational beds, which we will increase to 3,000 operational beds in the next 3 to 4 months, with 445 more in active deployment. That means a total of 2,800 to 3,000 beds will be ready in the next 3 to 4 months. Our target for 3 to 5 years is 7,000 to 10,000 beds. The target for the next 4 years is to increase PAT by about 4x to 5x from here. These targets are based on unit economics that we have already proven at scale.

For the first time, our balance sheet has been audited through Grant Thornton, one of the world's top four companies. In this quarter, the Jeena Sikho Swadeshi Health Card was successfully launched. This is a swadeshi initiative designed to strengthen patient loyalty through referral benefits, diagnostic discounts, and seamless service integration across the entire ecosystem. Its trial phase is currently ongoing.

We are expanding the product portfolio into OTC pharmacy retail. New launches have already begun, and international expansion has also started from the UAE, where demand for authentic, evidence-based Ayurveda care is increasing. We have 49 NABH accredited facilities, with three more in the pipeline.

We are empaneled with CGHS, CAPF, ECHS, Air India, and Air Asia. All major health insurance companies are empaneled with us on a cashless and reimbursement basis. We are

on

the panels of the state governments of Uttar Pradesh, Bihar, Haryana, and Punjab, as well as the Central Government, where bills for all employees are passed.

I have some good news, just four days ago, the Uttar Pradesh government launched its UPSS scheme, in which Yogi ji spoke about cashless Ayurveda. This is a very big deal for us because UP is India's largest state, and we have the most centers in Uttar Pradesh.

Our company is not a wellness company; it is a mainstream healthcare provider, measured just like any other hospital in India. We are both preventive and curative. We are always committed to creating long-term value for all our stakeholders.

With clinical depth, operational scale, research credibility, brand trust, and the balance sheet we have built, we are in the best position today to fulfil that commitment -- better than ever before.

The mission is clear, the model is proven, and the momentum is real.

Now, I ask Nanakji, our CFO, to provide a detailed financial review of FY26. Thank you. Jai Hind.

Nanak Chand: Thank you, Acharya Manishji. Good afternoon, everyone. I will now take you through the key

financial highlights for Q4 FY26 prepared in accordance with Ind AS. JSLL has delivered a robust financial performance for the quarter and year ended 31st March 2026. For the quarter, our revenue from operations stood at INR216 crore s, up by 55% year -to-year.

Gross profit increased to INR191 crore s with a gross margin of 88%. Our EBITDA came in at INR78 crore s, recording 70% year -on-year growth with a margin of 36%. Profit after tax stood at INR45 crore s, reflecting a 79% year -on-year growth. Our basic EPS for the quarter is INR 3.65.

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Similarly, for FY26, our revenue from operations stood at INR801 crore s, up by 71%. Gross profit grew by 71% to INR710 crore s. With a gross margin of 89%, our EBITDA came in at INR 349 crore s, growing by 149%, while the EBITDA margin grew by a whopping 1,360 basis points to 44%.

Our PAT also registered a robust growth of 177% to INR222 crore s, with the PAT margin registering a robust growth of 1,062 bps to 28%. The improvement in profitability was driven by higher operating leverage from increased patient volume, strong contribution from the higher margin product sales, and controlled operating expenses despite network expansion. Our balance sheet remains strong with prudent financial management, improving cash flow, and a capital -light expansion strategy supporting superior ROCE.

However, in Q4, on a quarter -on-quarter basis, there is a short -term impact of one -time non -recurring items, largely on account of higher provisioning related to the new labour code, ESOP provisioning, and certain performance -linked bonuses of approximately INR7 crore s. For further clarity on this, during the quarter, we made a one -time ECL provision of approximately INR5 crore s.

Additionally, following the appointment of new statutory auditors and adoption of the applicable accounting treatment under Ind AS, the company conducted a thorough review of leasehold adjustments. As part of this review, certain additional provisions were booked for lease -related expenses of approximately INR9 crore s, which impacted the profit for the quarter. The quarter further reflects incremental provision for approximately INR1.25 crore s related to loyalty points accrued under the company health card program.

I would like to clarify that the Q4 earnings decline is solely due to these one -time items and does not reflect any weakness in the business. Excluding these, operations remained stable, and the company stays committed to sustainable growth. Thank you.

Moderator: Sir, shall we open the floor for Q&A?

Manish Grover: Yes, please start the question -and-answer session.

Moderator: Thank you very much. We will now begin the question -and-a

nswer session. The first question

is from the line of Akshay from AK Investment. Please go ahead.

Akshay: Namaste, sir. Thank you for giving me the opportunity. First of all, congratulations on the great set of Q4 numbers. Sir, my first question is that our OPD, IPD, and daycare volume in Q4 has slightly declined, not by much. Can you clarify the reason for that? Because we spoke last time...

Manish Grover: Sir, that happened because of Trump, sir.

Akshay: Sorry?

Manish Grover: Sir, it happened because of Trump sir, because panic spread across the whole country. Because of that, the advance bookings people had for March -- people deposit advances with us -- our

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preventive customers, who are not sick but don't want to get sick in the future, they book packages in advance.

Someone says they will come on March 3rd, someone says March 9th, someone says March 21st for 10 days. When this Iran and America war situation arose, those bookings are still with us; they haven't been canceled. We have the people's advances. But they did n't come in March because a rumor about cylinders spread across the country -- that cylinders wouldn't be available. So, people first look after their homes before going for wellness. Because of that, the money is lying with us. Many of those people have started coming this month. For example, in my Meerut hospital, the average was never above 240 customers. But last night when I asked, the average was running at 316. So, the people who deposited advances are coming now. The sick customers came, but the preventive customers delayed their visits at that time. That caused a slight difference.

Akshay: Right, sir. And sir, the second thing is that our medicine sales in the last quarter were INR121

crores, and this quarter they are INR118 crores, meaning a decline of INR3 crores. It's not much, but last time we spoke, you said the tie-up with Entero was done and our medicines would be available in medical stores. So, what can we expect now? Will sales growth start from this quarter in the products business?

Manish Grover: Sir, actually, the tie-up with Entero was done, but the business model didn't start because Entero was asking us to bear the expenses. So, we have now increased our presence in newspapers, social media, and TV. We have also launched new products this month – 2 products in May. You will 100% see the impact of that in the next 2 to 4 months. The commitments we made in previous calls about future numbers and profits remain as they are; we have no problem of any kind. In such a large business with an INR800 crores turnover, seeing INR2 crores to INR4 crores up or down quarter-on-quarter doesn't matter to us. We are long-distance runners; we have to go further. We see INR3000 crores in sales in the next 3 to 5 years, so why would we focus on INR2 crores to INR4 crores?

Akshay: Okay, understood.

Manish Grover: What happens quarter-on-quarter is that when our auditors changed and GT came in, previously when we closed the financial year quarter, we used to book our advances as well. But GT said in our practices that you should only book the patient once the bill is generated and they leave. Otherwise, my fourth quarter is INR6 crores higher than the third quarter in terms of collections. But that sale was booked in the third quarter because the fourth quarter advance was booked then. In the fourth quarter, this practice was stopped. Otherwise, my fourth quarter would also have been INR9 crores higher.

Even now, as of May 31st, I have an INR8 crores advance, but that is now carried forward to June because we don't book advances anymore. Sir, when we bring in a large auditor, we have to change our practices as they guide.

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Akshay: Okay. And sir last question.

Moderator: I'm sorry

to interrupt, Akshay. You may please rejoin the queue for more questions.

Akshay: Okay, thank you.

Moderator: Thank you. We will take our next question from the line of Ankur from Genuity Capital. Please go ahead. Ankur your line is unmuted you may please go ahead with your question.

Ankur: Thank you. Sir, just to be clear, you mentioned an extraordinary INR21 crores in the presentation this time. Just to be clear, if I add that back to EBITDA, the EBITDA margin would be 46%, ignoring these one-time adjustments. Is that understanding correct, sir?

Manish Grover: You are absolutely right. Actually, what happened is that our labour laws changed, so the impact of that came in. Plus, we gave an extra bonus to our team, our junior staff whose salaries are low; that was about INR9 crores to INR10 crores. Some Ind AS rules were applied, so an INR7 crores rental provision came in for our built properties; about INR7 crores extra was booked because they are on 9-year leases.

Some impact came from the Swadeshi Health Card we launched, about INR2 crores. An INR3 crores gratuity impact came in. Otherwise, our profit is not lower anywhere. If you look at my EBITDA, it is INR349 crores on INR801 crores, which is approximately 45%. Last year, my EBITDA was INR125 crores, which was 27%. So, we are doing better year-on-year. And I have said in the last two or three calls that we have become a company that gives quarter-on-quarter results, but we are for the long term; don't judge us quarter-on-quarter.

When we implement a new policy, it sometimes takes a month or two for the impact to show.

Like the two products I launched last month, the impact will come in the next two or three months. Now, the expense gets booked in the previous quarter, and the benefit comes in the next quarter. Like in FY25, when I gave my final balance sheet, what happened then? The expense was booked, and we had an INR91 crores profit.

But in the next quarter, it was INR51 crores because the impact was booked in the previous year, but the benefit came in the next quarter. Where I earned INR47 crores or INR45 crores in 6 months, I gave INR51 crores in the next 3 months. And if you want, I can give you all a number.

In FY25, our direct cost was 12%, but in FY26, it became 11%, it decreased by 1%. Manpower cost was 21% in FY25, which decreased to 19% this year.

Advertisement cost was 12% in FY25, which remained at 8% this year. Rent expenses were 7%, which are now 6%. Other operating expenses were 15%, which are now 9%. So, if you look at all this, my total expenses, which were 70%, have decreased to 56% this year, which shows good efficiency. That's why my EBITDA and net profit have both increased. We earned a profit of INR300 crores and paid INR75 crores in tax to the government. Yes, tell me.

Ankur: So, two more things if you could explain. One, how will our average revenue per bed grow from here? And second.

Manish Grover: Sir, I will explain it to you in a different language, in a different way I calculated today. The model we used to explain was based on occupancy beds. Today, I've changed my calculation; I don't want to go by occupancy beds anymore. We have 2300 operational beds. 12 months times 30 days makes 8,28,000 bed-days. I collected a total revenue of INR375 crores. So, that makes INR4650 per bed if I assume all beds are occupied.

And if I tell you a new rule, the agreements we are having with health insurance companies are at a rate of approximately INR8,800 to INR9,200 per day. And the new draft from the Government of India for the Ayushman scheme, which God willing will be implemented in a month or a month and a half, is also coming in at a rate of INR8,800 to INR9,400. Even if I occupy all beds, I am currently doing INR4,650 in revenue. If the Ayushman scheme comes and health insurance

ce increases, then sir, look, the sky is the limit. And I will make all beds operational this year. I said in the last quarter's con call that I would make the beds operational in the next 3 months.

But because of the Iran and America situation, I deliberately didn't make them operational because the expense would have increased. Suppose the war had dragged on or an India-Pakistan war had started, it would have been a problem. So, we took a smart decision not to increase operational beds. But now I will increase them in the next 3 to 5 months; I will make all non-operational beds operational.

Ankur: Acharya ji, if this insurance or Ayushman Bharat comes even in the next year, then from here...

Manish Grover: No, sir, it will come in one to two months. The first draft of the Government of India has already reached the Ministry. It has also appeared in the newspapers. And the day before yesterday, Yogi ji's statement also came, he has announced the policy for cashless treatment for government employees in Uttar Pradesh. And UP is India's largest state, and I have the most centers in UP. I have details of seven colleges that I can take whenever I want, and I will be operational within 3 to 4 months. I can increase 2,000 beds in a year whenever I want. My entire homework is ready. I have the team of doctors ready. We are ready to bring a revolution. As soon as any news comes from the Government of India and their payment portal is fixed, I have the power to immediately increase 2,000 beds within a year.

Ankur: So Acharya ji, this INR4,560 you mentioned...

Manish Grover: INR4,650.

Ankur: INR4,650. This will gradually go back to INR8,500 or INR8,800. Is this correct?

Manish Grover: Sir, you do the calculation work. I have explained it to you in a new language because last time someone told me that you are showing 58% occupancy, calculate it on all beds. So, I came up with that formula today.

Ankur: Okay, sir. Okay. All the best sir. Thanks.

Manish Grover: And sir, look, even taking INR4,650, we are making a 45% EBITDA. How many companies are there in India earning more than INR200 crores in profit? So, think where we will go from here. And there are 19 lakh beds in the whole country. 11 lakh private and 8 lakh government. Out of those, I have only 2,300 beds currently operational. As I have said many times, in 3 to 5 years, I will make it 7,000 to 10,000.

Ankur: Okay, Acharya ji. Thank you.

Manish Grover: Thank you, sir.

Moderator: Thank you. Next question is from the line of Ankur Kumar from Alpha Capital. Please go ahead.

Ankur Kumar: Hello, Namaste sir. Very good results.

Manish Grover: Namaste ji, how are you?

Ankur Kumar: Good, sir. Sir, my one question was about what you said regarding March, that some things were pushed and people are coming now. So, can you tell us the monthly run rate, how it was in March versus how things are now?

Manish Grover: I didn't understand, ask again.

Ankur Kumar: Monthly revenue run rate, sir. You were saying March was a bit pushed, people didn't come because of the war. So, how was it in March versus now?

Manish Grover: No, sir, only 250 people didn't come -- only 250 people who had made advance bookings. Out of 40,000 people, it's an impact of only 250 people, and that's only for one quarter. If you look year-on-year, there's no problem. As I said, in Meerut alone, my highest never went above 230 - 235 patients staying at one time. Currently, I have a number of around 300 -350, around 330

average.

Ankur Kumar: In April and May, the war hasn't stopped, and there are some problems in India too. .

Manish Grover: But we changed our strategy. We understood that this Iran -Iraq, Iran -America, or India -Pakistan thing will keep happening. So, we made some changes in our business model. We launched the

Swadeshi Health Card. We increased focus on that and put it on a referral system , that patients

refer patients, and in return, their treatment will be free.

We linked people with loyalty points, showing loyalty towards an Indian -built organization that is preventing people from falling ill. Currently, the whole world is running on curative. We are the first such company in India talking about preventive - why fall ill at all? Now, I'll tell you a number. India has a population of 140 crores , sir. If I take 1% of that, it's about 1 crores 40 lakh. If I take 1% of that, it's 140 ,000, which is 0.01%. If I can explain to even 0.01% of the world that prevention is the only cure, then sir, I have collected INR400 crores from 40 ,000 people. So, if they become 140000, then sir, my INR1500 crores is collected, right? I'm talking about an INR3000 crores turnover. I just need to admit 140000 people, and my turnover will reach INR1500 crores . And in a population of 140 crores , are there not even 1 ,40,000 people who will

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understand that prevention is the only cure? But it will take me a year or two for that because I have to make people aware. Like you all are connected today, you too are going away hearing prevention is the only cure instead of prevention is better than cure.

So, you might tell your parents, your brother or sister, or some relative Brother, instead of falling ill, go and get done for 8 days first. It's all covered in health insurance; your treatment will be free. You used to go to Manali, Mussoorie, or Shimla; this time go for wellness. So, sir, the country will change. And we have brought it this far, haven't we? When we were doing INR140 crores in revenue, people used to ask us such questions .

"How will you go forward?" From INR140 crores revenue, we have reached INR800 crores today. Now, from INR800 crores , you will see us reach INR3000 crores revenue in no time.

Ankur Kumar: Yes, sir. Thank you. Sir, one more thing I was asking. This INR21 crores one-off -- was it all in this quarter, or will things like bonuses and rent continue in the future, or was it just a one -off?

Manish Grover: No, no, sir, this is a one -off. Everything from the last two or three years was collected, because when GT entered, all the old bonuses, new bonuses, and staff employee benefits were covered in this.

Ankur Kumar: Okay, sir. One last question...

Moderator: I'm sorry to interrupt, Ankur. You may please rejoin the queue for more questions.

Manish Grover: Whoever has more questions, Nanak ji, give them an email ID where everyone can mail. You can query the questions whose answers remain on mail. So, my IR team, Go India, will give you all those answers. And I'll tell you all one thing last year in FY25, we collected INR470 crores by spending INR58 crores on advertising. And this year, we collected INR800 crores by spending only INR66 crores on advertising. Which company is there in India that increased revenue by almost 70 -75% without increasing advertisement costs? Anyway, next question.

Moderator: Thank you. Next question is from the line of Chirag Shah from White Pine Investment. Please go ahead.

Chirag Shah: Thank you for the opportunity. Namaste, Acharya ji.

Manish Grover: Namaste, Chirag ji. How are you? It's been four years, and for four years, I've definitely taken your question every time. Your question didn't come in the last concall.

Chirag Shah: Yes, sir. Acharya ji, a clarification first before I ask my questions. Regarding what Nanak ji said about Q4, could you clarify how much of it is an accumulation from previous quarters and how much is recurring in nature? Like the INR7 crores mentioned for the new Labour Code and ESOP, some of it must be recurring, right? I mean, this entire INR21 crores breakup you gave - - was it all

booked in Q4 or over the whole year? That's the first question. And second, how much of it will remain? How much is a one -off impact and how much will continue going forward?

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Manish Grover: In my opinion, it was all booked in Q4, but I don't have full knowledge because I'm not the CFO.

Nanak ji can tell you. So, let's do one thing, Nanak ji, you answer this.

Nanak Chand: Hi, Chirag ji. How are you?

Chirag Shah: Very good. You tell me.

Nanak Chand: All good. Chiragji, the bifurcation is like this the employee benefit of INR7 crores and the ECL provision of INR5 crores are one -time. Apart from this, the leasehold impact we took will gradually reduce in each quarter from the next financial year onwards. And there are many other things, like our block addition this year is INR42 crores . INR32 crores is our new addition of assets, and around INR10 crores transferred from WIP. So, some additional depreciation has been incurred. So, there is a figure of approximately INR19 crores that is a one -time hit in our last quarter.

Chirag Shah: All in Q4. Okay. So basically, you're saying that like in the lease example, we have provided INR9 crores to INR10 crores more than our actual rent payment.

Nanak Chand: Yes, yes.

Chirag Shah: Right. And this will gradually decrease. Suppose we are giving INR50 crores , we have made a provision of INR7 crores .

Manish Grover : Chirag ji, these are some indigenous standards; even I didn't know them. These were implemented after GT came; we didn't even know. Because they then take it into provision, but it will gradually decrease. And tell me one thing, we are giving an INR9 crores bonus to the staff -- the junior staff who don't come under PF. Think how well they will work too, getting such a great bonus because the company is earning well. So, we are giving that benefit to our staff too.

Chirag Shah: That's a great thing. And sir, now the real question is, the way we've seen a decline in quarter - on-quarter bed utilization or IPD patients, you explained that. But we've been at a very fast pace for the last three or four years. Are we doing some re -jigging in the model so that macro events -- today there's a war, tomorrow something will happen because of rain, inflation will come -- so how are you looking to grow your bed addition in a sustainable manner? Utilization?

Manish Grover: Sir, what I did was, I appointed separate managers in small hospitals a month ago. Now we are treating each hospital as a unit and we have started camps for the reversal of local diseases there, like knee pain, back pain, diabetes, blood pressure. So now, whether it's an Iran -Iraq or Iran - America war, or an India -Pakistan war, it won't make any difference in the coming quarters. We suffered two shocks last year. First, the India -Pakistan war situation happened around our second quarter. Then this America and Iran shock happened around the fourth quarter. So now we've decided that we have to rise above the impacts of these. So, we've picked up all the common problems in India.

For example, no one is working on Parkinson's in India. No one is working on paralysis. We've made new hospitals into specialty hospitals. And now the new change we're going to start is Jeena Sikho Lifecare Limited
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super -specialty. We're going to do a lot of work on Gynae -- on female problems. And we're bringing products for children -- the problem of children being born sick, or children having diabetes or blood pressure.

We're also bringing products to be used during female pregnancy, which will be launched all over the world in the next 3 to 6 months. All R&D is done; trials are complete. Now we're just preparing to launch. So, this will bring a very big boom in our product marketing too, and the same patient will use the product and also come

for Panchakarma.

Moderator: I'm sorry to interrupt, Chirag. You may please rejoin the queue for follow -up questions. Thank you. Next question is from the line of Deepak Poddar from Sapphire Capital. Please go ahead.

Deepak Poddar: Hello, am I audible, sir?

Manish Grover: Yes, Namaste sir.

Deepak Poddar: Yes, a very big Namaste and many congratulations. Sir, I just wanted to understand first up, this one-off impact of INR21 crores -- how much of that are you saying was one -time and how much is recurring? What is the bifurcation of one -time versus recurring?

Manish Grover: Sir, I don't know that. Nanak ji? Sir, actually, I don't know ROCE and all that.

Nanak Chand: INR19 crores to INR20 crores is a one -time impact, and the remaining will be recurring. The major factor in this is...

Deepak Poddar: So, INR19 crores out of INR21 crores is one -time and INR2 crores is recurring?

Nanak Chand: Yes, yes.

Manish Grover: Because the health card one will continue in the future too. Otherwise, if you look at our profit, our EBITDA is INR349 crores , and last year it was INR125 crores .

Deepak Poddar: No, but in this, you mentioned some rental lease provisioning of INR7 crores . That will continue, right? So that was INR7 crores -- how are you saying INR2 crores is recurring?

Nanak Chand: Sir, that was an INR9 crores leasehold impact, but it doesn't include additional depreciation. Additional depreciation of approximately INR5 crores has impacted this year. The reason for this was that most of our leases were within a 3 to 5 -year range. Because of that, it was a one - time impact. From next time, it will be distributed in an equivalent amount throughout the year,

quarter -to-quarter.

Deepak Poddar: Okay. So in view of this.

Moderator: I'm sorry to interrupt, Deepak. Please rejoin the queue for follow-up.

Deepak Poddar: So this is a follow-up only on the question that I was asking.

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Manish Grover: So Deepak ji, you send a mail. Our IR team and our CFO sir will answer. Whoever's question remains; you send us a mail. And if you study our results well, I think GT's report alone is more than 200 pages. They have covered every single thing in it. They visited at least 25 of our hospitals became patients themselves, got admitted, purchased things there, gave cash and saw whether the entry comes or not.

Their sampling was very extensive. That's why the result was also delayed, because we had never worked with a large company like GT before. You can understand the difference between previous auditors and GT auditors and their check parameters. That's why our result has come for the first time on May 30th. Otherwise, we used to give quarter results around the 15th or 17th.

Moderator: Thank you, sir. Next question is from the line of Naveen Baid from Nuvama AMC. Please go ahead.

Naveen Baid: Thank you. Thank you for the opportunity. Namaste, Grover ji. My question was that we are talking about INR3000 crore revenue over the next 2 to 3 years?

Manish Grover: No, 3 to 5 years, sir.

Naveen Baid: Okay, 3 to 5 years. So, how will our mix change when we reach INR3000 crore? What will be the mix of the services business and the products business?

Manish Grover: Approximately everything will be 50-50. If you listen to my last four or five concalls, I have said that our mix will remain 50-50. Average say between 45% to 55%. Because in some months, one will be 2% higher and in some months, the other will be 2% lower. Even this time, there's only a 2% difference, 48% is services and 52% is products.

And services, otherwise, is 55%. If I give you a comparison, last year we did government sales of INR118 crore. This whole year, I did INR36 crore. So, if you see, I myself reduced my government sales by INR80 crore. It's not that the government is ref

using to do business. I

could have done INR80 crore, but the credit gets stuck, then provisions have to be taken, government money comes late.

Because of this, I said until the new portal opens, why even do the work? Now you see, last year my private sales were INR136 crore. This year my private sales are INR349 crore. That means I have increased private hospital sales by more than 2.5x. So, such efficient management that took an immediate decision when we said in last year's concall people said money gets stuck in the government.

So, we immediately shifted the business model to this model. And we increased private sales from INR136 crore to INR349 crore and retail sales from INR215 crore to INR415 crore.

So, I almost doubled retail and if you look, I increased hospital sales by 2.5x. It's just that I reduced government sales, so the impact is less visible.

Naveen Baid: Correct. So that's a strategic decision?

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Manish Grover: Yes, sir. Sir, there's no point in getting credit stuck until the government payments come on time. And I'm telling you again, all the big hospitals in India, whether Allopathy or Ayurveda, will have to remove their dependency on government business. Because the government is catching so many scams in hospital billing that those hospitals then get blacklisted.

Naveen Baid: Correct. And my follow-up question was, we don't give separate operating profit margins for both segments which is our services business and products business. But roughly, if I look at allocating costs, is it fair to say that in our product business, we make approximately 50% operating profit margin, EBITDA margin?

Manish Grover: No, sir, I calculated it; it comes to almost the same in both, maybe 1% up or down.

Naveen Baid: Okay in both the segment margins are similar?

Manish Grover: Because both complement each other. Look, the one who bought the product is naturally my customer who will come for Panchakarma later. The one who bought Panchakarma uses the product for many months. So, how will you differentiate your advertisement expenses? So, we had done a lot of calculations, and that's why when we reach INR3000 crore revenue, both will grow equally.

Like I gave a figure just now if I admit 1,40,000 people in a whole year, my revenue will reach INR1,500 crore s. And 1,40,000 people in India's population of 140 crore s make 0.01%. I just have to explain to them not to wait to fall ill; come before falling ill. Don't go for curative, go for preventive. And this is a completely new company talking like this. Sir, we will be taught in IIMs and IITs in a year or two.

Naveen Baid: Surely, sir. And when our revenue reaches INR3,000 crore s, will our margin remain similar in both segments?

Manish Grover: Sir, we don't work on less margin than this because my research will have become such a big wing by then that people from all over the world will start coming to India. In the next 2 years, I will build such a large research backend that when it becomes an INR3,000 crore s company, the whole world will come to India for treatment and prevention.

Currently, I'm only focusing on India. In the next 2 years, when my research wing is at the top, then people from America, UK, Middle East, Australia will come to India for treatment, right? When my research backing becomes very large. And my next investment is all in research.

Because you all say evidence based.

So, I said 190 of our papers have already been published. And if you look at our PPT carefully, seven Nobel Prize studies are mentioned in it, based on which we admit patients and do their treatment or give them preventive therapies. I request everyone, please read our PPT very carefully.

Seven Nobel Prize studies are mentioned in it. And you all should also take care of your families and yourselves don't fall ill. We don't

have to fall ill at all. We will build such an India that

doesn't want to fall ill. Because currently, the whole of India falls ill and then damages its GDP,

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damages its family. People become poor, lakhs of rupees are spent, and then nothing is gained.

So, I say we should service the car before it breaks down. And you all should do this too.

Naveen Baid: Surely, sir. Thank you, sir. That answers my question. Thank you and all the best.

Moderator: Thank you. Next question is from the line of Hitesh Jain from Avagrah Capital. Please go ahead.

Manish Grover: Namaste, sir.

Moderator: Hitesh, your line is unmuted. You may please proceed with your question. As there is no response from the current participant, we will take the next question from Deep Sanghavi from Dalal & Broacha. Please go ahead.

Deep Sanghavi: Hello, am I audible?

Manish Grover: Yes, Namaste sir.

Deep Sanghavi: Namaste, sir. Congratulations on the Q4 results. My first question was that as you said in the opening commentary, you will increase beds from 2,800 to 3,000. Could you give a bit more detail about that? And how will the occupancy be as it increases?

Manish Grover: Sir, we have caught many new small diseases knee pain, back pain, Parkinson's, paralysis, depression, anxiety, lack of sleep. We took data in India 10 crore s people are taking only BP pills in India. And 13 crore s people are pre-hypertensive. 10 crore s people are taking diabetes pills, and 7 crore s people are pre-diabetic. Similarly, 8 crore s people are taking thyroid pills. 5 crore s people have joint pain problems. So, the total number is coming to 44 crore s.

So, if I am starting the education of all these, I have started some new webinars in which I am addressing only blood pressure, only diabetes, only asthma. Previously, our focus was only on chronic disease. Now we have caught every disease in India. Currently, 80,000 children are born every day in India. That means 22.5 lakh children are being born in India every month. And all children are being born unplanned.

So, my new project is that before having a child, connect with Ayurveda for 3 months. That will be launched this month, in June. Sir, think how big a project that will be that every child in India, before being born, you prepare 3 months in advance as written in the books of Ayurveda. That no child should be born unplanned if you want the child to be strong.

We have developed the product; its trial is complete. So, we are going to start a campaign, a mission that if you want to have a child, then 3 months before, use our products, this Ayurveda, these mantras, this meditation, these eating and drinking things and then plan the pregnancy. So think, sir, 22.5 lakh children are being born a month. Even if 10% of the customers move towards me, sir, this turnover alone becomes at least INR250 crore s a month. I didn't want to say this in your concall, the product that is being launched this month.

Deep Sanghavi: Okay. So, what occupancy are you targeting for these 3,000 beds you mentioned?

Manish Grover: Ask again, sir.

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Deep Sanghavi: Sir, you said in the opening commentary that you will have 2,800 to 3,000 beds in 2 months to 3 months?

Manish Grover: Yes, I will do it in 3 to 4 months. I will do it because whoever wants to have a child will first come and get Panchakarma done with me, right? So, I will need beds for that too. These products are being launched this month. So, every woman in India who has problems with periods, who has uterus fibroids, who has PCOD and any male who has oligospermia or who has sexual wellness problems, they will all come to be admitted, right? We have made 8-day and 14-day packages and made them very cheap. And if they all get admitted, then I will need

beds too.

Correct.

Deep Sanghavi: Thank you. My second question was about our medicine.

Moderator: I'm sorry to interrupt, Deep. You may please rejoin the queue for follow-up questions.

Deep Sanghavi: Thank you so much, sir.

Manish Grover: You all please mail me if you have a question pending.

Moderator: Thank you. Next question is from the line of Hitesh Jain from Avagrah Capital. Please go ahead.

Hitesh Jain: Yes, sir. Thank you for taking my question. Acharya ji, my one question is about the unit economics you explained in the PPT on a per-bed basis. And as you said, your payback happens in about 12 to 18 months and in small hospitals, it happens in even less than 6 months. So, when we are at a top line of INR3,000 Crores, how will this change?

Manish Grover: Sir, in the INR3,000 crore top line, INR1,500 crore will come from products and INR1,500 crore will come from healthcare services. And if you look, for the last 1 year continuously 1 year ago, our health insurance contribution was 4%. Today, my health insurance contribution has reached 26% in total sales.

And I am having regular meetings with health insurance companies. Last month too, I was in Mumbai, where I had a three-hour meeting with the entire top management of HDFC ERGO. I explained to them that whoever is having knee surgery, you send them to us first, then send them for surgery.

Similarly, for eye surgery, I said whoever you give money to for eye surgery, send them to us first; if we can't do it, then send them there. So, 7 out of 10 jobs will be fixed in less than half the money because we have such research, such data. I took everything with me. They were very impressed and regular meetings are going on.

So, sir, the 40,000 patients admitted in a whole year and we gave an average ticket size of INR8,000-INR8,200, whatever came by calculation. But if I assumed all beds were occupied, then it's INR4,650, as I just calculated. And the rate the government is passing in the Ayushman scheme is INR8,800 to INR9,400.

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And health insurance companies are also giving a rate of approximately INR8,800 to INR9,200.

So, sir, there is no problem of any kind, no challenge for us. It's a very simple and easy model; nothing complicated in it. The more the occupancy increases, the more our benefit will increase.

Hitesh Jain: And sir, your incremental investment will these beds be on the scale of small hospitals or how?

Manish Grover: Sir, mostly, I am starting two luxury wellness centers this month in June one in Panchkula, Chandigarh and one I am starting in Manali in July. Ultra-luxury wellness, which has 108 rooms, 22 villas, two helipads and is in a 40-acre forest. This is ultra-luxury and I invite you all of you connected in the concall right now to register yourselves.

A 2-night, 3-day stay for all of you is free from my side. Because you should come and enjoy Panchakarma; you should know what Panchakarma is. So, currently, there are about 550 people.

Madam, send Yes in front of everyone's names. They will message us and we will give you a 2-night, 3-day stay, residence free, in ultra-luxury wellness.

I mean, you will enjoy it in Manali, and there are no buildings nearby. It's a private road, a 42-acre forest. And in that, for the first time in India, forest bathing, forest meditation with Deodar trees that will start for the first time in India. As I said, my entire purpose is one not to let the people of India fall ill.

You go to the doctor after falling ill. I am talking about an India that doesn't want to fall ill; go to the doctor first. Brother, doctor, teach us not to fall ill. So, sir, you all are my guests. Ultra-luxury is starting in Manali and a four-star is starting in Panchkula in June. That too will be for wellness.

Because currently, what happens is that the wellness patient

comes to my Mumbai or Meerut

one. But when he sees the sick patient, he feels a bit uneasy. So, I am going to separate my

wellness and my preventive and curative from this month. Meaning, prevent ion ones will go to separate centers and curative ones to separate centers.

Hitesh Jain: Right. Thank you, sir.

Moderator: Thank you. Next question is from the line of Abhishek from AB Capital. Please go ahead.

Manish Grover: Namaskar Abhishek ji.

Abhishek: Namaste, Acharya ji.

Manish Grover: Namaste ji.

Abhishek: One thing I asked -- one you said that we are targeting PAT to be 4 x to 5x in the next 3 years.

Did I hear correctly?

Manish Grover: 3 to 5 years.

Abhishek: In 3 to 5 years, you are th inking PAT will be 4x to 5x?

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Manish Grover: Absolutely. But now I feel I should talk about EBITDA. Because you people like EBITDA more.

When I said INR350 crore s EBITDA, people liked hearing that more.

Abhishek: No, PAT is good, sir. And sir by the end of next year, how many operational beds can we target?

Manish Grover: Sir, I said that all my non -operational beds, I will make them all operational within the next 3 to 4 or 5 months. Because I am launching a new product, as I said, for female wellness and male wellness. And for all of them, a 7 to 8 -day stay is mandatory b efore having a child. So then I will need beds too.

I am starting a new campaign in India that no child should be born without planning. First detox your body, first detox your liver, first detox your kidney, first make your sperm healthy, females make their ovum healthy, then plan the child. And I am not saying this; it is written in our scrip tures that no child should be born unplanned.

Abhishek: So more than 3 ,000 operational beds?

Moderator: I am sorry Abhishek please rejoin the queue.

Manish Grover: Yes. This year I will reach 3 ,000 to 3 ,500 beds before March 31st.

Abhishek: Okay. And one it's not a question, just a request. From the next quarter, please Acharya ji, do the concall on the same day. Otherwise, the market g ets shocked and the stock falls?

Manish Grover: Sir, actually, I was not in India and I was on a flight, so it couldn't happen. I am still on a mission. To train the young generation of India, I have come for a shoot for a show outside India. That show will be launched in August. And in that too, my mis sion is that people don't fall ill; I have come for that show for that reason. So, it was a compulsion; the flight was 14 hours long, what could I do .

Abhishek: Thank you.

Moderator: Thank you. Next question is from the line of Praveen Kumar from Praveen Investment. Please go ahead.

Manish Grover: Namaste, Praveen ji.

Praveen Kumar: Namaste. Sir, I have two questions and one quick comment. Okay, my first comment is that.

Manish Grover: First, I will ask you that have you done body detox in your lifetime?

Praveen Kumar: No yet. I will plan.

Manish Grover: Promise in front of everyone in front of 550 people that I will take care of my body.

Praveen Kumar: Definitely, sir. I am taking care, but I still have Sir my first comment is that y ou guys devise so many medicines and I kindly request you to devise a medicine for investors not to panic when

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this stock falls. Okay, this is on a lighter note. And coming to my question, sir, you said there is a great momentum on the ground.

Manish Grover: Sir it's my request if you ask me in Hindi, it will be good so that I will be able to answer easily.

Praveen Kumar: Yes, sir I am not that comfortable. I am sorry I am really not comfortable.

Manish Grover: Okay you speak I will tell.

Praveen Kumar: Yes definitely . My first question is you said there is a great momentum on the ground now. So, what can we expect for FY27? What can be the revenue guid

ance you want to say anything on

that? That's my first question?

Manish Grover: Sir, minimum INR300 crore s PAT is my minimum that I have to achieve. How much more it will go, I don't know.

Praveen Kumar: Understood, sir. And my second question, sir, you want to give any latest status on your engagement with Satkartar, Chandan, and then Entero? Any latest data you want to share with investors? And are they really helping us in moving our business? Anything on that you would

like to share, sir, to investors?

Manish Grover: Sir, from Chandan we have had a profit of INR1 crore s in the last one year. And now my average of INR4 lakh to INR4.5 lakh per day of tests has been reached, which will become very good in the next 3 to 6 months. And the other one, Satkartar , from next month, my business, the new business idea we had, will be launched next month. They have a call center of 1400 people which we will use. And as I said, Satkartar will help us a lot in the next growth that I am going to bring because they have a lot of children.

Praveen Kumar: Understood, sir. And did you say that your engagement with...

Moderator: I'm really sorry, Mr. Kumar. Please rejoin the queue for follow -up questions. Thank you. We will take our next question from the line of Deepak Pruthy from Wealth with Wisdom.

Manish Grover: Ma'am, I request you to please say beforehand to ask only one question. Because when someone starts asking a question and you stop them, it doesn't feel good. Say beforehand to ask only one question.

Moderator: All right, sir. Noted. So the next question is from Deepak Pruthy from Wealth with Wisdom.

Please go ahead. Deepak, please ask your question.

Deepak Pruthy : Pranam, Acharya ji.

Manish Grover: Namaste ji, how are you?

Deepak Pruthy : I am good. I had three questions, but I will restrict to one now. First, I will make a comment.

Acharya ji, I went to your Meerut center -- to the hospital and I met many people there. I went for OPD. There were people from Gujarat, from Madhya Pradesh, from the South too. So, when you have so much demand, why are you not opening more large hospitals like Meerut in other
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places? I felt that people have come from very far, spending a lot of money. And I felt that maybe if this facility was near their homes, they wouldn't have had to travel so much. So, what are you doing on that?

Manish Grover: Sir, if I tell you, everyone's expectations become very high that's why I didn't want to say. A very large hospital in Lucknow is going to be finalized this week. Work is going on for a large hospital in Ahmedabad. We are looking for a new property in Patna. We have found a new property in Kolkata. I am starting another new one in Pune and we are increasing beds in the Mumbai one. And ultra -luxury wellness is starting in Manali next month. A wellness one is starting in Panchkula this month too. So, we are on the job. That's why I said that all non - operational beds, I will make them all operational this year.

Deepak Pruthy : Okay. Acharya ji, I will say one more thing to you. First, thank you very much, you also gave the prasad of dividend this time. And second, to meet you and to take preventive healthcare , as you were saying, take preventive healthcare , so I booked for June 18th to 21st in Meerut.

Manish Grover: I am telling everyone who is on the call right now , all of you take an oath not to fall ill and to go for prevention. Because all you people with money, this money won't be of use if our health has any problem.

Deepak Pruthy : Yes, that's why for June 18th to 21st in your Meerut one -- I couldn't get a booking last month. I tried 10 days before, around May 20th, but I couldn't get a booking for the 28th, 29th, 30th, 31st.

Manish Grover: Actually, when Yogi ji spoke, advance bookings were made a month in advance. No problem, sir. M

ost welcome. I'll give your number to my Ravi, the PA; he will call you after taking the number from the team . So, you are welcome, sir. Thank you.

Deepak Pruthy : No, I have made the booking, sir. I have made the booking by giving advance payment. Thank you. So, I will meet you from the 18th to the 21st. If you shift this to Manali, it would be even better, otherwise Meerut.

Manish Grover: No, the Manali one is opening in July, so after that, we will hold a camp there too.

Deepak Pruthy : Okay, sir. Okay, okay. One last question if I can ask. I saw your expansion in the US on your website...

Moderator: I'm really sorry, Deepak. There are other participants waiting for their turn.

Deepak Pruthy : No worries, no worries. Thank you, thank you.

Manish Grover: Thank you, sir. Thank you. Namas kar.

Moderator: Next question is from the line of Priyanshu Jain from GrowthX Infinity. Please ask one question at a time.

Manish Grover: Namaste, Priyanshu ji.

Priyanshu Jain: Hello, sir. How are you? Congratulations on our results.

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Manish Grover: Good, good. You tell me.

Priyanshu Jain: All good. Sir, I had only one question; all my other questions have been answered. One question was, sir, like our quarter-on-quarter product mix, there has been a slight difference now. Sir, our main concern was not that there was a decline of INR3 crore s to INR4 crore s in sales. Sir, the main concern was that in this, what happens is that our repeat order , for example, if we started from the 1st and in 30 days our product was finished, so it feels like, from this number, it feels to us like -- can you tell us the repeat rate? How many customers took the product for the first time to try, and after that, how many of our customers are coming back for repeat?

Manish Grover: I can give you the data for the last quarter. In the last quarter's data, in our hospital business, if I take the admitted business, there was a 26% repeat business. And on our product side, there was a 34% repeat business because it hasn't been long since the products were launched, right? Now, the first product we launched, it's only been a total of 6 months.

A total of 7 months have passed, in which many ups and downs came in between, changes were made in the product, upgrades were done. And the second product I launched was this month, in May. And this month, in June, I am launching those pregnancy products that every female and every male has to use.

So, sir, product development is going on regularly. And the medicines I launched for BP, sugar, kidney, liver, and depression -- they will all be launched in June and July. They were supposed to be launched in February. What happened was, because of this Iran war situation, we -- because it costs money to launch once .

So we stopped that launch because we felt that this Iran -America instability is going on. So now we will launch them all in June and this month. And now we won't care about the war because we have adjusted all the medicines as a kind of food supplement. Meaning, we've linked them with the patient's regular needs.

Priyanshu Jain: Right, sir. So, sir, can you just tell me how many of our products have been launched in the market so far that are older than three months?

Manish Grover: Older than three months? Currently, in the OTC market, only one product has been launched that is older than three months , the Pet Yakrit Pleea Shuddhi , the green box one. That is the only product older than three months. The second products were launched just two months ago , our Sea Buckthorn juice, Sea Buckthorn Ashwagandha juice, Gut Care juice , they were all launched just two months ago.

A total of nine products have been launched, but they were all launched within three months. And their impact has come on sales this time. Last month, my product sales increased by INR3

crore s.

Priyanshu Jain: Okay, sir. Sir, just one more thing...

Moderator: I'm really sorry, Mr. Jain. Please rejoin the queue for follow-up. Thank you. Next question is from the line of Priyam from Trinetra Asset Managers. Please go ahead.

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Priyam: Hi, sir. Namaste. Thank you for the opportunity.

Manish Grover: Namaste.

Priyam: Sir, we just wanted to ask that our long-term vision of 7,000 to 10,000 beds in the next 3 to 5 years , as of today, we have 2,300 operational beds. So, this gap, I'm sure it will happen, but it's very large. So, does it mean that over the next 5 years, we will increase 1000 -1000 beds every year, or should we assume that this growth will be a bit lumpy , meaning 500 -700 beds this year, then 1200 -1300 beds the next year? So, if you could explain that once ?

Manish Grover: Sir, I will clarify. If you heard my opening remarks at the beginning, I clearly said that I can increase 2,000 beds whenever I want in 9 months to a year. My homework is done. I am just waiting to get the customers -ready , to maintain them, to make up their minds. For that, last month I went to the "Laughter Chefs " show on Colors channel twice. I mean, twice in two months, I went to "Laughter Chefs " on Colors. Two months ago, I went to "MasterChef ".

Currently, I have come to the "Khatron Ke Khiladi " show. I am changing the narrative of India. So, it will take me three or four months for that narrative to change. As soon as the narrative changes, sir, filling beds won't be difficult at all, because currently, the mind of India is curative. I have to put preventive in the mind of India. And my mission and purpose has always been this that my India becomes preventive rather than curative. Because you know what happens?

After falling ill, the whole house is finished in getting treatment. And not one person falls ill in a house; four people fall ill. Along with that, the lives of two or four people are ruined. The one who has mouth cancer, his wife is also ruined along with him. So, I want that we make the preventive market so large first, in the next one to two years, that people automatically start coming towards this in India.

And I request that at least 10 to 15 companies like mine should be listed in the next one, two, or

three years. Only then will a separate market be created, a whole separate world will be created. There are 140 crore people, sir. So, where do numbers value anything? And I also said that if I admitted even 1,40,000 people, the turnover would be INR1400 crore to INR1500 crore.

Management: This will be the last question.

Priyam: Right. Okay. Thank you, sir.

Manish Grover: Thank you, sir. Thank you. And whoever's questions remain, please mail them. Our IR team and our CFO sir, will answer everything.

Moderator: Thank you, sir. Ladies and gentlemen, we will take that as the last question for today. Kindly share your queries on email ID ; soumya@goindiaadvisors.com. And we apologize that we couldn't take all due to time constraints. I now hand the conference over to the management for closing remarks.

Manish Grover: Madam, I request you to tell everyone to read our PPT thoroughly. Prevention is talked about in it. Seven Nobel Prize studies are mentioned. Links to research papers are provided. Links to our Jeena Sikho Lifecare Limited

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nine clinical trial products are provided. We are all clinical trial -based, meaning the product we are launching is first having a clinical trial. You will even see the clinical trials on the CTRI website.

And more than 170 of our papers that have been published, the link to that is also in the PPT.

And testimonials of some recovered patients are also included, of VVIPs. And seven Nobel Prize studies are also included. Our numbers are also included. My vision and mission are also included.

included.

So, please, I request you all to see our company as a different kind of company and not see us quarter-on-quarter; see us as a long-term company as the future of India, as the forehead of India.

And I request all of you who are connected, take care of yourselves and your families. First get Panchakarma done, so that we don't fall ill in this birth and become ambassadors of our mission.

And don't see our company only from an earning point of view; see our company as the forehead of India, as a pride of India, that India can become a world leader in two to three years if, as I am thinking, such research papers come out and the message spreads to the whole world. So, we will invite people from all over the world for treatment in India. Thank you very much.

And one thing to tell you, all the foreign health insurance companies, they too have started moving towards preventive healthcare in Ayurveda; In Dubai too, in UAE too. Whoever brings a health insurance policy from a large foreign company to India, their treatment will also be completely free. Thank you.

Moderator: Thank you very much. On behalf of Go India Advisors, that concludes this conference. Thank you all for joining us today and you may now disconnect your lines.